

THE TECHNIQUE OF PSYCHOTHERAPY FOURTH EDITION LEWIS R. WOLBERG, M.D.

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Among these are (1) the availability of new psychopharmacological agents that are singularly effective for symptoms of anxiety, panic, obsessive reactions, depression, mania and schizophrenia; (2) a greater use of short-term approaches not only because third party payers have placed limitations on the number of reimbursable sessions, but also because brief therapy has established its validity beyond traditional limited goals; (3) a shift toward the sicker end of the spectrum of patients seeking help in outpatient clinics and private practice due to the reduction of inpatient facilities, and emphasis on short-term hospitalization, deinstitutionalization, and many other factors; (4) the continuing development of creative symptom-oriented and problemsolving, supportive, behavioral, milieu and outreach approaches as well as family, marital, and group treatments; (5) greater efforts toward more scientific treatment planning with emphasis on differential therapeutics; (6) expanding experimentation in reconstructive therapy through methods oriented around object relations theory and self-psychology, which have led to changes in the way the more severe personality problems are

being conceptualized.

While the field of psychotherapy is virtually bursting with luxuriant techniques and expanding dimensions of application, little attempt has been made to integrate the information that we have available today.

This poses a problem for students learning ways of managing emotional illness.

Added to their befuddlement is the persistent fervor of some established groups to maintain ideological purity.

Sugary pieties about the need for unity have not succeeded in halting adversarial verbal dueling.

What we have come to realize more and more is that psychotherapy is not a homogeneous operation, being burdened by many contradictory variables.

It is not only the actual interventions that determine the outcome, but also the degree to which the [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 4 patient accepts and utilizes them, the skill of the therapist, and the climate of the therapeutic alliance.

Countless snags and resistances arise in treatment that can sabotage the efforts of the most dedicated therapists and best motivated patients.

And many nonspecific factors influence therapeutic results for the good and bad.

Yet there is a possibility that all good therapists operate in somewhat similar ways if they are to achieve worthwhile results, and this is irrespective of the theories they espouse and the explanations they offer of what they do.

When the original edition of this book was published in 1954, partial solutions for some of the quandaries in psychotherapy were offered.

I had acquired a bulk of data of how therapists with different orientations conducted psychotherapy that registered a positive effect on their patients.

The locus of this information was a community health center in New York City, the Postgraduate Center for Mental Health, which my wife, a psychiatric social worker and clinical psychologist, and I founded in 1945 under the name The New York Consultation Center.

The Center provided a laboratory for study, and the relatively large numbers of patients being treated served as a rich resource for empirical observations.

Our case load in the early days consisted of a sizeable number of civilian patients as well as veterans of World War II who had come back from the European and Asian theaters and who were sent to us for treatment as a contract clinic of the Veterans Administration.

In a short while we were deluged with veterans being discharged from the service, and to treat this mass of emotionally disturbed patients as well as our habitual caseload of civilians in need of help, we had to enlarge our staff greatly.

World War II having ended, the army medical officers with psychiatric training were going back into private practice.

Drawing on this group, we developed a large staff of psychiatrists and a few clinical psychologists, all of whom had been trained in diverse methods of psychotherapy and belonged to practically every school of psychological thought existing at the time.

Although their theoretical belief systems varied widely, and their focus on pathology differed, we soon discovered that there were those who obtained good results and those who did not.

We tried to substantiate what it was that successful therapists did.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 5 Our initial hypothesis was that good therapists, irrespective of discipline, established rapid rapport with their patients, displaying empathy in dealing with expressed emotion.

They developed a theory about the patient's illness and around this formulated a treatment plan involving the patient in at least some of the decision making.

They took the time to resolve unreasonable expectations and to enhance motivation for therapy.

They adapted their techniques to the needs of each patient, e.g. utilization of structured approaches with confused and helpless patients and less structured ones with more highly organized individuals.

They did not hesitate to employ confrontation with patients with whom they had a good relationship in order to create a better learning medium through affect arousal.

They worked within the framework of the patient's belief systems where possible, but when these interfered with treatment they tactfully attempted to alter distorted cognitions.

Analytically oriented therapists did not hesitate to work toward symptom alleviation where symptoms were self-defeating, diverting the patient from selfobservation.

Aware of transferential projections, they controlled their own untoward countertransferential feelings.

The more intuitive therapists employed their personal feelings to divine what was on the patient's mind, conscious and unconscious.

They aggressively dealt with evasions and resistances, recognizing the carry-over of defenses and value distortions from the past.

They provided reinforcements for constructive behavior.

Finally, where the goal was not personality reconstruction, which was most often the case, they terminated therapy at a point where the patient could reasonably manage on his or her own in order to avoid a therapeutic stalemate in dependency.

All of these maneuvers were done irrespective of cherished theories.

As the Center expanded, it developed an organized interdisciplinary training program in psychotherapy and psychoanalysis, obtaining a charter for this from the Board of Regents of the State of New York.

It became one of the largest outpatient continuous treatment centers in the country, servicing a steady caseload of over 1600 patients in its Adult Clinic, Clinic for Children and Adolescents, Social Rehabilitation Clinic, Group Therapy Clinic, Family Therapy Clinic and Clinic for Alcoholism.

Its clinics were licensed by the New York State Department of Mental Hygiene and approved by the Joint Commission on Accreditation of Psychiatric Facilities of the AMA.

Apart from the [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 6 Fellowship program in psychotherapy and psychoanalysis, training programs for qualified personnel were developed within each of the clinics in child psychiatry, analytic group therapy, family therapy, supervision of the therapeutic process, and alcoholism.

The Department of Community Services and Education developed a counseling service and a training program in counseling for social workers, clergymen, teachers, rehabilitation workers, and other personnel dealing with problems encountered in their work.

A twoyear part-time mental health consultation course for certified psychotherapists enabled the servicing of over 600 agencies, institutions, and industrial groups in the community.

As part of public education an ongoing lecture series was developed for the general public.

Continuing education programs for mental health professionals rounded out the activities of the Center.

All of the foregoing activities enabled detailed observation of aspects related to the practice of psychotherapy and, coupled with studies of cases in my own private practice, inspired later editions of this book.

Because of the wide range of its coverage and step-by-step delineation of techniques, the book became known as the "cookbook of psychotherapy," and was also referred to as a treatment manual.

However, it was never intended as either a "cookbook" or treatment manual with standard recipes of operation.

Too many variables exist in the continuum of patient-therapist-environment to allow for universal formulas applicable for all patients and therapists under differing environmental circumstances.

Although the methods described in the book have been tested, they are best utilized as guidelines to be altered to the needs of individual patients and the styles and personalities of practicing therapists.

Throughout the book allusions have been made to a dynamic way of thinking about what goes on in psychotherapy.

The reason for this is that even though the methods employed by the therapist may be non-analytic, one sometimes cannot escape intrapsychic interferences such as those that issue out of the pool of early developmental conditionings.

The form of these interferences are embodied in transferential corruptions and manifold resistances, identification of which would seem to fall within the premises of [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 7 psychoanalysis.

Although in the past few decades we have witnessed the erosion of psychoanalysis as a preferred method of treatment, this has not lessened its value in understanding operative forces in psychotherapy.

“Psychoanalytically oriented psychotherapy” accordingly has become the most common form of psychotherapy in use today.

The psychotherapist who has not been trained as an analyst may still be able to identify transference and resistance, and one may recognize some countertransferential responses within oneself.

In utilizing psychoanalytic concepts in treatment, as in the identification and resolution of resistance, the therapist may still be effective even though not practicing formal psychoanalysis.

Since over 90 percent of patients cannot afford, or tolerate, or constructively utilize intensive psychoanalysis, psychotherapy will constitute the best approach for most patients. Blending dynamics with a skilled use of techniques, the therapist may enable the patient, not only to achieve symptomatic relief, but also to gain sufficient self-understanding to effectuate some important reconstructive personality changes.

The present volumes detail methods through which such changes may be accomplished. It is impossible to include all of the sources I have drawn on for help in the research and writing of this new edition.

I am particularly grateful to my colleagues at the Postgraduate Center for Mental Health who perused parts of the manuscript and offered valuable suggestions: Dr.

Ava Siegler on Child and Adolescent Psychiatry; Dr.

David Phillips on Social Workers and Casework Approaches; Dr.

Marvin Aronson on Group Psychotherapy; Dr.

Harold Kase on Rehabilitative Approaches; Dr.

Gary Ahlskog on Pastoral Counseling and Religious Approaches, Dr.

Maria Fleischl on Eastern Philosophies, Dr.

Zane Liff and Dr.

Henry Kellerman on Psychologists in Psychotherapy.

Thanks are due to Dr.

Cyril Franks for his help on Behavior Therapy, to Dr.

Norman Sussman on Pharmacotherapy and to Dr.

Susanne Lego on The Nurse in Psychotherapy.

I am especially grateful to Lee Mackler for her meticulous work on the sections on Bibliotherapy, Selected Texts, and Films, Audiotapes, and Videotapes.

Thanks are also due to my publishers for allowing me to utilize material from my books: to Brunner/Mazel "The Practice of [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 8 Psychotherapy," to Thieme-Stratton "The Handbook of Short-Term Psychotherapy," to Grune & Stratton "Short-Term Psychotherapy" and "The Dynamics of Personality." Finally thanks are due my secretary, Jeannine Matthews, for her help with the physical preparation of the manuscript.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 9 Part I.

The Scope, Types, and General Principles of Psychotherapy [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 10 1  
What Is Psychotherapy?

Few words in the lexicon of the mental health field are as ambiguous as the term psychotherapy. It is loosely employed to connote, among other meanings, helping, treating, advising, guiding, educating, and even influencing.

Definitions of psychotherapy are often bridled to fields of disciplinary operation, e.g., psychiatry, psychology, casework, etc., sanctuaries for such characterizations being sought in specialized societies.

Diffuseness, in definition, has converted the arena of psychotherapy into a swamp of murky ideas, fostering many divergent theories and techniques.

Yet a brief and precise description of therapy is important if no more than to circumscribe boundaries of operation and for purposes of hypothetical construction and empirical study.

A comprehensive working definition might be the following: Psychotherapy is the treatment, by psychological means, of problems of an emotional nature in which a trained person deliberately establishes a professional relationship with the patient with the object of (1) removing, modifying, or retarding existing symptoms, (2) mediating disturbed patterns of behavior, and (3) promoting positive personality growth and development.

This formulation requires additional elaboration.

Psychotherapy is the treatment.

No matter how much we attempt to dilute what we do in psychotherapy, it constitutes a form of treatment.

Such terms as reeducation, helping process, and guidance are merely descriptive of what happens in the course of treatment and do not really disguise the therapeutic nature of the process.

Forms of intervention other than therapy do exist in the mental health field and will be described later.

Psychological means.

Psychotherapy is a generic term covering the entire spectrum of psychological treatment methods.

These range from designed maneuvers of the therapist-patient relationship to indoctrinations fashioned to change value systems, to [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 11 tactics aimed at intrapsychic processes, and to conditioning techniques that attempt to alter neural mechanisms.

The repertoire of strategies is thus legion, and formats are varied, e.g., individuals, couples, and groups.

They are all, nevertheless, dependent upon the establishment of adequate communication, verbal and nonverbal.

Excluded are such modalities as somatic therapies (drugs, convulsive therapy, surgery, etc.) and "trial action" therapies such as occupational therapy, dance therapy, music therapy, psychodrama, etc.

that, though psychotherapeutic in effect, are not, strictly speaking, forms of psychotherapy.

Problems of an emotional nature.

Emotional problems are diverse, influencing every facet of human functioning.

They are manifest in distortions in the individual's psychic, somatic, interpersonal, and community life.



Manifestations of emotional illness are thus multiple, involving the total human being. In view of this totality of disturbance, it is arbitrary and unsound to dissociate psychic from interpersonal, social, and psychophysiological difficulties, aspects of which are usually concurrent, though not always obvious.

A trained person.

In search of relief, the individual is apt to involve oneself in a relationship with a friend or authority.

The motivations that prompt such a relationship are disabling symptoms or a realization that one's happiness and productivity are being sabotaged by inner forces that one is neither able to understand nor to control.

Sometimes the consequences of this relationship are registered in a restoration of homeostasis, a product of healing forces liberated by the helping process.

At other times, particularly when attempts are made to handle the sufferer's emotional turmoil in depth, the relationship may become disastrous to both participants.

Dealing most adequately with an emotional problem requires a high degree of skill that may best be acquired through extensive postgraduate training and experience.

Deliberately establishing a professional relationship.

The relationship, the core of the therapeutic process, is deliberately planned and nurtured by the therapist.

Unlike nonprofessional relationships, which are part of the social nature of man, the therapeutic relationship is a collaborative undertaking, started and maintained on a professional level toward specific therapeutic objectives.

More than one therapist (co-therapist, multiple therapists) may work together.

The patient.

An individual in psychotherapy receiving treatment is best called a patient rather than some other designation such as a client.

The therapist may relate capably to more than one patient during a session, as in marital or group therapy.

The object is removing existing symptoms.

A prime goal in therapy is to eliminate the patient's suffering as well as to remove the handicaps imposed by symptoms.

Modifying existing symptoms.

Despite our wish for complete relief, certain circumstances may militate against this objective.

Chief deterrents are inadequate motivation, diminutive ego strength,<sup>1</sup> and limitations in the patient's available time or finances.

These will impose restrictions on the extent of help that can be rendered and make for modification rather than cure of the patient's symptoms.

Retarding existing symptoms.

There are some malignant forms of emotional illness, such as certain fulminating schizophrenic and organic brain disorders, in which psychotherapy, no matter how adroitly applied, serves merely to delay an inevitable deteriorative process.

This palliative effect is eminently worthwhile, however, often helping to preserve the patient's contact with reality.

Mediating disturbed patterns of behavior.

The recognition in recent years that many occupational, educational, marital, interpersonal, and social problems are emotionally inspired has extended the use of psychotherapy into fields hitherto considered provinces of the psychologist, teacher, sociologist, religious leader, and lawmaker.

Realization that the character structure is involved in all emotional illness has broadened the objectives of psychotherapy from mere symptom relief or removal to correction of disturbed interpersonal patterns and relationships.

Promoting positive personality growth and development.

The final use of [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 13 psychotherapy is as a vehicle for personality maturation.

This has introduced a new dimension into the field of psychotherapy—a dimension that deals, on the one hand, with problems of immaturity of the so-called normal person and, on the other, with characterologic difficulties associated with inhibited growth previously considered inaccessible to treatment.

Here psychotherapy aims at a resolution of blocks in psychosocial development in order that the individual may aspire to a more complete creative self-fulfillment, more productive attitudes toward life, and more gratifying relationships with people.

The goals of psychotherapy thus extend from the limited objective of helping to control symptoms to the liberation of the rich resources of the human mind from neurotic obstructions that thwart its purpose and stunt its growth.

IS THE MEDICAL MODEL OUTMODED?

Therapists who venture into rendering essential human services in mental health will inevitably find that they must penetrate into zones alien to the medical model.

Of necessity, they will have to adapt to concepts and interventions that are more related to the social than to the biological sciences.

For example, if one seeks to engage in preventive work or in some forms of short-term therapy, it is necessary to become familiar with the dynamics of social systems and the world of learning and rehabilitative procedures.

The information one already possesses of biological and psychological systems will, of course, be fed into these areas in a way that adapts one to the exigencies with which the therapist must deal.

Unless the therapist is willing to restrict operations to the handling of a narrow band of problems, models of helping will have to be employed that go beyond training in traditional psychotherapeutic education.

Whether we need to call such types of helping psychotherapy is a moot question.

Shall we restrict the word to the treatment of outright emotional illness, or shall we extend it to dealing with behavioral and adjustment problems that are not conspicuously pathological?

A good deal of the confusion about psychotherapy stems from the fact that over the years psychological help has been extended to larger and larger groups of people with increasingly diverse complaints.

These range from physiological disruptions of [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 14 anxiety involving almost any organ, to depressive manifestations, to phobias and compulsions, to psychotic dislocations, to habit disorders, to behavioral aberrations, to interpersonal difficulties, to marital and family disturbances, to educational and work blockages, to sexual malfunctions, to addictions, and to a host of other vexations that can plague human beings. The models most appropriate for the understanding of and the dealing with this plethora of troubles must be spread over a number of fields.

Indeed, in the course of helping the same individual we may have to apply information from medical, sociological, educational, rehabilitative, and other sources.

In a way, the term psychotherapy is a limiting one because literally it means treatment of mental or nervous disorders.

This suggests a restrictive medical model.

To encompass all the troubles for which psychological help is sought under the term

psychotherapy will necessitate a borrowing from theories and stratagems more appropriate to structures other than medical ones, such as sociological or rehabilitative and learning models. Functionally, this is what has happened over the years, often with protests from professionals in disciplines of sociology, anthropology, psychology, and education, who resent encroachment of their domains.

Actually, psychotherapy should not be regarded as a cormorant intent on swallowing diverse disciplines.

It is a body of procedures that overlap techniques used in counseling, social casework, education, and rehabilitation, even though its goals may be different.

There are advantages and disadvantages to the medical model per se; but, on the whole, it has a proven utility for a bulk of problems seen by the psychotherapist.

It is possible that Third Force psychology embracing humanistic aims, as well as the human growth potential movement, may some day provide a viable alternative.

As matters stand today, we have not yet achieved this goal.

Substituting for the concept of "mental health/mental illness," "different modes of coping with life" does not necessarily lead to greater clarification of the many problems with which we have to deal in psychotherapy.

The fact that therapists work within the orbit of the medical model does not mean that they must propel themselves into an absurdly authoritative position, thus

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 15 perpetuating a parent-child relationship.

Authoritativeness is more a product of the personality of the individual therapist than the model that an individual pursues in practice.

Nor does it follow that patients need be depreciated by being labeled "sick" or "mentally ill," thus pinning on them awkward pathological labels.

Nor, if we adopt the medical model, need they be deprived of their freedom or liberty, nor robbed of the option of deciding for themselves their suitability for therapy, nor forced to sacrifice responsibility for their own destiny and the right to their own sense of values, nor prevented from being active participants in their own treatment.

The medical model does not necessarily have to restrict the focus purely on disease.

Expanded psychological growth and development are within its purview, much as physical hygiene is within the scope of good medical practice.

Merely because our present nosological systems are not entirely satisfactory does not sanction the abandonment of diagnosis that is an essential aspect of the medical model.

A proper diagnosis can be helpful to the institution of a rational therapeutic program.

For example, violence may appear as a symptom of a variety of causes.

It may be a simple behavior problem nurtured by situational dislocations.

It may be a habitual egosyntonic display in a psychopathic personality.

It may be a manifestation of failing repressive control in a borderline patient.

It may be the expression of a delusional system in a schizophrenic.

Or it may be a symptom of the manic phase of a bipolar disorder.

Unless a correct diagnosis is established, we may fail miserably in providing effective help.

Thus, elimination of violence due to environmental difficulties can be helpful in simple behavioral problems, but it will usually be ineffective in the other conditions cited.

Anyone who has witnessed the ameliorating effect of neuroleptic medication in schizophrenia and of lithium in manic disorders will attest to the value of a diagnostic survey.

Violence in a borderline case will require special psychotherapeutic management that might not be applicable to other conditions.

Diagnosis can be as important in psychological as in physical problems.

On the other hand, a disadvantage fostered by the medical model is that it concerns itself with

techniques of therapeutic intervention that are sometimes dissociated from the  
[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 16 daily life of the individual.

Diagnosis and pathology, legitimate as they may be in disease areas, are sometimes not applicable to certain behavioral zones.

To classify these as normal and abnormal or as symptoms and defenses neglects considerations of background, culture, and life style, which require a different perspective.

Moreover, while the medical model has in the past sponsored a limited training perspective, it has rarely equipped trainees to deal with many behavioral difficulties that are disruptive to the individual and the community.

It would seem appropriate then in a comprehensive training program to expand the education of psychotherapists toward a wider understanding of the behavioral sciences and toward the use of a broad range of techniques additional to the conventional psychotherapeutic procedures.

Under these circumstances, the therapists would be better equipped to move beyond the boundaries of the medical model toward a more pragmatic commitment to the spectrum of problems challenging them in their practices.

**PSYCHOTHERAPY VERSUS PSYCHOANALYSIS** According to Webster's New Universal Unabridged Dictionary, psychotherapy connotes "the application of the various forms of mental treatment, as hypnosis, suggestion, psychoanalysis, etc., to nervous and mental disorders." This generic definition is, in mental health circles, supplemented by a more specific usage of the term in relation to goals and methods of treatment as contrasted with psychoanalysis.

Psychoanalysis aims at a systematic and total resolution of unconscious conflicts with structural alteration of defenses, and the character organization.

Psychotherapy is less ambitious, reaching for the practical and less arduously achieved goals of resolving some conflicts, modifying others, and even retaining and strengthening certain neurotic defenses that permit individuals to contain their anxiety and to function.

This does not necessarily make psychoanalysis a better kind of treatment than psychotherapy or vice versa.

Some patients fail miserably at one and do quite well with the other.

The key issue is the proper selection of cases for the two different techniques.

Patients exposed to formal analysis, centered around evolvment of a transference neurosis and its resolution [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 17 through interpretation, will require so many qualifications in terms of personality characteristics, motivation, available time, finances, etc.

that they are relatively few in number.

Most patients, on the other hand, will qualify for psychotherapy.

Accepting the fact that psychoanalysis has provided us with concepts and techniques that can lead to the recognition and exposure of conflicts that operate beyond the zone of awareness, what is of concern to clinicians is how useful this information is in treating and resolving emotional problems.

Most students are no longer willing to accept psychological theories on the basis of faith or literary elegance.

Some are relatively unimpressed with both the pronouncements and achievements of psychoanalysis, moving toward psychotherapy with its active approaches directed at symptom relief and problem-solving.

Among the expressed doubts are (1) that psychoanalysis is the best treatment for most problems of an emotional nature; (2) that unconscious conflict is necessarily at the root of all emotional difficulties; (3) that every communication of the patient to the therapist during a session has an unconscious meaning, and that through free association one eventually can reach this repudiated core; (4) that verbal unburdening has a greater impact on the individual than

behavioral solutions to a problem; (5) that supportive and educational interventions are temporary and inevitably lead to greater avoidance and repression; (6) that an adequate cure of a neurosis necessitates its duplication in treatment through the relationship with the therapist (transference); (7) that psychoanalytic theories can be validated through either research or careful clinical inquiry; and (8) that psychoanalysis is the only method through which reconstructive change can be achieved.

The upshot of these questions is that students in progressively larger numbers are doubting the clinical usefulness of psychoanalysis and the need for personal training in psychoanalytic techniques, with the sacrifices of time, energy, and money that disciplined analytic studies would entail.

More insidiously dynamic theory, which is useful in understanding psychopathology, is downgraded and credited with little clinical utility and therefore considered not worthy  
[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 18 of study.

This is unfortunate since insight into anachronistic coping patterns, so essential in reconstructive treatment, will draw upon certain psychoanalytic concepts such as unconscious ideation, repression, resistance, and the survival in the present of needs and defenses rooted in the development past.

Among some psychoanalysts, psychotherapy is sometimes employed to distinguish a wide variety of superficial supportive and reeducative procedures aimed at more conscious mental processes, from psychoanalysis that supports techniques of "depth therapy" focused on the unconscious. While utilizing methodologies for expediency's sake, which may be condoned under certain circumstances, they consider it a baser metal than the pure gold of analysis.

The latter alone of all therapies is targeted at the surviving nucleus of emotional illness in the residual "infantile neurosis." Psychoanalysis, they say, offers itself as a technique that may, in the cases where it can be successfully employed, promote maturity by eliminating the infantile neurosis as a source of emotional pollution.

Psychotherapy is more modest in its goals.

It can help to strengthen the individual's defenses so as to prevent the infantile neurosis from interfering too much with a reality adaptation.

It can also provide guidelines for more competent coping with everyday stress.

In this way the individual is better able to live with the infantile neurosis and to make an adjustment that is no better or worse than the normal individual who possesses some neurotic defenses, but never sees the inside of a psychotherapist's office and still gets along satisfactorily with life and people.

Tarachow's (1963) differentiation of psychoanalysis from psychotherapy is still serviceable.

He considers that in the former "transference, repression, other ego defenses, and resistances are all freely subjected to analysis and resolved," while "psychotherapy, on the other hand, is a selective, limited treatment in which a rearrangement rather than a resolution of these elements is aimed at." Furthermore, as more and more defections from Freudian theory have occurred, classical (orthodox) Freudians have tended to claim priority for the term psychoanalysis and to label any derivative neo-Freudian method as psychotherapy.

Factional quarrels have accordingly developed.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 19 Since psychoanalytic doctrines have permeated into the fiber of mental health practices and theories, the question of where psychoanalysis belongs is an arbitrary one.

A well-trained psychotherapist is usually schooled in analytic doctrines and methods and is capable of applying these as part of the treatment program.

**PSYCHOANALYSIS VERSUS PSYCHOANALYTICALLY ORIENTED PSYCHOTHERAPY** A good deal of strife among psychoanalysts who adhere to classical theory and those who have deviated in their

ideas and methods centers around the word psychoanalysis.

Classical analysts claim, with some justification, that the term is being watered down to include stratagems that are not even remotely related to psychoanalysis.

They insist that psychoanalysis is restricted to a specific mode of treatment, focused on the unconscious, in which the uncovering of repressed childhood conflicts is achieved through the gradual resolution of resistance.

This aim is accomplished through an intensive therapist-patient relationship, insured by frequent treatment sessions (preferably five times weekly) and the employment of the techniques of free association, dream analysis, and the evolvment and “working through” of a “transference neurosis.” The latter embraces the projected distortions in the therapeutic relationship of traumatic experiences with early parental figures.

The transferential development is guaranteed best by the employment of passive, neutral, anonymous, and nondirective attitudes on the part of the therapist.

This tends to mobilize the most repressed components of conflict (the infantile neurosis) and to permit the patient to work through with a new parental image (as embodied in the therapist) more perspicacious attitudes toward authority, toward oneself, and toward one’s own impulses. Resultant is a modification of the severity of the superego, a releasing of the strangulations that characterize the archaic defensive maneuvers of the ego, and a freeing of the constructive elements of the id.

Since there are patients whose problems, motivations, and life circumstances are such that they are unable to respond to the passive methods of psychoanalysis, and since there are therapists who are unable to function in an anonymous, nondirective, nonauthoritative manner, dictated by classical technique, a number of modifications have [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 20 been devised.

Because these serve some patients effectively, the revisions have been proposed as “refinements” and “improvements” over the orthodox procedures.

The hard core of psychoanalysts connected with the Freudian school have challenged such modifications, considering them a reversion to preanalytic methods or a blocking of the true aims of psychoanalysis—which deals intensively with the unconscious and the noninterfering resolution of the transference neurosis.

When we examine the therapeutic tactics of the schools that advocate modifications, we do find a diversion from classical aims and techniques of psychoanalysis— such as the institution of activity in the relationship, a reduction in the number of weekly sessions, a substitution of the focused interview for free association, a consideration in the interview of the present rather than the past, the introduction of adjunctive devices, the proffering of suggestions and directives, coordinate interviews when necessary with other family members, and even the restraining of the development of a transference neurosis.

While Freudian analysts generally do not object to these innovations, and even admit that they may serve some patients better than formal psychoanalysis, they do object strenuously to the labeling of these newer techniques as psychoanalysis.

Neo-Freudians, on the other hand, object to the narrow definition of psychoanalysis defined by the Freudians.

After all, they insist Freud himself defined psychoanalysis as any method that dealt with resistance and transference.

Why circumscribe it to a special kind of orientation?

Freud (1952) persistently contended that no person was a psychoanalyst who did not accept the foundations of the theory of psychoanalysis, namely the existence of unconscious mental processes, the recognition of the theory of repression and resistance, and the importance of sexuality and the Oedipus Complex.

Some authorities now employ the term “psychoanalytically oriented psychotherapy” or “dynamic psychotherapy” to those approaches that accept some, but not all of Freud’s premises.

Others consider a psychoanalytically oriented psychotherapy the modulated utilization of the techniques of psychoanalysis that circumvent or dilute a transference

www.freepsychotherapybooks.org 21 neurosis, and that deal with the understanding in dynamic terms of a limited area of the pathology.

However, because the word psychoanalysis carries with it connotations of “depth” and “thoroughness” and because there is still a status and economic advantage in some parts of the country in being known as a psychoanalyst and in doing psychoanalysis, therapists resist debasing their activities with a term that might be interpreted as second best.

Where psychoanalysis ends and psychoanalytically oriented psychotherapy begins has become a matter of opinion.

One may remember the fruitless struggle of the American Psychoanalytic Association to establish, through its Committee on Evaluation of Psychoanalytic Therapy, a baseline from which to approach the contradistinction between psychoanalysis and psychoanalytic psychotherapy.

The effort bogged down, and a report was issued to the effect that even investigation of possible differences mobilized resistance among the members of the Society (Rangell, 1954).

Orthodox Freudians insist that psychoanalysis presupposes an acceptance of the instinct theory (libido theory) and the primacy of early sexual conflicts.

They contend that deviants of cultural or sociological theoretical persuasions are simply not doing psychoanalysis, even though there is an employment of free association and dreams, a delving into childhood conditionings, an uncovering and resolution of resistance, and a setting up and working through by means of interpretation of a transference neurosis.

This position is gradually being softened with the current interest in cognition and “ego psychology,” which considers behavior too complex to be accounted for solely by innate psychological events.

There is then a tendency to embrace as psychoanalysis only those treatment processes that (1) are executed by trained psychoanalysts, (2) have as their goal the overcoming of resistances to unconscious conflicts, whatever their nature may be, (3) deal with a continuity of experience back to early childhood, and (4) encourage the building up of a transference neurosis and its resolution through interpretation.

Techniques that diverge from these methodologic objectives, and yet employ some of the uncovering www.freepsychotherapybooks.org 22 procedures for delving into the unconscious, such as dreams, free association, and analysis of resistance and transference (which is not to be confused with the more intense transference neurosis) are best classified as

“psychoanalytically oriented psychotherapy,” or “dynamic psychotherapy,” or “exploratory psychotherapy.” OTHER DEFINITIONS OF PSYCHOTHERAPY The comprehensive definition of psychotherapy given above has many advantages.

However, other explanations of the meaning of psychotherapy exist that may be interesting to review.

The sundry published definitions of psychotherapy agree on one point— namely, that psychotherapy constitutes a form of approach to many problems of an emotional nature.

They do not agree on other aspects, such as the techniques employed, the processes included, the goals approximated, or the personnel involved.

Typical definitions are these: 1.

“Psychotherapy is the formal treatment of patients using psychological rather than physical or chemical agents, principally verbal communication.” 2.

“Psychotherapy may be defined as the treatment of emotional and personality problems and disorders by psychological means.” 3.

"We shall define psychotherapy as any type of professional interpersonal situation between a therapist and his patient designed to help the patient to resolve emotional problems." 4.

"Psychotherapy is primarily a transaction between the patient and his therapist." 5.

"Psychotherapy is ...

a developing transaction between two people, one suffering from some type of distress or exhibiting disordered behavior, the other offering amelioration as part of his professional activity." 6.

Psychotherapy consists of "techniques derived from established psychological principles, by persons qualified through training and experience to understand these principles and to apply these techniques with the [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 23 intention of assisting individuals to modify such personal characteristics as feelings, values, attitudes, and behaviors judged to be maladaptive or maladjustive." 7.

"For a very simple realistic definition, one could say that psychotherapy is the utilization of psychological measures in the treatment of sick people." 8.

[Psychotherapy endeavors] "to alter the behavior and change the attitudes of a maladjusted person toward a more constructive outcome." 9.

[Psychotherapy alludes] "to the entire collection of approaches attempting to influence or assist a patient toward more desirable ways of thinking, feeling, and behaving." 10.

"By psychotherapy is meant the use of measures which it is believed will act upon the patient's mind and thereby promote his mental health and aid his adjustment to the particular problems which have disturbed his happiness or adaptation." 11.

"Psychotherapy is a form of treatment in psychiatry in which the psychiatrist, by his scientific thinking and understanding, attempts to change the thinking and feeling of people who are suffering from distorted mental or emotional processes." 12.

Psychotherapy "aims to help the impaired individual by influencing his emotional processes, his evaluation of himself and of others, his evaluation of and his manner of coping with the problems of life.

It may also include changing his environment ... and simultaneously increasing his potentialities of mastery and integration." 13.

"[Psychotherapy includes] a multitude of psychological methods, all having one thing in common—the intent to help a suffering individual through psychological means." 14.

"Psychotherapy is a planned and systematic application of psychological facts and theories to the alleviation of a large variety of human ailments and disturbances, particularly those of psychogenic origin." 15.

"In general, psychotherapy can be defined as the provision by the physician of new life-experiences which can influence the patient in the direction of health."

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 24 16.

"Psychotherapy is the art of combating disease and promoting health by mental influences." 17.

"[Psychotherapy] connotes the use of definitive psychological techniques designed to relieve demonstrable disturbances in psycho-social adjustment." 18.

"Psychotherapy includes all kinds and ways of utilizing psychologic means to achieve beneficial psychobiologic changes." 19.

"Psychotherapy consists of any considered and competent medical endeavor directed toward the improvement of the emotional health of the individual, based upon the understanding of the psychodynamics involved, and of the need of the individual under treatment." 20.

[Psychotherapy is the] "treatment of mental or emotional disorder or of related bodily ills by psychological means." 21.

[Psychotherapy is] "the use of any psychological technique in the treatment of mental disorder or maladjustment... .



The term carries no implication about the seriousness of the disorder ... the duration or intensity of treatment, or the theoretical orientation of the therapist.” 22.

[Psychotherapy is] “an emotional exchange [process] in an interpersonal relationship which accelerates the growth of one or both participants.” 23.

“Psychotherapy is ... a cooperative enterprise for clarifying purposes and modifying attitudes in the direction of greater integrity of personality.” 24.

“Psychotherapy is a certain kind of social relationship between two persons who hold periodic conversations in pursuit of certain goals: namely, the lessening of emotional discomfort and the alteration of various other aspects of client behavior.” 25.

“Psychotherapy may be defined as the treatment of emotional and personality problems and disorders by psychological means... .

Types of psychotherapy fall into two general groups.

One may be described as genetic-dynamic, the other as supportive, suppressive, non-exploratory, or non-specific.” 26.

“Psychotherapy: The generic term for any type of treatment which is based primarily upon verbal or nonverbal communication with the patient, in [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 25 distinction to the use of drugs, surgery, or physical measures, such as electroshock or insulin shock, hydrotherapy, and others.” 27.

“Psychotherapy is a process in which changes in an individual's behavior are achieved as a result of experiences in a relationship with a person trained in understanding behavior.” 28.

“In its classic sense, psychotherapy is defined as the restructuring of the malfunctioning personality.” 29.

“... psychotherapy is a form of help-giving in which a trained, socially sanctioned healer tries to relieve a sufferer's distress by facilitating certain changes in his feelings, attitudes, and behavior, through the performance of certain activities with him, often with the participation of a group.” 30.

“Psychotherapy is a form of treatment of a client by a therapist for disorders of emotional adjustment, with the purpose of bringing about his readjustment so that he will become more comfortable in his emotions, thoughts, and attitudes, and so that his social relationship will be improved.” 31.

Psychotherapy is a “process recurring between two (or more) individuals in which one (the therapist), by virtue of his position and training, seeks systematically to apply psychological knowledge and interventions in an attempt to understand, influence, and ultimately modify the psychic experience, mental function and behavior of the other (the patient).” 32.

[Psychotherapy is] “a procedural arrangement between a therapist and patient in which the therapist, by explicit or implicit contract, attempts to improve, by psychological means, conditions and behavior which interfere with the patient's well-being— and which are ascribed wholly or in part to psychological causes.” 33.

“Psychotherapy: The generic term for any type of treatment that is based primarily upon verbal or nonverbal communication with the patient as distinguished from the use of drugs, surgery, or physical measure such as electroconvulsive treatment.” 34.

“Psychotherapy in its broadest sense is the systematic effort of a person or group to relieve distress or disability by influencing the sufferer's mental state, attitudes and behavior.”

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 26 35.

[Psychotherapy is the] “use of psychological techniques in the treatment of mental disorders. Psychotherapeutic methods are employed by a trained psychotherapist who helps the patient by means of verbal and emotional communication.” 36.

“The science and art of influencing behavior so as to make it (a) more efficient and satisfactory to the individual and (b) more compatible with social norms.” 37.

Psychotherapy is “a process in which a person who wishes to relieve symptoms or resolve problems in living or seeking personal growth enters into an implicit or explicit contract to interact in a prescribed way with a psychotherapist.” 38.

“Psychotherapy is an interpersonal process designed to bring about modifications of feelings, cognitions, attitudes, and behavior which have proven troublesome to the person seeking help from a trained professional.” 39.

“Psychotherapy is a confiding, emotionally charged relationship between a trained, socially sanctioned healer and a sufferer.

The healer seems to relieve the patient’s suffering and disability ...

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New York, Praeger, 1984 [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 30 2 Varieties of Psychotherapy An  
impartial observer of the contemporary psychotherapeutic scene would be forced to admit that  
present-day theories about how people become emotionally ill are exceeded in number only by the

available remedies for making them well again.

The individual seeking help for an emotional problem, the student in quest of training in psychotherapy, the investigator searching for answers to the puzzling problems of process and outcome research—all are confronted with an imponderable dilemma of choice.

Indeed, the field of psychotherapy has become a vast supermarket where consumers may shop for stores of interventions that can satisfy the most capricious tastes.

These range from mundane traditional interview methods to more exotic excursions into marathons, mantras, and massage.

There is no need to agonize over reasons for the predicament that psychotherapy finds itself in today.

Empirical studies have shed little light on how treatment processes operate and when they are most effective.

Emotional problems are difficult to treat.

Welltrained therapists are hard to find.

Good therapy is expensive and, even when implemented, encounters great resistance in some patients.

Professionals are constantly devising unique approaches that coordinate with their personalities, philosophies, and styles of relating.

Often they become so convinced of the virtues of their systems that they incorporate in it the entire world of psychopathology.

Vast pockets of need remain unfulfilled and into these areas pour the less trained, less experienced practitioners, as well as a host of charlatans, drum beaters, and mental health evangelists whose enthusiasm for innovation is matched by their ignorance of things psychological.

Lacking the discipline of experience, they promise rapid cures with a murky collage of half-truths, naive opinions, and outright distortions.

The average person is thus in a quandary when exploring the field in the hope of finding an adequate resource for personal emotional difficulties.

Inquiries will generally [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 31 bring as many opinions about preferred courses of action as there are people to consult.

The family physician may admonish to “slow down and relax,” supplementing this advice with such medications as tranquilizers, vitamins, energizers, hypnotics, or placebos.

A minister may enjoin more assiduous devotion to religion and faith that God will show the path to follow.

A lawyer may counsel a long vacation in order to get away from the press of business responsibilities.

Friends may urge one to leave a job, or to divorce a mate, or to find an absorbing hobby, or to take up meditation, or to see a chiropractor, or to read selfhelp books.

If the average person decides to get professional help, there will be no less confounding advice, particularly if a resident of a large city where there are many representative types of therapy.

If such is the case, what should he or she do?

Should he choose a therapist who practices psychoanalysis in spite of the current unfavorable publicity regarding its values?

Should she go to a behavior therapist even though some of her friends have warned her that symptom alleviation will not get down to the bedrock of her troubles?

What about group therapy, or marital therapy, or Scientology, or primal scream therapy, or transcendental meditation, or reality therapy, or any of the scores of other approaches that he or she has heard or read about?

Perhaps his body needs balancing with psychotropic drugs, or maybe she can gain control over her physiological responses with biofeedback, or is hypnosis or strategic therapy the answer? It is confusing, even for the average professional person, to view the multiform methods of treatment that are promulgated for emotional illness and to listen to the exaggerated claims of their devotees and the violent denunciations of their critics.

Equally puzzling is the fact that published statistical data, tabulating percentages of cure and improvement and failure, reveal that results obtained by various methods of treatment are strikingly similar.

Indeed, people seem to be benefited by all kinds of therapy—by those that have a scientific stamp of approval as well as those on the fringe of quackery.

How can we explain such inconsistencies?

How can we, for instance, explain the failure of people to respond to years of intensive and skillful psychoanalysis who, [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 32 succumbing to the blandishments of untutored charlatans, lose their symptoms in a few weeks and take up life with renewed vigor?

Are there differences in the quality of improvement with varied forms of treatment, or do certain kinds of problems respond better to specific types of treatment methods?

Are there differences in the permanency of the results obtained by the respective procedures?

Can we say that there is a “best” treatment for neurosis?

Added to these confusing issues is the fact that in some instances an individual suffering from a severe emotional problem may experience considerable relief, and even a so-called cure, without the formality of having received any kind of treatment whatsoever.

How much of a spontaneous reparative element, therefore, is present during the course of any therapy?

Answers to these questions will be considered in the chapters that follow.

To describe the countless permutations of therapy that exist today would be an almost impossible task since all practitioners evolve a unique format influenced by their background and character structure.

Corsini (1981) lists no less than 250 different kinds of psychotherapy, and Herink (1980) describes as many in his book.

Nevertheless, there are identifiable pools within which practitioners flexibly float.

It may be useful to classify the chief varieties of psychotherapy in relation, first, to the objectives they pursue (Table 2-1) and, second, to the fields from which they derive their substance (Table 2-2).

The varieties of psychotherapy may conveniently be divided into three main groupings: supportive therapy, reeducative therapy, and reconstructive therapy.

The distinctions among these as they relate to objectives and the more common approaches are outlined in Table 2-1.

The many psychotherapies in supportive, reeducative, and reconstructive categories become more diverse from year to year as an increasing number of professionals enter the psychotherapeutic field, introducing into it their unique technical modifications.

Schools of psychotherapy have crystallized around the various approaches, each of which has its zealous disciples as well as its staunch critics.

Each lays claims to multifarious successes and admits to some failures.

Actually, radical divergencies in technique are more apparent than real, many distinctions vanishing as soon as semantic differences are resolved (Rosenzweig, 1936; Watson, 1940).

Some [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 33 differences, however, do exist and will be considered in detail in the next chapters.

Schools of psychotherapy are characterized by the aggregation under one banner of a host of

professionals, trained to respect one or another of the pioneers in the biological, psychological, social, and philosophic fields and espousing, often fanatically, a general point of view and basic postulates, which, upon close examination, reflect wide personal differences in interpretation and application.

Indeed, the variations and shades of divergence between members of the same school are often greater than between members of rival schools.

Professionals develop individual styles of doing psychotherapy, as artists evolve unique ways of painting.

They then become wedded to these styles, perfect them, find them effective to their satisfaction, and then sometimes promote them as the best of all treatments.

A general theory is fabricated sooner or later to provide a rationale which, more or less, draws from their personal lives and intrapsychic experiences.

If they are sufficiently persuasive, they may attract a host of followers who are searching for answers to the dilemmas of treatment.

And if they possess an abundance of charisma, they may even initiate a revolution in the form of a new school, which lasts for a period until sufficient failures in therapy accumulate to convince therapists that the psychiatric messiah has not yet arrived.

When we distill out the important essences of all of the existing therapies, the differences are not as profound as they seem on the surface, although several classes of therapies do exist with distinctive goals and operational modes.

While all schools of psychotherapy explicitly avow an empirical dedication rooted in observation and experiment, some find their pith in ontologic and even theologic-like precepts. Difference is registered in the degree of allegiance to biological as contrasted with psychological and sociological principles.

On the one hand, there are behavioristic schools that orient themselves around a stimulus-response psychology or social learning model, focused on objective observation, accepting psychic phenomena merely as intervening variables.

There are hormic schools, which include the dynamic systems, that [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 34 deny that behavior is explicable in purely mechanistic or physiochemical terms but rather requires the consideration of contents of mind as objects of awareness.

The various approaches to emotional illness involve many affiliated fields (see Table 2-2).

In the main, the following avenues are pursued: 1.

Organic-physiological regimens that, rooted in the soil of biology, are strictly speaking not psychotherapeutic.

2.

Behavioral-conditioning techniques, psychologically oriented, that deal with the effects of conflict and depend on relearning and the retraining of responses without concerning themselves too much with conflictual sources.

3.

Supportive relationship tactics, sociologically based, that draw upon factors that are operative in any "helping process." 4.

Dynamic-reconstructive methodologies, psychologically and sociologically inspired, that focus on insight, delineating the origins of conflict, its effect on intrapsychic functioning, its manifestations in character structure, and its impact on problem solving and other vital behavioral activities.

5.

Philosophic-persuasive ideologies aimed at alteration of values and meaning systems, toward the development of more adaptive ways of approaching life's burdens.

In actual practice there is often a fusion of approaches, although the followers of specific schools may be loathe to admit this.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 35 3 Extratherapeutic (non-specific) Healing Aids: I.

The "Spontaneous" Cure Psychotherapy attempts to alleviate emotional suffering and to enhance personality adjustment through planned psychological interventions.

It is by no means the only medium through which such benefits may be achieved.

Indeed, operative constantly on the individual are a variety of forces that serve to ameliorate neurotic symptoms and, even, under fortunate circumstances, to sponsor personality growth.

These forces work so subtly that they are usually overlooked.

They are an inherent and inevitable part of the living experience.

Because they exert their effect during psychotherapy, as well as outside of it, it is difficult to know the proportion of benefit the individual has derived from these congenial non-specific agents and how much that person has gained from the specific psychotherapeutic maneuvers themselves.

More importantly, if we are able to define the forces that nurture emotional health apart from psychotherapy, we may be able to incorporate these into our therapeutic program, or at least prevent ourselves from inhibiting them.

Among the coincidental factors associated with psychological change are (1) "spontaneous" remissions or cures, and (2) influences that automatically issue out of any "helping" situation.

The first of these will be considered in this chapter, the second in Chapter 4.

Spontaneous cures probably occur more frequently than we are wont to admit, for both physical and emotional difficulties are associated with periods of exacerbation and periods of remission and, often, without apparent cause, vanish of their own accord (Stevenson, 1961).

Even the most pernicious forms of psychoses show a tendency toward spontaneous remission, and any institutional psychiatrist, even before the era of psychotropic drugs, is acquainted with the patient who, having spent years in the hinterlands of a chronic mental ward, suddenly returns to the world of reality and expresses eagerness to take up his place as a member of society (Wolberg, 1944).

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 36 The circumstances surrounding such a "spontaneous cure" are so nebulous that it is customary to look on the phenomenon, when it occurs, as a sort of fortuitous development.

Were we, however, able to peer into the complex mechanisms of the psyche, we might be able to discern definite laws of cause and effect relating to the spontaneous recovery.

It is unfortunate that so much emphasis has been placed on the evil consequences of illness that we are prone to concern ourselves chiefly with destructive pathologic effects and forget that constructive regenerative influences may be coincidentally present.

It is as if in an infection, we were to concentrate on morbid elements of the disease without considering such protective manifestations as the antibody defenses and the activities of the reticuloendothelial system.

Any somatic disease whips into action reparative mechanisms in the direction of health.

Cases of unquestionable cancer that spontaneously resolved themselves without benefit of any treatment have been authenticated (Medical Tribune, 1963).

This indicates that the individual has internal healing forces that are capable of altering an even presumably fatal illness.

A mental or emotional disorder, too, will set into action defensive devices to restore the individual to homeostatic equilibrium; indeed, a study of the symptoms of emotional illness, whether the difficulty be a mild behavior problem or a pernicious form of schizophrenia, will always demonstrate a fusion of dynamisms representative both of the conflict and of the

defenses the organism elaborates in an attempt at cure.

The exact mechanisms involved in spontaneous remissions or cures are not exactly known, but a number of operative factors, singly or in combination, suggest themselves.

First, life circumstances may change and open up opportunities for gratification of important but vitiated needs, normal and neurotic.

Thus a man with a passive personality organization may, in fighting against passive impulses, elevate himself to a high political post that enables him to exercise secret cravings for power.

The expression of these may neutralize inner fears of weakness and lead to feelings of well-being.

A masochistic individual may appease a strong sense of guilt through the incidence of a physical illness, [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 37 an accident, or a catastrophic life happening.

A man with strong dependency strivings may flounder around helplessly until he chances onto a stronger individual, an alliance with whom infuses him with strength and vitality.

A woman with schizoid tendencies may be afforded, through advantageous circumstances, an acceptable opportunity to detach herself in her interpersonal relationships.

A shift in the current situation may, therefore, work in the interest of promoting psychic stability.

Second, provocative stress sources may disappear as a result of the removal of the initiating environmental irritant, or because individuals extricate themselves from it.

A child, selected by a classroom bully as a focus for sadistic attacks will, in all probability, lose tensions and fear with the forceful removal from the class of the disturbed assailant.

A young dependent adult overprotected by a neurotic parent may decide to move away from home and be rewarded with an expanding sense of independence.

A man, promoted into a highly competitive position that sponsors fears of failure, may, by returning to his previous, more mediocre job, overcome his anxiety and depression.

A leave of absence or a vacation may, in a similar manner, remove a person from a disturbing life situation, helping in self stabilization.

Third, crumbling or shattered defenses, whose failure promotes adaptive collapse, may be restored to their original strength, or be reinforced by new, more adequate, and less disabling defenses.

The return of a sense of mastery in the course of buttressing failing defenses, will help to restore the individual to functional equilibrium.

One way this is accomplished is by consolidating the mechanism of repression, sealing off disturbing conflicts that have managed to get out of control.

For instance, a detached individual who has remained celibate all his life due to deep fears of sexuality, falls in love with a young woman.

Her demands for reciprocal affection excite sexual fantasies and impulses that frighten him.

Anxiety and psychosomatic symptoms precipitately develop.

The man then discovers that he is completely impotent and that his interest in the young woman abates and dies.

Restored repression of his sexual desires, with a return to isolation from women, brings about a recovery from symptoms.

Sometimes, however, the defenses of the individual undergo modification in the direction of greater adaptability to inner demands [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 38 and external pressures.

More constructive ways of dealing with current difficulties may be evolved.

The means by which this is accomplished will vary with the circumstances and with the



opportunities at one's disposal.

Not only may the old neurotic balances of power be restored, but surprisingly, without design, enduring personality changes may develop.

No matter how victimized by past unfortunate conditionings, each individual is always capable of some determinate learning under propitious circumstances.

The expectation of failure, the misinterpretations of the reality situation, the transferential projections, the repetitive compulsive reenactments of self-defeating behavior will tend, of course, to sabotage successes.

Inferential discoveries, nevertheless, are going on around the matrix of the neurotic responses, reconditioning at least some neurotically structured reactions.

Instances of reconditioning in even severe neurotic and psychotic disorders have been recorded in persons who have by chance come under the guidance of a well-disposed and intelligent authority or group or who have found a congenial environment that is different from their habitual milieu.

These changes are not fortuitous; they follow certain laws of learning.

Where the individual finds an environment that does not actively repeat the punitive, frustrating conditions of the past; where it provides a modicum of security and some means of gratifying vital psychological needs; where it gives a feeling of knowing what is expected of each person and some ability of living up to these expectations; where it nurtures in the individual a sense of belonging and of being wanted; where it provides opportunities to expand capacities and skills—there will be optimal conditions for learning.

Obviously, those individuals whose emotional problems have undergone minimal structuralization in the form of neurotic and character disorders will have the best chances to learn in a favorable environment.

Where extensive organization has occurred, conflictual strivings will tend to be perpetuated even in a good setting.

There still may be spontaneous periods of improvement, particularly where the environment absorbs and deals in a tolerant and constructive way with the individual's neurotic impulses and behavior without excessive punishment or withdrawal of love.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 39 Alleviation of symptoms may follow change in one's philosophies and altered ways of looking at things.

For example, some people adapting to difficult life situations from which there is no escape may insulate themselves by making a virtue out of suffering or by maintaining a stoical attitude accepting their plight as inevitable, and their endurance a test of competence.

Some may even make capital out of personal disasters and secure equanimity through martyrdom. Many of the present-day mystical philosophies stress the unimportance of adversity and external hardship, focusing the individual's attention on establishing contact with the inner self, the essence of being.

In this way the individual detaches from suffering and seeks joy in meditation and self-contemplation.

An individual may resort to other devices in the belief that one has control of one's destiny, for instance, through reliance on astrology.

By classifying oneself in a special category, subject to designated occult influences of the stars, security is gained through regulation of one's life according to a dictated design.

The patient may also evolve the tactic of pushing out of mind sensations of pain, substituting for them thoughts of health and pleasure, hereby eliminating customary preoccupations with suffering.

Through proper thinking habits, like "looking at the cheery side of life," "counting one's blessings," and engaging in thoughts of happiness and health, attempts are made to sidestep

calamities and illness.

Troubles may be rationalized by assuming that one is not alone in strife, that there are others more disabled, more unfortunate.

Striving for success and perfection are false objectives that can bring only exhaustion and disappointment.

It is better to scale down ambitions and content oneself with modest goals that are within the range of immediate fulfillment.

Some people make an inventory of their good qualities and recognize that there is much for them to be thankful.

They may rule themselves to live in the spirit of forbearance, tolerance, sympathy, and altruism, reviving, in the course, religious convictions.

By turning to religion, some are able to help themselves tolerate the destructive effects of their neuroses.

In the hands of a Power stronger than a human being who can lead one to paths of safety and glory, one can be reassured.

In union with God, one does not have to struggle alone; one may be helped to endure travail, to conquer evil thoughts [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 40 and impulses, and to find confidence and strength in living.

In addition to philosophic and religious aids, the individual may indulge other modes of gaining relief.

Remediable, stressful environmental distortions that sustain and exaggerate tension must be rectified.

Moving to a residential area, for instance, that is not so riddled with quarreling and disturbed neighbors may eliminate one source of irritation.

Or, some difficulties may spontaneously cease to exist, as the case where a destructive supervisor or foreman is replaced with a genial one, or a hostile member of one's family may regain control over explosive emotions or takes up residence in another city.

The patient may also find relief from symptomatic preoccupation by externalizing interests in arts, crafts, hobbies, music, games, dancing, physical exercise, sports, and recreation.

Diverting energies into useful channels of activity may thus have a salubrious effect.

Engagement in social activities, joining clubs and other groups, in which there is a sharing of experiences can be most constructive.

The patient may also gain relief from some personal problems by running away from trouble sources.

A woman may rightfully decide that her difficulty is stirred by a destructive home environment, or an impossible work situation, or a relationship problem with which she cannot cope.

She may consequently remove herself bodily from her predicament by going on a long vacation or permanently rupturing her contacts.

Thus, she may seek a divorce, quit her job, or abandon her boyfriend; and these disengagements may, if auxiliary problems do not intervene, temporarily at least, relieve her distress.

She may act more destructively to escape her suffering by indulging in excessive drinking that deadens her sensations, take tranquilizers that ameliorate them, or take sleeping potions that submerge them.

Some people seek peace of mind by providing what they consider rational answers to explain what they are experiencing.

They may do this through self-probing, examining their reactions, questioning their motives, or cogitating on their fantasies.

Under situations of stress individuals are particularly disposed to such self-analysis.

Adolescents particularly attempt to resolve warring elements within themselves through

selfwww.freepsychotherapybooks.org 41 searching.

Such efforts are not always successful since sources of difficulties may be hidden from their rationalizing probes.

The capacity for insight varies.

The desire to reach an understanding of one's turmoil will invoke different degrees of logical compromise.

Education helps especially through proper readings and personal therapy.

Those individuals who have evolved reasonable understanding are most capable of illuminating dark recesses within themselves.

Spontaneously, a recovery of forgotten important memories, a relating of present-day patterns to past experiences, a recognition of inexorable and self-defeating demands they make on themselves may occur.

Ultimately, new solutions for old problems may emerge.

Sometimes such self-understanding is mobilized by single interviews with a psychotherapist with no further professional help.

Malan et al.

(1975) at the Tavistock Clinic made a study of untreated neurotic patients who had exposed themselves to one initial interview and had been seen by a therapist no more than twice in their lives.

On follow-up, 51 percent were found to have improved symptomatically, and 24 percent were judged to have improved dynamically, some even to the point of recovery.

This is a significant number.

Considering the fact that the work was done at the clinic by established psychoanalysts who utilized rigid standards of measurement, we, therefore, have evidence that even single encounters may prove of great benefit to some patients.

Whatever activity was mobilized during the interview, the patients continued spontaneously on their own to work through residues of their problems.

It will be seen from the above that the so-called spontaneous remission or cure does not take place in a vacuum.

It results from the operation of many forces that exert a healing influence.

Even though these processes are not deliberately designed, but obtrude themselves through what appears to be chance, each individual takes advantage of some healthful elements in the environment and within the self to reduce suffering and pain.

As a general rule, spontaneous remissions or cures are of a palliative nature, with no great change coming about in the existing mental mechanisms and intrapsychic apparatus.

In a few instances, however, rather surprising spontaneous structural alterations develop www.freepsychotherapybooks.org 42 in the personality organization itself.

Most recalcitrant to spontaneous resolve are deepseated neuroses linked to repression of infantile anxieties, which are so split off from access to awareness that they do not lend themselves to dissolution.

Nevertheless, even here they may be more readily held at bay by a spontaneous strengthening of repressive forces, or improved by avoiding situations that tend to activate them.

Focal conflicts may in this way be isolated sufficiently to enable the individual to function satisfactorily without being aware of their content or even existence.

It will be recognized that some of the undercurrent processes in spontaneous cure are similar to those in psychotherapy.

Whereas in spontaneous cure the individual inadvertently arrives at conditions in which it is possible to work out problems, in psychotherapy, circumstances conducive to relief or recovery are actively manipulated.

Many opportunities exist in the environment that would lend themselves to a corrective influence on the patient.

Unfortunately, the average person does not often take advantage of them because there is no awareness of their potential or because their meaning is distorted, or because they pose a threat to neurotic defenses.

During psychotherapy, the resistances of the individual are handled so that the most can be made of the opportunities encountered.

In contrast to the fortuitous nature of the spontaneous development, psychotherapy is a planned effort to promote symptom alleviation and constructive learning both within and outside of the psychotherapeutic setting.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 43 4 Extratherapeutic (non-specific) Healing Aids: II.

The "Helping" Situation Turning to another human being for help is an inevitable consequence of a shattered sense of mastery.

It represents acknowledgment of the inability to cope with difficulties through one's own resources.

Contact with a trusted individual can inspire rapid relief from tension and even restore adaptive functioning.

Physicians, ministers, lawyers, teachers, psychiatrists, psychologists, social workers, and even considerate friends may serve as elegant resources for a helping relationship.

The effect, while therapeutic, is epiphenomenal.

It reinforces the impact of formal psychotherapy.

It is a vital, indeed inescapable, constituent of the therapeutic gestalt.

Most prominent among the liberated forces are the placebo effect, relationship dimension, emotional catharsis, suggestion, and group dynamics.

**THE PLACEBO EFFECT** If a patient and therapist are both firmly convinced that eating oatmeal will cure the patient's symptoms, it is likely that relief will be forthcoming as long as the proper rituals have been organized around the taking of the cereal, and the patient maintains faith in its efficacy.

If as a consequence of the conviction of being helped the patient dispels tension and anxiety sufficiently to restore to oneself a sense of control, he or she will possibly return to habitual routines and sooner or later reach an optimal level of functioning.

Moreover, if the problems are not too serious and one is fortunate enough to live in a constructive environment, particularly in a productive relationship with some other human being, he may even undergo deeper personality changes.

The placebo phenomenon responsible for these permutations is usually minimized by therapists who have been seduced by a belief in the scientific validity of their systems.

This is not to minimize the impact of a rational psychotherapy, but it is essential to give credit for part of the benefit to adventitious elements other than technical interventions.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 44 The placebo has been called a "nuisance variable," and yet it historically has served as an important, albeit often unacknowledged, aid in medicine (Shapiro, 1964).

Indeed, the prescription of inert substances was a standard part of medical practice for many years, being embodied in the tradition of giving pink sugar pills and injecting sterile water for their suggestive effects.

These effects may be quite powerful, even to the point where direct or suggestive statements regarding a presumed drug action induce a psychological reaction diametrically opposed to the true pharmacological effects of the drug (Wolf S, 1950).

Research explorations on medicaments in the form of double-blind studies pay respect to the placebo factor, recognizing that a mental influence may contaminate the impartial appraisal of

a substance.

There is ample experimental and clinical evidence to substantiate this view.

Laboratory reports indicate that pain thresholds are heightened and pain tolerance increased by placebos.

Beecher (1970) has stated that placebos relieve pain one-half to two-thirds as effectively as optimal doses of morphine.

Apart from the considerable relief of pain, various physical ailments may be helped (Lasagna et al, 1954; Wolf & Pinsky, 1954).

The placebo effect probably accounts for the fact that most patients who visit a general medical physician improve with almost any symptomatic treatment.

Hypodermic injections of distilled water accompanied by statements that a new remedy for bleeding ulcers was being administered helped 70 percent of patients over a long-term period (Volgyesi, 1954).

A study of vaccines for the common cold revealed that while 55 percent of persons injected with the vaccine showed a reduction in the number of yearly colds, fully 61 percent was recorded in a control group who had received nothing more than harmless injections of saline solution (Diehl et al, 1940).

Patients with chronic headaches experienced relief with placebos in 60 percent of cases (Jellinek, 1946), and the same percentage of patients noted a reduction of the frequency of herpes simplex with placebo therapy alone (Conant, 1974).

Beecher (1955), evaluating 15 studies, including 7 of his own, involving a total of 1082 patients, found that placebos had significant effectiveness of 35.2 percent  $\pm$  2.2 percent. He also found that the effect was most powerful when stress, discomfort, and pain were greatest.

His remark that “many a drug has been extolled on the basis of clinical impression when the only power it had was that of a placebo” has been [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 45 repeatedly confirmed.

The impact on the individual of a placebo is all the more astonishing when we consider that through its suggestive influence it can produce objective physiological changes (Abbot et al, 1952).

Thus placebos given to patients with duodenal ulcers produced the healing of ulcers in 65 percent of the cases (Hollander & Harlan, 1973).

A significant number of side effects may even be credited to an inert material (Feather & Rhoads, 1972; Rickels et al, 1965).

Toxic phenomena have been induced by substances to which the individual could not possibly have been sensitive, solely on the basis of the individual's conviction that he could not help responding in an untoward way.

Sugar pills, for example, have produced weakness, palpitation, nausea, maculopapular dermatitis, epigastric pain, watery diarrhea, urticaria, and angioneurotic edema (Wolf & Pinsky, 1954).

According to S.

Wolf (1950), physiological effects are brought about by a placebo through “objective changes at the end organ which may exceed those attributable to potent pharmacologic action.” In patients with anxiety placebos have produced physiological responses similar to those of corticotropin (ACTH) in normal patients.

How placebos work is not known.

Faith in a minister, guru, charismatic leader, shaman, physician, spiritualist, shrine, acupuncture, or scientific method—irrespective of on whom or what the individual pins one's hope—this can promote high levels of wellbeing and healing that on the surface sound

miraculous.

But modern research points out that the miracle is chemical and not spiritual.

Concentrated in the hypothalamic-pituitary region, opiatelike pentapeptides, the enkephalins and endorphins, are apparently liberated through a placebo effect producing analgesia that modulates pain and activates drive and reward systems in the brain.

These endogenous substances also interact with other neurotransmitter systems influencing neuroendocrine functions that sponsor important psychoadaptive effects.

The healing value of faith is very well documented in popular and scientific literature.

Its operation may make the difference between death and survival in serious accidents, illnesses and surgical operations.

When individuals with tension, anxiety, and other neurotic symptoms take recourse

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 46 to medicaments in order to subdue their suffering, the effectiveness of the drug is greatly enhanced if there is a conviction that it will work.

In double-blind studies with tranquilizers, a sizable number of subjects report marked relief of their symptoms with placebos that are packaged similar to the tranquilizers under test because the deceit excites confidence in the inactive substance.

However, even when the patients are told that the material given them is inert, they will still respond positively (Park & Covi, 1965), probably on the basis of faith, hope, and expectancy.

In many parts of the world charms and amulets are still worn for their placebo effect.

These objects work their wonders among the less educated by dispelling evil spirits, invoking benevolent deities, and generally protecting the individual against misfortune and disease.

Counterparts of the ancient mandrake root and unicorn horn are still sold to quell ague, to staunch blood, to cure headaches, to facilitate childbirth, and generally to promote health and well-being.

In some areas the sale of herbs and curious nostrums flourishes, such as in Chinese herb shops in Hong Kong where dried sea horses are prescribed for chest congestion, ground pearl powder for pimply complexions, rhinoceros horn for heart ailments and impotence, tiger bones for rheumatism, and fungus from a coffin growing near the corpse's mouth for tuberculosis.

Among the better educated, more refined and highly personalized tokens are coveted to bring good luck and to allay misfortune.

Confidence in the charm or token is, as in drug placebos, the essential ingredient.

The conviction that a powerful protective agent is at work to support one's claims on life, and to protect against dangers, is sufficient to calm anxiety, to lessen tension, to restore physiological homeostasis, and to promote self-confidence and assertiveness.

A more accurate perception of possibilities of action leads to mobilization of effective resources and to more constructive problem solving.

Shattered repressions may be restored, and the individual brought back to a functional personality equilibrium.

What holds true for drugs probably holds true for systems of psychotherapy (Rosenthal & Frank, 1956; Frank, 1961; Friedman, 1963; Campbell & Rosenbaum, 1967).

Individuals project into these systems their expectations of cure, and they may react quite remarkably to techniques and agencies that they enshrine with powers that the objects [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 47 may not really possess.

This is suggested by studies of psychiatric outpatients where the sole therapy was oral placebos and in whom a significant (55 percent) improvement rate was actually recorded (Gliedman et al, 1958).

Relief of discomfort and symptomatic amelioration have lasted for years solely on the basis of an expectancy of help (Frank et al, 1959).

There is a definite correlation between conviction and cure.

Thus sitting in the waiting room of an outpatient clinic has resulted in signal improvement in patients (Frank et al, 1963).

Some time ago I was consulted by a man whose sole complaint was that he smoked too much. He wanted me to help him to control his craving so that he could forever eliminate tobacco from his life.

He had, he confessed, at one time been a schizophrenic but was now completely cured, having been treated by a therapist who employed a unique method.

Twice weekly the patient would appear for his sessions.

He would strip to nudity and lie on a couch near an open window.

Verbalizations were forbidden.

Instead he was ordered to bear, Spartan-like, the discomfort of cold and drafts in complete silence.

The therapist, during this ordeal, sat near the patient, huddled in a blanket, recording the patient's muscle movements and general bodily reactions.

Interpretations were not offered.

The object of his tribulation was to raise the patient's frustration tolerance by exposing him to external suffering so severe as to induce a psychological callus formation that would ultimately encapsulate his weak and sensitive ego.

The treatment, while uncomfortable, was, according to the judgment of the patient, eminently successful.

In fact, the therapist was the only one of a series of four psychiatrists who had given him any help whatsoever.

The previous therapists, applying the conventional psychotherapeutic procedures, had failed completely.

While many elements probably entered into the helping process, the fact that a friend of the patient had praised the therapist to the skies had undoubtedly mobilized the patient's expectations of hope and had convinced him that he would be cured.

Therapies that in the mind of the patient possess esoteric or mystical qualities are most likely to induce a placebo effect.

Thus, hypnosis and psychoanalysis, charged as they are with supposed meta-psychological powers, are apt to instill in the patient a feeling of [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 48 magical influence.

This instrumentality, although an artifact, may promote confidence in the treatment method.

By the same token, a negative placebo effect may be obtained, if, by virtue of what is imagined will take place, the patient has lack of faith in, or fears being hurt by, a special kind of therapy.

The chances are then that this attitude will inhibit the effectiveness of treatment.

Medicaments, amulets, and techniques are potent placebo sources, but not nearly so powerful as a human authority with an inspirational flair.

It is here that the quack has an advantage over the ethical practitioner, who, fettered by factual propriety, is unable to flaunt his or her wizardry, and becomes for the patient a vehicle of hope.

In their helpless desperation, many victims of illness reach for a magical helper.

Medicine man, shaman, priest, physician, or psychotherapist, the patient importunes the agency for relief earnestly engaging in avid prayer at the promise of divine reward, and while eagerly incorporating any worthy theological, philosophic, or psychological formulations as a pledge of trust.

The placebo effect of prayer, and of conversion to a system of religion, philosophy, or psychology, are explicable in terms of the bounties the supplicant expects to receive as a

result of his dedication.

In psychotherapy we are mindful of the phenomenon of patients who regurgitate the prophetic revelations of their therapists concerning the cause, meaning, and future of their illness.

How much this “insight” is an invocation for support and how much it constitutes a true discernment of the underlying difficulty must carefully be evaluated.

Unfortunately, the therapist is not the most objective and unbiased referee for this task.

Interestingly, a theoretical system may act as a placebo for the therapist.

If there is faith in its verity, the therapist will approach the patient with dogged

confidence, and will undoubtedly unearth in the patient the exact constituents that compose his or her theoretical scheme.

In this way a placebo feedback becomes operative between therapist and patient.

The conviction on the part of the patient that the therapist knows what is wrong with him or her and has ways that will bring health encourages restoration of mastery.

This is irrespective of how valid the therapist's system may be.

Certainly with the abundant theories rampant about health and disease, and the varied psychotherapeutic [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 49 techniques that are being practiced, we may rightfully be persuaded that some systems are at least fanciful if not actually wrong.

But right or wrong, a patient will be benefited by all systems if there is enough confidence in the therapist.

This is probably because the enthusiasm and firm belief of the therapist has aroused the patient's expectations of cure.

On the other hand, if the therapist has doubts about the validity of the therapeutic system, it is possible to communicate pessimism, insecurity, and helplessness, nonverbally if not verbally, and thus the element of faith that can kindle hope will be extinguished.

In this way the therapist will rob the patient of an important element in the healing process.

Many observers have commented on the powerful factor of confidence in the treatment method as an ingredient in cure.

Among native groups who are imbedded in magic and witchcraft, the most adroit application of dynamic psychotherapy produces meager results probably because it does not accord with their belief systems.

Where the victim of an emotional problem is convinced that the suffering is the consequence of a curse or other evil magic from an offended spirit, the adroit exorcism of the force presumably responsible for the bewitchery will, in a surprising number of instances, bring about the improvement or cure of even long-standing neurotic and psychotic illnesses.

Assignment of the illness to its presumable responsible determinants tends to alleviate fear of the unknown.

It matters little whether the identified source is factual or not.

As long as the patient believes it, the catastrophic sense of helplessness is palliated.

The healer may diagnose the condition as due to infestation with evil spirits, or regression to an anachronistic psychosexual level of development, or operations of unconscious conflict, or a disrupted biochemical balance within the body.

If the patient accepts the proffered “insight,” the very focusing on a presumed source of mischief opens up new possibilities of action.

The helping agency, having delivered an explanation, proceeds to deal with the matrix of trouble.

Rituals to exorcise offended spirits or to destroy them, free associations to liberate unconscious foci of conflict, medicaments to reinstate the biochemical balances, conditioning to restore the individual to healthy habit patterns— whatever the theory of etiology—pertinent measures are executed to resolve the problem source.



If the patient has faith in the healer, the virtue of the methods will be endorsed, [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 50 whether these be anomalous or scientific.

Thus an individual, impressed by a practitioner dedicated to meditation, will do better with relaxing exercises than with insight therapy prosecuted by a dynamically schooled therapist in whom there is little confidence.

The feeling that one is being helped, that a curative agency is at work, subdues anxiety and diverts the individual from self-defeating defenses, toward more effective dealings with existing problem situations.

The disruptive physiological effects of anxiety are brought under control, and restoration of self-confidence and assertiveness ensues.

Repressions are restored, and psychological homeostasis is reestablished.

Placebo effects are usually, but not always, temporary.

In many instances the improvement acts, as in the "spontaneous cure," as a vehicle for the reorientation of the individual in the total adjustment.

Generally, the resulting security feeling leads to a heightened capacity to handle challenging relationships with people.

The placebo may thus act as a basis for reorganization of attitudes, which may then become reinforced and entrenched in a favorable milieu.

The operation of the placebo element helps to explain why patients may be helped by psychotherapies that seem scientifically unsound, provided that there is faith and trust in their validity and power.

**THE RELATIONSHIP DIMENSION** Every helping situation is characterized by a special kind of relationship that develops between the authority and subject.

In this relationship the individual invests the authority with benevolent protective powers and relates to the latter with expectant trust.

Implicit, if not explicit, is the understanding that the authority has the knowledge, the skill, and the desire to help the person overcome the problem for which professional services have been sought.

The more bewildered and helpless the person, the greater the reliance that individual places on experts.

This is certainly the case in the sick patient afflicted with a physical ailment who seeks relief from pain and distress from a physician.

It is a most important factor in the psychotherapeutic situation, particularly at the beginning of treatment.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 51 One way that the relationship operates is that while the patient may want to cling to his or her neurotic patterns, the patient may be willing to experiment with different behaviors solely to please the therapist.

Once the patient receives rewards in the form of approving gestures and words for changes in attitudes and behavior, such changes may become solidified.

Of importance here is an alteration of the self-image, which contributes to the permanence of change.

Modeling oneself after an idealized therapist is another change mechanism and may gear the patient toward new constructive experiences.

A relationship with the therapist helps the patient to tolerate, explore, and accept personal aspects that have been repudiated and repressed.

The incorporation of a new image of authority as embodied in the more tolerant, non-judgmental therapist helps alter a punitive, intolerant superego.

As a consequence of a trusting relationship, the patient may be willing to accept a proffered rationale or myth to explain the existing problem, and this may lessen tension if it accords

with the patient's belief system.

Through the instrumentality of the relationship other important processes are liberated that may exert a healing effect on the patient.

First, the patient, terrified by the disruptive emotions that are out of control, has a chance to put his or her destiny in the hands of a helpful, understanding, protective, and non-punitive authority.

There is a tendency to regard the latter as an idealized parental image who is more or less infallible.

The sicker and more anxious the patient, the more godlike the projected image and the less realistic evidence the patient will need to reinforce the illusion.

As a consequence of this union, morale may be restored to the patient, a contingency believed by some authorities to be the primary function of all the rituals lumped under the term of psychotherapy (Frank, 1974).

Second, superimposed on this dependency is a need to obtain from the authority a factual explanation for the emotional turmoil in terms that are understandable and acceptable to the patient.

The validity of the proposed facts, as has been explained previously, is not as important as the patient's willingness to accept them.

Third, the patient demands a formula from the authority that will rectify the actual or imagined sources of the difficulty.

Fourth, the patient utilizes the relationship for encouragement or help in putting into effect the actions that will resolve troubles.

Fifth, the patient finds in the relationship a medium in which to review attitudes and values.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 52 Since these are in a direct or indirect way associated with concepts of authority, a constructive relationship serves to alter ideas about hostile or rejecting authority.

The content of the interviews, and the activities inherent in carrying out the treatment plan, may not be as important as the nonverbal emotional crosscurrents that can potentially provide for the patient a truly corrective experience.

Impressive changes under these circumstances can eventuate.

The patient will tend to interpret the past and present in a new and less threatening light.

These processes operate in all helping relationship situations whether or not they are dignified with the name of psychotherapy, conditioning approaches, pharmacotherapy, etc.

The importance of this relationship factor is too often minimized, and complete credit for ensuing benefits is falsely extended to the special techniques and maneuvers executed by the therapist.

It is important to realize that, irrespective of the brand, the depth and the real worth of the healing measures that are being employed, improvement may be sustained in some instances for an indefinite period as a pure product of the relationship.

These "transference cures" are not always temporary as has generally been believed.

Sometimes they have lasted for years, but if stress sources have been corrected, transference distortions worked through with alteration of perceptions of authority and the patient's self, and if personal values and meaning systems have deviated in the direction of a healthy integration, the change may be a permanent one.

On the other hand, where associations with previous authority figures have been disturbingly stormy, the patient from the start may deploy defenses against a trusting involvement.

Or after a brief positive period defiant attitudes may erupt to confound both patient and therapist, the sources of which are rooted, not in reality but in residues of early unresolved childhood ambivalencies toward parental and other important adult figures.

Such “transference” outbreaks will interrupt or explode therapeutic progress. The restoration of basic trust is the foremost task in all forms of psychotherapy (Strupp, 1972a).

This is most effectively accomplished by a therapist who does not have residual interactional difficulties (countertransference) that he or she projects into the therapeutic situation. [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 53 While the relationship dimension is operative in every human encounter, it is most effectively utilized by a trained psychotherapist who knows how to deal with the resistance pitfalls of transference and the obstructive ravages of countertransference.

These are the usual impediments in nonprofessional relationships or in associations between patients and untrained surrogates.

**THE FACTOR OF EMOTIONAL CATHARSIS** The sheer act of talking can provide an individual with considerable emotional palliation (54).

It furnishes a motor outlet for the release of tension.

It softens inhibitions and liberates conflicts that have been held in check.

It exposes suppressed attitudes and ideas that the person has been concealing, and it encourages the individual to subject these to the light of critical reasoning.

It brings to the surface repudiated and fearsome impulses, with their attendant feeling of shame.

In this way, it takes the strain off autonomic channels that have been used to unload accumulated neurotic energy.

In the unburdening process, there is often a relief of guilt feelings in relation to past experiences, particularly sexual acting-out, hostile or aggressive outbursts, and competitive strivings.

Guilt is appeased as one examines presumably shameful fantasies as well as antisocial and unethical impulses.

Discussing these with an empathic person gives reassurance that one is not a helpless victim of uncontrollable strivings, that one has not been irreparably damaged by one's past.

Reviewing incidents in which one has been hurt, humiliated, or exploited also tends to put these into proper perspective.

Sharing one's fears of catastrophe and illness lightens their formidable quality.

Relinquished are conscious restraints that rob the person of spontaneity.

In short, the putting into words of diffuse and terrorizing feelings, and the acceptance by the listener of the pronouncements without expected condemnation and rejection, enables the person to gain greater control over emotions, to reconstitute new defenses, and to enforce a constructive action plan.

The incorporated image of harsh authority softens.

These developments may occur in the presence of any listener, whether this be a sympathetic friend or a respected authority, such as a physician, teacher, lawyer, minister, [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 54 or psychotherapist.

During psychotherapy emotional catharsis is especially prominent as imaginative conceptions and unconscious thoughts and feelings break through.

Training in interviewing and knowledge of how to encourage verbalizations, to direct them into productive channels, to give measured reassurance, or to challenge productions in a deliberately provocative way are important in facilitating the effect.

As the patient realizes that there is a consistent absence of vindictiveness from the listener, concepts of punitive authority tend to soften and some alteration of a harsh superego may possibly develop.

**THE FACTOR OF SUGGESTION** Human beings are constantly being influenced by various authoritative

formulations and directives.

Education is dependent on this process, students incorporating the ideas of their teachers whose wisdom and experience qualify them to instruct and indoctrinate.

In any helping relationship many forces are operative, including the need to identify oneself with the helping personage who serves as a model.

There is then an unqualified tendency to assimilate the precepts and injunctions of the helper purely on the basis of suggestion.

The influence of suggestion in one's daily life cannot be minimized.

Propaganda and advertising are dependent on it.

It is a factor that promotes thoughts, feelings, and behavior that operate for and against the individual.

It is the motor behind the placebo effect, fashion, vogue, and cultism.

It shapes many human activities.

It is an aspect of every helping situation.

Not only may it serve to bring about a relief of tension, but, if the helping agency happens to possess the proper values, it may, through the instrumentality of identification, register a reconstructive personality alteration.

Suggestion operates in all forms of psychotherapy even psychoanalysis (Bibring, 1954), and it has even been postulated that the suitability of an individual for treatment is dependent on the potential openness to the suggestive influence (Strupp, 1972b).

The exact nature of suggestion as a psychological phenomenon is not understood, and its eventual identification may supply the sought-for synthesis between the various schools of psychology (Winkelman & Saul, 1972).

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 55 During psychotherapy the therapist deliberately or obliquely throws out ideas and directives that the patient often will pick up and utilize.

Sometimes these suggestions are helpful and constructive, at other times less so.

The therapist may not be aware that cues are constantly being released to the patient, not only from verbal statements, but also from nonverbal signals such as facial expressions of approval or disapproval, hesitations, silences, the accenting of some of the patient's comments, nodding, shaking the head, grunting, and various physical movements.

In this way, suggestive elements may come through, even where the therapist believes that little or nothing is being said.

A simple nod in relation to something the patient has said or done will give the patient the idea that what is being said or done is good and that one should continue in this approved vein.

But if the therapist says nothing or shakes the head, this may constitute an aversive suggestion for the patient and deliberately or unconsciously discourage certain types of activity.

The technique of paying no attention to a psychotic person's delusional ramblings or hallucinations, but expressing interest in the patient when acting reality-oriented will tend to reinforce constructive preoccupations.

All patients pick up from verbal and nonverbal cues of the therapist certain things they should do and believe in.

This includes insights that initially act as a placebo.

Even if an insight is wrong, if the person believes in it and imagines that it can help, he or she may start feeling better as a placebo consequence.

In dynamically oriented psychotherapy, direct suggestions are kept at a minimum, the patients being encouraged to think through their own solutions.

Nevertheless, the factor of prejudicial, inexpedient, or unwise suggestions unwittingly being

made must always be kept in mind.

On the other hand, if the suggestions are productive ones, the patient may benefit from pursuing them.

Nor is it essential in therapy always to abstain from direct suggestions or ego-building persuasive formulations.

Homework given to the patient is an example of the constructive use of suggestion.

The therapist will have to gauge the patient's readiness to experiment with any anxiety-provoking action before making a direct or indirect suggestion that the patient undertake it.

A premature exposure resulting in failure may merely intensify a phobia.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 56 A number of variables may be distinguished that appear to regulate the forcefulness of suggestion (Wolberg, 1962).

The first variable is concerned with the significance to the individual of the suggesting agency.

The anachronistic residue of need for a protective parent that is present in all persons makes the individual more suggestible in the presence of an authority symbol who approaches an ideal.

The higher the dependency level the more apt is the individual to design out of every relationship a child-parent tableau.

Behind the helplessness of such gestures is a drive to absorb the strength of the authoritative token through submissive identification.

By yielding to the authority's power the hope is to become powerful.

Where the dependency need is sufficiently intense, the individual may respond to instrumentalities that possess any kind of protective promise—including drugs, placebos, faith healing, yoga, hypnosis, psychotherapy, etc.

Another factor that heightens dependency, and hence intensifies suggestibility, is anxiety.

Individuals whose mastery is shattered will cling with desperation to any potential helping resource, responding dramatically to preferred injunctions and commands.

The intrusion on the individual of some catastrophic event that damages defensive integrity and diminishes security and self-confidence will thus tend to lower the suggestive threshold.

Soldiers after battle, and persons exposed to accidents and natural disasters, often verbalize the need for a loving, protective agency.

Under such circumstances the person may respond intensely to suggestions that are more easily resisted later.

A momentary regression to the actual or fantasied securities of childhood is the prompting that inspires the anxiety driven soul to aggrandize the expediency that is offered.

Qualities in the helping agency positively correlated with suggestibility are those that inspire confidence in the subject and raise expectations of spectacular performance from the agency.

Negatively correlated are characteristics that promote lack of confidence or that arouse resentment.

Appearance, diction, status, reputation, and fame usually enhance the authoritative image.

Group acceptance and acknowledgment of power of the authority as well as strong charisma of the authority augment the suggestive mystique.

The second variable is the significance to the subject of the specific content of the preferred suggestions.

The precise meaning of the communications to the individual is [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 57 what is of determining importance, rather than their true content.

Many vectors are operative, not the least of which is motivation.

If the recipient of a communication, verbal or nonverbal, is attuned to respond positively,

material will be plucked out of context that seems to justify a positive response. Utterances by prophets wedded to contemporary philosophical and religious movements may sound nonsensical to a casual and uninvolved observer, yet to a believer they are spell-binding and heavenly inspired.

Every individual is to some extent the victim of a mirage inspired by the projection of inner needs.

Thus the world is fashioned to personal improvisations.

If the helping authority is able to divine the incentives of the client, more suitably meaningful arguments and memoranda will be supplied.

The third variable that modulates suggestibility is the degree of anxiety that is mobilized in the subject by the acceptance of a specific suggestion or by the relationship itself.

Individuals who fear submitting themselves to others, who retreat from domination, who are reluctant to yield to their dependency drives, who are compulsively independent, who preserve a defensive detachment, or who are fiercely competitive may resist suggestions, even those that can be helpful to them, no matter how convincingly these are phrased.

Transfereential contaminants often enter into relationships, distorting their dimensions.

To some extent, transference may be controlled if the helping person is aware of its presence and restrains untoward countertransfereential impulses that neurotically interlock with those of the subject.

Nevertheless, even though the helping person attempts to keep the relationship reality-oriented, it may disintegrate through assignment to the agency of the most noxious characteristics of significant personages in the subject's past.

As a hated parent, the sentiments of the agency will be bitterly resisted.

In the absence of obstructive transference, suggestions may still be resisted if they challenge or run counter to vital defenses.

In the latter case, anxiety will tend to neutralize or modify the suggestion.

This was dramatically illustrated by a patient I was asked to see to help differentiate psychic from organic pain.

The surgeon who had advised a disc operation for severe back pain suspected a hysterical reaction; to rule this out, he requested a consultation.

To see if the patient's pain could be removed by suggestion [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 58 (which would help establish a functional basis for the pain), I induced hypnosis, during which I suggested that upon awakening the pain would disappear from his back, but would instead be transferred to his right arm.

The patient responded positively with considerable surprise.

Upon rehypnotizing him, I demonstrated to him that I could transfer the pain from his arm to his right leg.

Thereafter I attempted its displacement to his left arm.

To my consternation this suggestion was resisted, but instead the pain was again felt in the habitual back zone.

I reinduced hypnosis and repeated the transfer of pain to various areas, which proved successful except for the left upper extremity.

Questioned under hypnosis regarding this puzzling phenomenon, the patient replied that his father had died from a heart attack.

Prior to his death he had experienced angina pectoris, with referred pain to his left shoulder and arm.

The patient feared succumbing to the same illness.

Apparently the meaning to him of accepting my suggestion to feel pain in the left arm was to acknowledge succumbing to angina pectoris.

His anxiety blocked my suggestion.

Where suggestions upset the adaptive equilibrium, they will tend to be resisted.

Thus, where repression is threatened by a suggestion, or where positive gains and pleasure associations are in danger of renunciation, or where unconscious masochistic needs power the existing neurotic operations, the individual may be oblivious to the most sensible maxims or magisterial commands.

If prevailing values and philosophies are contradicted, suggestions may also be repelled.

Suggestions are best formulated, therefore, in terms that are congruent with the person's ideologies.

The fourth variable concerns the subject's critical judgment.

In spite of the fact that suggestion resides in the penumbra of illogic, critical reasoning and common sense are still posted as sentinels.

Periodically they will interfere with suggestion's automatic pursuits.

One may observe this in somnambulistic subjects during hypnosis, who, compulsively responding to irrational posthypnotic suggestions, force themselves to challenge, to inhibit, and, finally, to oppose acts that are foolish or without purpose.

Eventually, an interference with helping exhortations will probably occur should they not harmonize with the individual's intellectual understanding.

First accepted on faith, they [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 59 finally fall under the critique of logicity.

**CONSTRUCTIVE GROUP EXPERIENCES (GROUP DYNAMICS)** Groups exert a powerful influence on individual.

They may be responsible for significant changes among the constituent members.

The effect of Alcoholics Anonymous on victims of drink, of Synanon on drug addicts, and of Recovery on former hospitalized patients are examples of how even serious personality and emotional problems may be benefited through constructive group adventures.

In order for change to occur, however, a new and unique group experience is required, one that contributes to the individual's security and desires for belonging, at the same time that it challenges the customary patterns of thinking, feeling, and acting.

This generally occurs when the person is forced to participate in a problem-solving situation with an assemblage that is respected, the members of whom join their forces for some united purpose.

For example, a body of parents may meet at regular intervals for deliberation and action on problems affecting their children at school.

A special committee of a fraternal order convenes for a cooperative pooling of ideas and skills.

A council is elected in a neighborhood to devise tactics to fight racial discrimination.

A select crew of supervisors in an industrial plant organize to bolster the morale of the workers.

A small recreational or hobby club is developed that offers an exciting program.

Even though individuals have functioned in various groups during their life span with usual controls and defenses, the new group, if it is to be effective, will not sanction, indeed may challenge, their customary behavior if it runs contrary to the group's standards.

This creates conflict and may force them to interact in a different way.

Deviants—that is, persons whose standards are in conflict with the group norms—are particularly affected since their conduct upsets the members, who may turn on the deviants and try to force them to conform.

If the person has an investment in remaining with the group, the price may be a change in manner or ways of thinking.

As people collaborate in activities and relationships, they learn which of their traits and actions encourage rewards and which bring punishments.

Thus a person who seeks to [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 60 control the group will sooner or later realize that domineering tendencies are resented, but that acceptance depends on acting cooperatively.

The timid, submissive, and detached soul may be goaded into harmonious team work and assertiveness as security in the group becomes more firmly entrenched.

Identification with the group assuages the individual's helplessness and isolation.

Approval and disapproval cut deeply into personality defenses.

As the individual reacts repetitively with customary patterns to manipulate, monopolize, bully, undermine, lampoon, withdraw, reassure, comply, or collapse, oppositional responses of the group create a crisis.

Attacked, the person may then be tempted to leave the group or may be invited to leave.

It is at this point that change becomes possible, prompted by a need to remain with the group.

The individual may then learn that it is not necessary to shout to be heard or to flaunt one's virtues to be acknowledged.

There may be the realization that criticism can be constructive without offending and, conversely, that one can be criticized without being a victim of an evil design; that one may relate without becoming enslaved, accept help without loss of status, stand up for one's rights without being attacked, perform without being rejected, give without feeling exploited; and that one can be liked for what one is and not for what one does.

These experiences have a forceful effect, particularly where defenses are not too rigidly set. A remodeling of values, attitudes, and behavior follows upon a more accurate perception of one's role in the group.

Actually, while these happy effects are psychotherapeutic, they cannot be considered in the same light as those accomplished through a structured psychotherapeutic program, for instance, group psychotherapy.

Social or problem-solving groups are organized with an objective other than the treatment of its members; beneficial results in personality are a mere by-product of the existing group dynamics.

While the individual may retain new patterns, generalizing them toward situations other than the group in which they were evolved, the tendency is to slip back into old habits when pressures to conform are no longer present.

If, however, an understanding of disturbed ways of relating has been integrated, a permanent effect on character structure may be registered.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 61 DYADIC GROUP DYNAMICS In individual psychotherapy a dyadic group situation exists that inspires some of the identification phenomena observed in larger groups.

Various cues (e.g., the magazines and books in the waiting room, the pictures on the walls, the office furnishings and decor, the style of therapist's clothing and grooming, and the therapist's comments and behavior) will lead the patient to speculate on the value systems of the therapist and to attempt selfmodeling along similar lines, or at least to challenge some personal standards and patterns.

In short-term therapy interpersonal dynamics involving the therapist must be considered also as a factor in reeducation.

Karl Menninger (1952a) has emphasized that the value of the psychiatrist is greatest as a person rather than as a technician or scientist.

What one is has more effect upon the patient than anything one does.

Because of the intimate relationship between patient and psychiatrist, the value systems,



standards, interests and ideas of the doctor become important." A therapist cannot help but communicate personal values to patients.

This will occur no matter how passive, non-interfering and nonjudgmental the therapist tries to be.

An attempt may be made to suppress verbal valuations, but nonverbal prejudices will nevertheless come through.

Nods, grunts, groans, smiles, immobility, fidgetiness, pauses, choice of topics for questioning, emphasis, repetitions, and interpretations will soon convey to the patient the therapist's worldviews and tendencies toward deviance and conformity.

Subtle indications such as the kind of waiting room furniture, pictures and magazines, the therapist's hair style and clothing preferences, and the manner in which office routines, billing, and appointments are conducted are as eloquent in revealing standards as any direct verbal avowal of values.

It is useless to try to conceal the fact from the patient that the therapist has a definite point of view and possesses distinctive tastes and prejudices.

Indeed, the patient may even divine unconscious values and during transference confront the therapist with a bill of particulars, the validity of which may be staggering.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 62 If a therapist's values are apparent to the patient, the question may be asked whether to express them verbally as articles of personal conviction, at least those of which the therapist is aware?

It is obvious to most therapists who believe this that they must nevertheless restrain themselves from political and other declarations, and especially not force their values on patients, even when the therapist is convinced of their moral and pragmatic worth.

The patient's right to accept or reject the therapist's standards is usually respected.

Moreover, the therapist, assuming an ability to be objective, may subject personal value systems to soul-searching to discern which of these are warped.

A frank encounter with oneself, buttressed if necessary by personal therapy, may be a boon to both therapist and patient, since many patients will incorporate the therapist's theories and moral precepts more or less uncritically on the basis of a need to please, in order to learn from and amalgamate with the idealized authority figure who is rendering help.

This is probably allied to mechanisms that take place in any educational process.

**EFFECTS OF EXTRATHERAPEUTIC AIDS ON PSYCHOTHERAPY** How can we assay the effect of ad non-specific changes that have nothing to do with a formal psychotherapeutic situation?

It is doubtful if we can fully differentiate intercurrent from therapeutic factors.

Forces prevailing upon the individual outside of the psychotherapeutic process are often so elusive that they defy detection, let alone analysis.

For example, results of psychotherapy are undoubtedly influenced by the implicit assumption of the patient that one is expected to and will change for the better.

Under these circumstances, irrespective of the specific theories of the therapist and the tactics that the therapist employs, the patient may tend to show improvement.

Suffice it to say that the therapist accepts with gratitude the fact that there are supplementary "spontaneous" and "helping" healing aids to assist in therapeutic efforts.

The therapist tries not to interfere with these elements except insofar as they detract from treatment objectives.

For example, where the individual utilizes escape and control mechanisms to withdraw from relationships, or where existing philosophies negate a productive adjustment, the therapist will treat these devices as resistances to productive [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 63 change. But, by and large, the adventitious ameliorative agencies are accepted as unavoidable.

Some psychotherapeutic systems attempt to encourage them, exhorting the individual to seek out

spontaneous activities that help in growth; creating an atmosphere in therapy to inspire trust, faith, and confidence in order to enhance the placebo element; stimulating emotionally cathartic expressions of feeling; promoting a positive relationship situation by assuming the role of a benevolent, giving authority; suggesting participation in social or problem-solving groups; and fostering the development of philosophical formulas that take pressure off the individual and enhance a new way of looking at one's responsibilities.

Indeed some treatment schemes, adopting the tactic that if we cannot eradicate the coincidental therapeutic forces, we should join them, incorporate the extratherapeutic aids into the body of their systems.

From a practical point of view, the therapist may assume that whether he likes them or not, these elements are going to play some part in the patient's getting better.

The therapist would do well then to acknowledge their part in any results claimed.

Logic convinces that the effect of any psychotherapy actually is a summation of that psychotherapy plus intercurrent non-specific healing adjuncts.

Once we stop struggling against the impossible task of separating these two protagonists, we can apply ourselves to the task of utilizing both psychotherapeutic techniques and coincidental constituents that make for therapeutic gains.

Can we delineate the kind of psychotherapeutic climate that will expedite the nonspecific healing forces?

A considerable number of observers have attempted to do this in the past, among whom are Alexander (1948), Rogers (1951), Fromm-Reichmann (1952b), Raush and Bordin (1957), Strunk (1958), Halpern and Lesser (1960), Strupp (1960), Jourard (1959a), and Truax and Carkhuff (1964).

We may speculate that the climate of successful therapy depends on a number of factors that are operative in all good helping situations.

These include the following: 1.

Hope.

Patients approach the ministrations of the therapist with assured expectancy.

A sanguine anticipation of success is present.

Therapists have confidence in their methods and theories.

This activates the placebo effect.

2.

Trust.

Patients see their therapists as sincere, honest, reliable, guileless,

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 64 trustworthy, undeceitful, unaffected, straightforward, and authentic beings.

Therapists display respect for their patient in spite of the latter's pathological behavior.

Therapists also reveal their own genuineness.

3.

Freedom to respond.

Patients permit themselves to think, feel, and act without restraint.

This permits them to verbalize freely and to unburden themselves.

Therapists encourage freedom in reactivity.

This sponsors emotional catharsis.

4.

Faith.

Patients have a conviction that therapists possess the percipience, sensitivity, wisdom,

experience, skill, and ingenuity to understand basic presenting problems and difficulties and to know what to do about them.

Therapists convey assuredness, positiveness, and empathic understanding. This facilitates the suggestive factor.

5.

Liking.

Patients conceive of their therapists as empathic individuals who relate to them personally and warmly; in turn, therapists feel a nonpossessive warmth toward the patients.

This releases dynamic ingredients present in all human encounters (dyadic and larger groups) that constructively influence attitude change.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 65 5 Significant Variables that Influence Psychotherapy Apart from extratherapeutic aids that can positively influence results in psychotherapy, multiple variables enter into the treatment gestalt that bear upon results for the good or bad.

At the outset it is necessary to recognize that no psychotherapeutic method exists today that is applicable to all patients or germane to the styles of all therapists.

As disparate as the various approaches to psychotherapy may seem, their impact on the patient is often registered in similar ways.

First, they offer a unique kind of interpersonal relationship in which one feels accepted for what one is and where judgements concerning attitudes and behavior do not agree with habitual expectations.

Second, there is an explicit and implicit reinforcement of selected responses with the object of overcoming important behavioral deficits and of extinguishing maladaptive habit patterns.

Third, there are direct or indirect attempts made at cognitive restructuring, through various instrumentalities, such as (1) persuasive arguments and proffering of philosophical precepts, (2) the exploration of conscious and unconscious conflicts aimed at the inculcation of insight, and (3) the provision of a corrective behavioral and emotional experience within the matrix of the patient-therapist interaction.

Irrespective of behavioral parameters that purportedly are selected for inquiry and rectification, the patient will respond to the therapeutic interventions being utilized in accordance with personal needs and readiness for change.

These facts have fathered a common idea among professionals that therapists of different theoretical orientations do essentially the same things.

The patient presents a problem; an attempt is made to establish a meaningful relationship; some formulation is presented to the patient as a working hypothesis; and special procedures are implemented to enhance the patient's mastery and eliminate disruptive elements in adjustment.

If this be true, then techniques are merely forms of communication secondary

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 66 to a host of transactional processes that draw from many biological, intrapsychic, and interpersonal vectors.

A reasonable question that may be asked is whether current research can shed some light on how these vectors can be organized and manipulated to make our operations more effective.

Specifically, do we have reliable data that will permit us to match patients, therapists, and techniques?

Offhand we must admit that empirical studies to date have not settled this question.

In an extensive review of research on the assessment of psychosocial treatment of mental disorders by an NIMH working group functioning as an advisory committee on mental health to the Institute of Medicine of the National Academy of Sciences, it is concluded that we do not yet have the answers to the basic question of "what kinds of changes are effected by what kinds of techniques applied to what kinds of patients by what kinds of therapists under what kinds of conditions" (Parloff et al, 1978a).

We have insufficient data to date regarding the relative effectiveness of the different therapeutic modalities as well as the patient-therapist preferences to make a scientific

matching feasible.

When we consider the choice of the best kind of therapy, the report sums up the following way: "In summary, the data does not show that any of the tested forms of therapy are unconditionally superior to any other form." The report goes on with the statement that the existing single studies have proved inadequate to answer questions that we need to resolve.

What is required is a coordinated planning of wide-scale sophisticated research utilizing an agreed upon minimal set of standardized measures for describing and assessing the kinds of problems, the variety of treatment interventions, and the nature and degree of behavior change. Since help from present-day research is so problematic, we must depend on clinical hunches in sorting out the significance of the many variables in psychotherapy.

For convenience we may group these into: 1.

Patient variables 2.

Therapist variables 3.

Social and environmental variables [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 67 4.

Transfereential and countertransfereential variables 5.

Resistance variables PATIENT VARIABLES The Syndrome or Symptomatic Complaint Are there special techniques that coordinate best with selected symptom complexes, methods that are more rewarding with some types of complaints as compared to other types?

How really important are techniques in psychotherapy?

Can different therapists get the same results by utilizing various kinds of interventions with which they are individually expert?

By and large techniques do operate as a conduit through which a variety of healing and learning processes are liberated.

How the techniques are applied, the faith of therapists in their methods, and the confidence of the patient in the procedures being utilized will definitely determine the degree of effectiveness of a special technique.

But techniques are nevertheless important in themselves and experience over the years with the work of many therapists strongly indicates that certain methods score better results with special problems than other methods.

Thus symptoms associated with biochemical imbalances, as in schizophrenia or affective disorders, may be relieved with proper psychotropic medication, for example, neuroleptics in the case of schizophrenic thinking disorders, antidepressant drugs in depressive states, and lithium for manic excitement and bipolar depressions.

While thinking disorders in schizophrenia are helped by neuroleptics, problems in social adjustment are better mediated by behavior therapy, family therapy, and counseling.

Indeed, milieu therapy in the form of a therapeutic environment staffed by accepting, supportive persons or proper accepting, nonstressful surroundings in which to live and work (Warner, 1985) often permit an adequate adjustment without the use of drugs.

Such a congenial atmosphere must be continued indefinitely, in some cases over a lifelong period.

The presence of a hostile or nonaccepting family correlates with an increased [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 68 rehospitalization rate.

Research studies, i.e., controlled outcome studies for individual or group therapy in schizophrenia, provide no strong evidence for or against the value of psychotherapy.

Generally with continued aftercare following hospitalization in the form of a behaviorally oriented program and an adequate residential regime (halfway houses, partial hospitals, rehabilitation units and other community support systems), over 90 percent of patients can be discharged in the community with a 2-year rehospitalization rate of less than 5 percent (Paul & Lentz, 1977).

Our experience at the Postgraduate Center for Mental Health confirms the value of a rehabilitation program in lowering rehospitalization rates and, in rehospitalized patients, reducing the time of confinement.

Behavioral token economies are often valuable in hospitalized patients toward regulating ward behaviors in areas such as self-care, grooming, and social adjustment, and to control target behaviors such as job performance.

Behavioral extinction procedures have been utilized to manage gross pathological behavior, like violence and destructive tantrums in psychotic autistic children, especially when coupled with differential reinforcement of constructive behavior.

Such behavioral gains are consolidated when management techniques are taught to parents or other adults with whom the patient lives.

No cure of the basic condition is brought about by these methods, but definite improvement of behavioral repertoires may be achieved.

While drugs influence depressive symptoms, they have little effect on patterns of behaving and thinking, which are best mediated through dynamic interpersonal therapy, cognitive therapy, and behavior therapy aimed at reinforcing involvement with pleasant activities and changing target symptoms and behavior toward improvement of interpersonal relations and social functioning.

Combined drug and psychosocial therapy is thus best in most depressions.

In suicidal depression electroconvulsive therapy has proven to be a lifesaving measure.

In hyperkinetic children it is generally found that we may be able to reduce the overactivity with psychostimulant medications, like Ritalin® and Dexedrine®, plus behavioral forms of reinforcement of positive behavior or such mild aversive techniques

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 69 as removing the child with disturbing behavior to a "time-out" room for a few minutes.

To influence neurophysiological dimensions, as reflected in severe tension states, simple relaxation procedures like meditation, relaxing hypnosis, or autogenous training, or emotive release therapy have been instituted.

Certain psychosomatic conditions, such as muscle spasms, migraine, hypertension and arrhythmia, may sometimes be helped by biofeedback.

Unfortunately, biofeedback has been oversold, its utility being limited to selected patients (Miller, 1978).

Behavioral advocates claim that best results with obesity are obtained with measures directed at self and stimulus control.

Behavioral operant reinforcement techniques and systematic desensitization are being used in anorexia nervosa, although the evidence of their effectiveness is still unclear.

More recently progress has been reported with anorexia and bulimia utilizing antidepressants.

Smoking control, it is claimed, is best handled by multicomponent treatment packages that program a reinforcement of nonsmoking behaviors and utilize smoking suppressive tactics (Bernstein & McAlister, 1976) supplemented by nicotine gum chewing.

Stimulus satiation, i.e., rapid smoking to satiation, is temporarily effective but the physical side effects must be considered.

Hypnosis is also useful in combination with follow-up behavioral methods.

In enuresis behavioral approaches (such as a urine alarm bell) are claimed to be superior to dynamic psychotherapy and imipramine.

Sexual disorders (voyeurism, fetishism, sadomasochism, exhibitionism, transvestism), though difficult to treat, are approached by some therapists with aversive therapy, such as electrical stimulation for transvestism (Marks, 1976) and with certain idiosyncratic methods like having an exhibitionist appear before a female audience to expose himself while talking about his condition, the extraordinary procedure being credited with some success on follow-up

(Wickramasekera, 1976).

Sexual dysfunctions (impotence, frigidity, dyspareunia, vaginismus, anorgasmia, premature ejaculation) do well with various behavior therapies, hypnosis, and dynamic psychotherapy. Of all of these programs, the Masters and Johnson techniques have proven most popular and have in the opinion of some authorities been made more effective when blended with [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 70 psychodynamic and interpersonal techniques (Sollod & Kaplan, 1976).

Phobias that have their origin in conditioning experiences (i.e., exposure to parental fears, like a mother who is in terror of mice or a father who shies away from heights), or to a catastrophic personal happening (i.e., an accident in an airplane in which a person was traveling, resulting in a fear of flying), or to an anxiety or physiologically distressing experience linking itself to a coincidental stimulus (i.e., nausea or gastric upset occurring at the same time that one is eating a certain food, resulting in a subsequent refusal to eat that food), seem to respond more rapidly to systematic desensitization, flooding or graded exposure to a phobic object than to any other kind of technique.

On the other hand, phobias that are the product of deep personality conflicts, the projected symbols of unconscious needs and fears, are in a different class from conditioned phobias and do not respond as well to behavioral methods.

In fact, they may stubbornly resist those techniques and are better suited for dynamic approaches.

Of all phobias, agoraphobia seems to be the most resistant to verbal therapies but may respond to antidepressant drugs.

About two-thirds of patients with anxiety reactions are helped by both dynamic psychotherapy and behavioral techniques, such as systematic desensitization, progressive relaxation, "participant modeling" (i.e., the therapist modeling how to master anxiety), and temporary drug therapy.

Behavioral therapy has moved toward recognizing the importance of cognitive factors (such as irrational self-statements and false attributes) and the need not only to modify environmental parameters, but also to consider the patient's interpretation of events and thoughts that mold reactions to the environment.

In line with this, applications of behavior therapy have expanded toward a wide spectrum of neuroses and personality disorders.

Some obsessive-compulsive disorders respond to such antidepressants as clomipramine (Anafranil®), others come under control with aversive behavioral techniques when they resist every other type of manipulation.

Prolonged exposure to thoughts or situations that provoke compulsive rituals combined with blocking of the [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 71 patient from engaging in such rituals has yielded some successes.

In vivo exposure is superior to exposure to fantasies.

We may minimize persuasion as a technique, but it can have a potent effect on some obsessive states, adjustment reactions, and related conditions (Truax & Carkhuff, 1967).

Stubborn as they are, personality problems and most neurotic disorders that are bracketed to personality problems are conditions most subject to the utilization of a dynamic orientation that probes for provocative conflicts and defenses.

Relationship difficulties are particularly suited for group therapy, marital therapy, and, especially where children are involved, family therapy.

Some addictions seem to respond best to certain inspirational groups (e.g. drugs with Synanon, alcohol with Alcoholics Anonymous, food gorging with Weight Watchers, gambling with Gamblers Anonymous).

These groups are preferably led by a person who has gone through and has conquered a particular addiction and is willing to serve as a model for identification.

Accordingly, where a therapist works with such groups, an exaddict co-therapist may be a great asset.

Where repressions are extreme, classical psychoanalysis, intense confrontation, hypnoanalysis, narcoanalysis, and encounter groups have been employed in the attempt to blast the way through to the offensive pathogenic areas.

Understandably, patients with weak ego structures are not candidates for such active techniques, and therapists implementing these techniques must be stable and experienced. Antisocial personalities subjected to a directive, authoritarian approach with a firm but kindly therapist sometimes manage to restrain their acting-out, but require prolonged supervised overseeing.

Apart from the few selected approaches pointed out above that are preferred methods under certain circumstances, we are led to the conclusion that no one technique is suitable for all problems.

Given conditions of adequate patient motivation and proper therapist skills, many different modalities have yielded satisfactory results.

It is my feeling, however, that whatever techniques are employed, they must be adapted to the patient's needs and are most advantageously utilized within a dynamic framework.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 72 Transference reactions may come through with any of the techniques, even with biofeedback and the physical therapies.

Alerting oneself for transference, one must work with it when it operates as resistance to the working relationship.

Unless this is done our best alignment of method and syndrome will prove useless.

The fact that certain techniques have yielded good results with special syndromes and symptoms does not mean they will do so for all therapists or for every patient.

Interfering variables, such as will henceforth be described, will uniquely block results or will make the patient susceptible to other less popular methods.

**Selective Response of the Patient to the Therapist** At its core the patient's reaction to the therapist often represents how the patient feels about authority in general, such emotions and attitudes being projected onto the therapist even before the patient has had his first sessions.

The patient may rehearse in advance what to say and how to behave, setting up imaginary situations in the encounter to come.

Such a mental set will fashion feelings that can influence the direction of therapy.

Thus, if the patient believes that authority is bad or controlling, oppositional defenses may be apparent during the interview.

These global notions about authority and the reactions they sponsor are usually reinforced or neutralized by the response to the therapist as a symbol of an actual person important to the patient in the past (transference).

Some characteristic in the therapist may represent a quality in a father or mother or sibling and spark off a reaction akin to that which actually had occurred in past dealings with the person in question.

Or the reaction may be counteracted by a defense of gracious compliance or guarded formality. Confidence in one's therapist is enormously important—even when the therapist's ideas of the etiology or dynamics of the patient's difficulty are wrong.

The patient's acceptance of explanations proffered with conviction can have a determining influence on the patient.

They are incorporated into the patient's belief system and sponsor tension reduction and

restoration of habitual defenses.

Through what means this alchemy takes place is not clearly known, but suggestion, the placebo effect, and the impact of the [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 73 protecting relationship offered by the therapist undoubtedly play a part.

Sometimes unpredictable elements operate in the direction of cure.

This is illustrated by the case of a beautiful, poised female writer of 32 who was referred to me for hypnosis by a friend of mine after many attempts to stop smoking by herself, and after several unsuccessful tries at psychotherapy.

While there were no serious emotional problems that I could detect, and her present work adjustment, relationships with people, and sexual life seemed fairly satisfactory, one piece of data that she gave me put doubts in my mind as to how successful I would be in getting her off tobacco.

She confessed to me, somewhat shamefacedly, that she still used a baby pacifier that she stuck into her mouth alone at home (she was single) at times when a cigarette wasn't between her lips.

Often she went to sleep with it in her mouth.

I asked her if she knew what this meant.

"After all, doctor," she replied, "I'm no fool.

I'm just a plain oral character." Under the circumstances, I confessed to her that the chances of success with a symptom-oriented approach like suggestive hypnosis were poor, and that she had better get herself into analysis without wasting any more time or money.

"God forbid," she retorted.

Since I had promised to hypnotize her, I induced a trance with less than ardent enthusiasm, and went through my usual paces, hoping to demonstrate that my predictions of failure would surely come to pass.

Two days later she telephoned me to tell me she had quit smoking, and she asked if I would make a hypnotic tape for her.

We had three visits in all, and while she retained her discrete pacifier habit, she fooled me by staying off tobacco completely.

A year went by and she still had not resumed smoking, nor were there any substitute symptoms.

This abstinence was confirmed by several reliable friends who knew her well.

Now I have no idea of the dynamics of the symptomatic cure but it is possible that my pessimistic attitude was just the right tactic to use with her.

She may have decided to accept the challenge and to show me that I was wrong concerning my estimate of her.

Or perhaps the idea she would have to get into long-term analysis was more distasteful to her than giving up smoking.

Frankly, I was delighted that I was such a poor prognosticator, even though I still am convinced that a good analysis would have been a worthwhile investment.

The point I am trying to stress is that as thorough as we believe our initial interviewing may be as a way of estimating outcome, we still do not have all the variables at our fingertips.

We still can be right in our estimate, but sometimes, as in this case, we can be wrong, and to the patient's benefit.

Where the therapist becomes for the patient an idealized figure, the initial therapeutic impact may be enhanced.

Or if the transference is to an irrational authority, [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 74 resistance is more likely in evidence.

The degree of charisma possessed by the therapist also influences the patient's responses. Reputation, clothing, manner, and appearance all function to nurture the illusion of miracles



to come.

The various patterns that evolve have a powerful and often determining effect on any techniques the therapist may utilize.

**Selective Response of the Patient to the Therapist's Techniques** Patients occasionally have preconceptions and prejudices about certain techniques.

Hypnosis, for example, may be regarded as a magical device that can dissolve all encumbrances, or it may connote exposing oneself to Svengali-like dangers of control or seduction.

Misconceptions about psychoanalysis are rampant in relation to both its powers and its ineffectualities.

Frightening may be the idea that out of one's unconscious there will emerge monstrous devils who will take command—for example, the discovery that one is a potential rapist, pervert, or murderer.

Should the therapist have an inkling as to what is on the patient's mind, clarification will then be in order.

The manner of the therapist's style is also apt to influence reactions of rage at the therapist's passivity, balkiness at what is considered too intense activity, anger at aggressive confrontation.

Some patients are frustrated by having to talk about themselves and not being given the answers.

Moreover, responses to different methods will vary.

There is a story of a Gideon Bible (which may be apocryphal) that illustrates this.

As is known, practically all hotel rooms contain, as part of the general equipment, a Bible placed there by the Gideon Society, which in the front pages are suggested reading for the weary traveler.

For instance, if one is in need of salvation, the reference is to John 3:3; or desires peace to neutralize anxiety, Psalm 46; or relief in the time of suffering, Psalm 41; or consolation in conditions of loneliness, Psalm 23.

In a certain hotel Bible opposite Psalm 23, there was written in the margin this inscription:

"If after reading this Psalm you are still lonesome, upset, and feel life is not worthwhile, telephone 824-3921 and ask for Phyllis." This is eclecticism!

To help in the selection there are minimal techniques an eclectic therapist should know.

In my opinion, the following are most useful: dynamically oriented interview

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 75 procedures, group therapy, marital therapy, family therapy, behavior modification, pharmacotherapy, cognitive therapy, relaxation methods, and hypnosis.

But irrespective of the techniques we select, we must be sure that they accord with the patient's belief systems.

As has been mentioned previously a patient who is convinced that spirit infestation is a cause of illness will do much better with a shaman or witch doctor than with a psychiatrist.

The patient's lack of faith in what the psychiatrist does will render worthless the most sophisticated treatment efforts.

Knowledge of the cultural concepts that mold a patient's ideas about what emotional suffering is all about may make necessary some preliminary education to prevent embarking on a futile therapeutic journey.

Some attempts have been made to assign patients to certain styles of therapeutic operation according to their unique characterological patterns (Horowitz, 1977).

For example, hysterical personalities are presumed to require a mode of therapist management that differs from that effective in obsessional personalities.

This distinction considers that certain optimal learning patterns correlate with identifiable personality typologies.

The relationship between character structure, diagnostic category, and learning abilities, however, never has been fully clarified.

Thus, among patients suffering from the same syndrome, let us say obsessive personality, there are great differences in the way they will respond to certain techniques and therapist styles. While some general principles may be applicable to all or most obsessives, the existing differences prevent our using a blanket approach.

People possess different modes, even within the same syndrome, of absorbing, processing, and responding cognitively, emotionally, and behaviorally to therapeutic interventions.

Since psychotherapy is in a way a form of reeducation, the learning characteristics of a patient should best correspond with the techniques that are to be used.

Problem-solving activities are often related to the kind of processes found successful in the past.

Some patients learn best by working through a challenge by themselves, depending to a large extent on experiment.

Some will solve their dilemmas by reasoning them out through [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 76 thinking of the best solution in advance.

Others learn more easily by following suggestions or incorporating precepts offered by a helpful authority figure.

Some are helped best by modeling themselves after an admired person, through identification with that person.

Some patients work well with free association, others do not.

Some are able to utilize dreams productively, or behavior modification, or sensitivity training, or other methods.

It would seem important to make the method fit the patient and not wedge the patient into the method.

The fact that learning patterns are so unique and modes of learning so varied lends justification to an eclectic approach.

It would be advantageous, of course, in the initial phases of therapy to find out how a patient might learn best, but no expedient is available today that can give us this information.

We usually settle for the fact that when a patient is first seen, learning capacities and styles are unknown and the therapist must proceed somewhat blindly.

We more or less shoot in the dark in order to coordinate a patient's specific problem and personality with our techniques.

We may get some help during interviewing in discovering how the patient has learned best in the past.

More pointedly, we get the most reliable data by actually exposing the patient to the interventions we have to offer and observing the response to what is being done.

A few other attempts have been made to identify variables that can make the selection of a therapeutic method more feasible.

Among these are the patient's response to hypnotic induction (Spiegel & Spiegel, 1978) and the isolation of core conflicts through the making of a developmental diagnosis (Burke et al, 1979).

Some interesting speculations may emerge in watching how patients react to the induction of hypnosis, but are these sufficiently reliable to pinpoint either the existing diagnosis or choices in therapeutic method?

More experimental substantiation is required.

Some therapists have attempted to utilize the area of developmental arrest as a way of selecting the ideal technique.

Where the patient's prime difficulty is centered around resolution of separation-individuation,

this is believed best accomplished through a technique such as described by Mann (1973) in which the struggle over short-term [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 77 termination of treatment threatens the patient's dependency, lights up the separation problem, and offers the opportunity to resolve the conflict in a favorable setting.

Where the oedipal conflict is primary, the confrontation styles of Sifneos (1972) and Malan (1964, 1976a) are believed to be most effective.

For problems originating in the latency period that precipitate out in the mid-life transition around issues of productivity and creativity, the "corrective action" approach of Alexander and French (1946) is recommended.

However, it is difficult to substantiate these views because of the interference of so many other variables that can vaporize our best choice-of-method intentions.

While empirical studies tell us little about factors that make for a good patient/therapist match, we may speculate that the personalities, values, and physical characteristics of both patient and therapist must be such that severe transference and unfavorable countertransference problems do not erupt to interfere with the working relationship.

A giving, accepting, warm, and active but not too interfering or obnoxiously confronting manner in the therapist is most conducive to good results.

Readiness for Change Another important factor is the individual's readiness for change.

This is a vast unexplored subject.

A person with a readiness for change will respond to almost any technique and take out of that technique what he or she is prepared to use.

What components enter into a satisfactory readiness for change have not been exactly defined, but they probably include a strong motivation for therapy, an expectation of success, an availability of flexible defenses, a willingness to tolerate a certain amount of anxiety and deprivation, the capacity to yield secondary gains accruing from indulgence of neurotic drives, and the ability either to adapt to or constructively change one's environment.

Patients come to therapy with different degrees of readiness to move ahead.

Some have worked out their problems within themselves to the extent that they need only a little clarification to make progress, perhaps only one or two sessions of therapy.

Others are scarcely prepared to proceed and they may require many sessions to prepare [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 78 themselves for some change.

We may compare this to climbing a ladder onto a platform.

Some people are at the bottom of the ladder and before getting to the top will need to climb many steps.

Others will be just one rung from the top, requiring only a little push to send them over to their destination.

In therapy we see people in different stages of readiness for change, and we often at the start are unable to determine exactly how far they have progressed.

One may arrive at an understanding of what is behind a patient's problem rapidly.

From this we may get an idea that benefits will occur with little delay.

Yet in relation to readiness for change, the patient may still be at the bottom of the ladder.

Others are at a point where almost any technique one happens to be using will score a miracle.

We may then overvalue the technique that seems to have worked so well and apply it to many different patients with such conviction that the placebo effect produces results.

We sometimes see patients who have been in therapy for a long time under the guidance of another therapist and who, on the surface, have made no progress at all.

Surprisingly, after a few sessions the patient will begin to progress remarkably, creating the impression in both participants that the therapist is a genius in being able to do in three sessions what the previous therapist was not able to accomplish in three years.

What actually may have happened is that the previous therapeutic effort succeeded in pushing the patient up the ladder toward the top, requiring only a bit more therapy to be able to take the last step that was accomplished by the second therapist taking advantage of the work achieved by the first.

I remember one patient who came to see me who advertised the fact that I had cured her in one session.

She had been under the care of physicians for years, suffering from a host of physical complaints, and her last few doctors, who were burdened by her incessant shifting symptoms, had given up on her as an obstinate hypochondriac.

No medicines seemed to help, and whenever psychiatric referral was mentioned she responded with an angry refusal and a host of new symptoms.

Finally, one day she announced in frustration to her doctor that she was ready to see a psychiatrist and the doctor then referred her to me.

At the appointed time a buxom, handsome, middle-aged lady walked into my office and from the moment the door opened started and continued talking without giving me a chance even to introduce myself.

My initial interview sheet in hand, I waited for a pause so that I could at least get some statistical data.

After what [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 79 seemed like an interminable span, she stopped for breath and I threw in an introductory: "By the way, how old are you?" Without pause she avoided the question and continued on her odyssey of voyages to doctors' offices around New York.

After several futile attempts to interrupt.

I gave up, put my sheet down, settled back in my chair and listened, interpolating an occasional "yes" and "hmm humm." I broke in at the end of the hour with: "Unfortunately our time is up." "Doctor," she said.

"I feel so much better—thank you very much," and she got up and walked out without making another appointment.

I was curious about what had happened, so that a week later, I telephoned her doctor.

His startling reply was: "I don't know what you did for her, but it was like a miracle.

For the first time since I've known her, she's lost her symptoms and is interested in getting out of the house and doing things.

And," he added humorously, "she says you are a brilliant conversationalist." Three months later the doctor called me and confirmed her continued improvement.

I do not know what happened but apparently my respecting her need to talk without interrupting was probably the best approach I could have used.

She was under such great tension that had I not permitted her to let off steam verbally I may have succeeded merely in frustrating and antagonizing her.

Grilling her with questions might have given me more information, but I am not sure what it would have done for her.

She apparently had climbed the readiness ladder by herself up to the top rung and what she needed to push her over the top was to have someone sit back and listen.

She undoubtedly wanted to talk freely to somebody who was not a member of her family, who was not going to judge her, who was passively objective, and who was, hopefully, nonjudgmental.

Whatever qualities she projected onto me, she utilized the relationship in line with her needs.

Obviously, while this passive stance worked well with this woman, it would not have served other patients who might have required more active confrontation over a more protracted period.

A person's readiness for change may, more than any other factor, be responsible for how rapidly movement proceeds in psychotherapy.

Degree and Persistence of Childish Distortions The distorted images of childhood, the

ungratified needs, the unwholesome defenses, may persist into adult life and influence the speed, direction, and goals of therapy.

These contaminations may obtrude themselves into the therapeutic situation irrespective of what kinds of technique are practiced.

Insidiously, they operate as [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 80 resistance and they can thwart movement toward a mature integration, no matter how persistent and dedicated the therapist may be.

For example, one patient, a successful businessman of 50, came to therapy in a crisis over abandonment by his mistress 25 years his junior who had run off with another man.

A deep depression and anxiety were the chief symptoms for which he sought help.

He blamed the young lady's defection partly on his inability to compete with his rival and partly on the ubiquitous duplicity of all females.

When he described his life, it became apparent that he had from the earliest days of adolescence looked for and pursued a certain physical type: blonde, fair skinned, long-legged, big-bosomed tarts whose teasing sexual provocativeness and irresponsibility added a fillip to his affairs.

He invariably would select young women who were unable to establish a meaningful relationship and who eventually, despite his wealth and generosity, would reject him and finally leave him.

This would make the relationship all the more precious in his mind and the rupture of the romance more disastrous.

During therapy he beat at himself, unmercifully proclaiming himself a worthless and destroyed person, a victim of the treachery of womankind.

No amount of reassurance, challenge, and interpretation could penetrate his overwhelming despair, and antidepressants proved of little benefit.

The fact that the great jewel in his life during the heyday of their relationship had led him a merry chase, deceiving and exploiting him mercilessly, made little difference.

Her destructive exploits constituted the main content of his discussions with me, but his depreciating her had little effect on his yearning.

After 12 sessions of gripes in therapy, the lady returned without warning, complaining that her most recent paramour was unworthy of her, unreliable and, most importantly, penniless.

She apologized for having hastily run off after knowing the man only several weeks and she begged the patient to take her back.

The effect on the patient was electric.

What common sense, medications, and psychiatry failed to do she accomplished in one evening. Depression, anxiety, and physical debility vanished.

To justify the reunion, the patient offered to bring the lady to me for an interview to prove that she was now reformed.

At the interview there was no question that she had been trading solely on her physical assets, which were indeed ample, but emotionally and intellectually there was much to be desired.

In fact, the most generous diagnosis one could bestow on her was that of a borderline case— and this was stretching a point.

It was certainly apparent insofar as my patient was concerned that the only motivation he had for therapy was a desire to dull the pain of deprivation, not to inquire into or eliminate its source.

My best efforts to halt the affair were of little avail since the lady could accomplish more with a casual pout than I could with all the armamentaria that Freud, Pavlov, and the other great pioneers had to offer.

And even though she soon again started her nonsense, he hung on desperately to the relationship at the same time that he bellowed like a wounded buffalo.

www.freepsychotherapybooks.org 81 In going into the history, the background for his enslavement became apparent.

The death of his mother when he was an infant and his placement with a series of relatives who provided him with a succession of nurses had failed to fulfill his need for real mothering.

He was told, he said, that he was a colicky baby with respiratory trouble that was diagnosed as asthma.

He failed to see any connection between his childhood and what was happening to him in the present.

Interpretations of his orally frustrated dreams fell on deaf ears.

One day on inspiration I asked if he had ever seen a picture of his mother.

This he denied.

However, a later search by a relative through an old album yielded a startling picture that he excitedly handed me—a picture of a blonde beauty who presented an almost exact image of his present girlfriend.

It took no great work to convince him that he had practically all his life been searching for a physical duplicate of his mother.

This dramatic discovery had not the slightest effect on his futile mission to look for a symbolic breast, because after another abandonment, he started searching for a substitute blond, long-legged, big-breasted, unreliable paramour, whom unfortunately he found, starting a further round of exploitation, punishment, and anxiety.

Where severe traumas and deprivations are sustained in early infancy, especially prior to the acquisition of language, the damage may be so deep that all efforts to acquire that which never developed and to restore what never existed will fail.

Transference with the therapist may assume a disturbingly regressive form and, while the genetic discoveries may be dramatic, the patient, despite intellectual understanding, will not integrate any learning and will fail to abandon patterns that end only in disappointment and frustration.

Very little can be accomplished under such circumstances in short-term therapy, and even long-term depth therapy may lead to nothing except a transference neurosis that is difficult to manage or resolve.

Lest we be too pessimistic about what may be accomplished through psychotherapy, there are some patients who, though seriously traumatized, may when properly motivated be induced to yield the yearnings of childhood and to control if not reverse the impulses issuing from improper discipline and unsatisfied need gratification.

But this desirable achievement will require time, patience and, above all, perseverance.

**APTITUDE FOR DYNAMICALLY ORIENTED PSYCHOTHERAPY** Practically all people who apply for help in managing emotional problems can be approached successfully with supportive and educational therapies.

Eligibility for www.freepsychotherapybooks.org 82 dynamically oriented treatment, however, requires some special characteristics.

Some of the available research indicates that patients who respond best to psychodynamically oriented therapy need treatment the least.

What this would imply is that persons with good ego strength can somehow muddle along without requiring depth therapy.

That this is not always so becomes obvious when we examine the quality of adaptation of these near-to-healthy specimens.

In view of the shortage of trained manpower, we may want to look for characteristics in prospective candidates for therapy that have good prognostic value.

The following positive factors have been emphasized: 1.

strong motivation for therapy (actually coming to therapy represents some commitment); 2. existence of some past successes and positive achievements; 3. presence of at least one good relationship in the past; 4. a personality structure that has permitted adequate coping in the past; 5. symptomatic discomfort related more to anxiety and mild depression than to somatic complaints; 6. an ability to feel and express emotion; 7. a capacity for reflection; 8. desire for self-understanding; 9. adequate preparation for therapy prior to referral; and 10. belief systems that accord with the therapist's theories.

The patient's expectations, age, and socioeconomic status are not too significant, provided the therapist and patient are able to communicate adequately with each other.

CHOICE OF GOAL AND FOCUS [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 83 If a patient through therapy expects to be a Nobel Prize winner, the patient will be rudely disappointed and soon lose faith in the therapist.

There are certain realistic limits to how much we can accomplish through treatment, the boundaries largely being determined by the patient's dedication to the assigned task. Added to these are the curbs imposed by the many therapist variables soon to be considered. A great deal of tact will be required in dealing with inordinate expectations so as not to undermine further the already existent devalued self-image.

The selection by the patient of the area on which to concentrate during therapy is a legitimate and understandable theater around which initial interventions can be organized.

It may not be the most culpable area stirring up trouble for the patient.

But to push aside the patient's concerns with a symptom or a disturbing life situation and insist on attacking aspects of problems the patient does not understand or is not motivated to accept will lead to unnecessary complications and resistances.

It is far better to work on zones of the patient's interest at the same time that we make connections for the patient and educate the need to deal with additional dimensions.

Thus a man in the manic phase of a bipolar disorder may complain of a marital problem and press for its urgent resolution.

Should we attempt to bypass his complaint factor and merely press lithium on him, we may be rewarded with an abrupt termination of treatment.

His manic symptoms may subside, but his marital troubles may continue to plague him.

In attempting to choose the most productive arena for intervention we must keep in mind the fact that behavior is a complex integrate of biochemical, neuro-physiological, developmental, conditioning, intrapsychic, interpersonal, social, and spiritual elements intimately tied together like links in a chain.

Problems in one link cybernetically influence other links.

Elsewhere I have delineated the affiliation between the different links, contingent fields of inquiry, associated therapeutic modalities, and related syndromes.

Without denigrating the importance of the patient's chosen area of focus, we may most propitiously deal with a link in the behavioral chain that, in our opinion, needs the most urgent attention, that the patient is willing to work on, and that is realistically modifiable with the patient's current or potential resources.

Once we strengthen a pathogenic link [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 84 through therapy, the effects will usually reverberate through the entire behavioral chain.

THERAPIST VARIABLES Personality Factors Observations of the determining influence of personality traits in the therapist on outcome have been repeatedly made and reported by

Whitehorn and Betz (1960), Betz (1962), Rogers et al, (1967), Truax and Carkhuff (1967), and Truax and Mitchell (1972).

One finding is that a relatively untrained person with a concerned manner and empathic personality will get better immediate results, especially with sicker patients, than a highly trained therapist who manifests a “deadpan” detached professional attitude.

One should not assume from this that a therapist with a pleasing personality without adequate training will invariably get good results.

Some of the available research alerts us to the fact that the level of therapist expectations and the triad of empathy, warmth, and genuineness do not invariably represent the “necessary and sufficient” conditions of effective therapy (Parloff et al, 1978b).

A well-trained therapist, however, who also possesses the proper “therapeutic” personality is by far best qualified to do successful therapy.

A good deal of the flesh and blood of what happens in treatment, short-term or longterm, is provided by the relationship the therapist establishes with the patient, the quality of which is largely influenced by the therapist's personality in operation.

Personality expressions, good and bad, come through not only in the content of verbal communications but in nonverbal manifestations.

The latter are not merely the epiphenomena of interaction but are directly related to the outcome.

Nor does the factor of experience always operate to subdue damaging traits.

In some cases earnest, dedicated beginners may relate better to patients than do more experienced, scientifically oriented, passive practitioners.

Of course, we must consider some other variables also since we may be dealing here with different classes of patients, i.e., patients seeking a warm, giving authority as contrasted to those who want less personal involvement and greater ability to probe for and resolve defenses in quest of more extensive self-understanding.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 85 After many years of training students and observing their work, I would estimate that the most meritorious personality traits sponsoring a good relationship are objectivity, flexibility, empathy, and the absence of serious emotional pathology.

Successful therapists possess a bountiful blend of these attributes, unsuccessful therapists a dearth.

Where a student therapist in training possesses a healthy combination of such positive personality characteristics, we may anticipate a good career, although this is not guaranteed.

Where a candidate is less bountifully blessed, but cherishes rudiments of essential traits, these may be maximized by careful instruction and personal psychotherapy.

A few enter training with such rigid defenses that they scarcely budge even after years of intensive analysis.

How to find candidates who personality-wise have a good chance of becoming competent therapists is a challenge confronting all training institutions.

In the early days of existence of the Postgraduate Center for Mental Health, I once asked Paul Hoch, who was then Commissioner of Mental Health of New York State, what he believed the value to be of recommendations for admission to training from a candidate's personal analyst.

“From my own experience,” replied Hoch waggishly, “when you first start treating or supervising students, your immediate impression is that they are practically psychotic.

But shortly thereafter you develop a relationship with them and you believe that they are only neurotic.

And a while later you start endorsing them as either completely normal, or even better than normal.” Could it be, I inquired, “that therapy changed their personalities for the better?”



"More likely," answered Hoch, "as a therapist works with a student, this changes the therapist's judgment for the worse." What he was referring to, of course, was the ubiquitous problem of countertransference that can mask or distort one's appraisal of a partner in a relationship dyad.

Other criteria for selection are undoubtedly more reliable than endorsements from one's personal analyst.

But the real test of how effective a candidate will be as a therapist is the actual performance with patients with varying syndromes and degrees of pathology, under the surveillance and tutelage of a competent supervisor.

Observation of a student's performance behind a one-way mirror and the use of videotape recordings are also of substantial help.

Specifically, especially where the patient requires rapid stabilization, the therapist's manner must convey empathy, confidence, and understanding of the patient's turmoil and what is behind it.

The patient, no matter how upset, will usually discern these qualities in the therapist and react to them.

It is important also that therapists be able to control their own difficulties sufficiently so as to avoid the pitfalls of their own countertransferential problems interlocking with the problems of their patients.

Particularly important is sensitivity to and the ability to manage irrational projections of patients, hallmarks of transference neuroses.

Considering that desirable character traits, if absent, are difficult to acquire even with personal therapy, it would seem important in the selection of candidates for training that some criteria be available to spot in advance students who possess or will be able to develop appropriate personality characteristics.

This is more easily said than done.

When we first founded the Postgraduate Center we experimented with many devices, including projective psychological testing and structured and nonstructured interviews.

We failed to come up with any foolproof selection procedure.

This is probably because the role that a candidate plays with an interviewer or psychologist tester is different from that assumed with patients.

My own experience convinces me that two personality qualities are especially undesirable in a therapist doing therapy: First and most insidious is detachment.

A detached therapist will be unable, within the time span of treatment, to relate to the patient or to become involved in the essential transactions of therapeutic process.

A detached person finds it difficult to display empathy.

To put it simply, one cannot hatch an egg in a refrigerator.

A cold emotional relationship will not incubate much change in treatment.

The second quality that I believe is inimical to doing good therapy is excessive hostility.

Where therapists are angry people, they may utilize select patients as targets for their own irritations.

A patient has enough trouble with personal hostility and may not be able to handle that of the therapist.

A therapist, exposed to a restrictive childhood, having been reared by hostile parents, or forbidden to express indignation or rage, is apt to have difficulties with a patient who has similar problems.

Thus the patient may be prevented from working through crippling rage by the subtle tactic of the therapist changing the subject when the patient talks about feeling angry, or by excessive reassurance, or by a verbal attack on the patient, or by

making the patient feel guilty.

However, a therapist who is aware of personal hostile propensities, who can be objective about these and willing to back down under a patient's challenge, may be able to do fairly good therapy, provided the patient is permitted to fight back, and is not too frightened to challenge and stand up to the therapist.

But where the therapist refuses to allow the patient this freedom, and gets upset and vindictive, rejecting or punishing the patient, therapy has a good chance of coming to a halt. By the same token, a therapist who has serious problems with sexuality may not be able to handle a patient who also has certain sexual impediments.

For example, a therapist struggling with a homosexual impulse, may, when a patient with homosexual tendencies brings up the subject, become defensive, overmoralistic, punitive, or so intensely interested in the topic that the patient will be diverted from constructively dealing with other important concerns.

All in all, we hope that in doing therapy the therapist will be slightly less neurotic than the patient.

The least we may expect is that the therapist will have a reasonable capacity for maintaining objectivity.

Some neurosis or character disturbance is probably residual in all of us, but this need not interfere with doing good therapy provided that we have an awareness of our failings and do not permit them to contaminate the therapeutic atmosphere.

One of my teachers, an analysand of Freud, once remarked: "If there ever were such a monster as a completely 'normal' human being, he would very soon get psychotic trying to adjust to the rest of us neurotics." "Normal" probably embraces a host of minor neurotic vexations, but where a therapist finds that good results are not forthcoming, some personal psychotherapy or analysis would seem indicated.

**Choice of Techniques** Technical preferences by therapists are territories ruled by personal taste rather than by objective identifiable criteria.

As has been previously indicated, it matters little how scientifically based a system of psychotherapy may be or how skillfully it is [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 88 implemented—if a patient does not accept it, or if it does not deal directly or indirectly with the problems requiring correction, it will fail.

Because of the complex nature of human behavior, aspects that are pathologically implicated may require special interventions before any effect is registered.

Prescribing a psychotropic drug like lithium for an excited reaction in a psychopathic personality will not have the healing effect that it would have in violent outbursts of a manic-depressive disorder.

Unfortunately, some therapists still cling to a monolithic system into which they attempt to wedge all patients, crediting any failure of response to the patient's resistance.

A young analyst, for example, one year after having completed her training in a classical psychoanalytic school, wanted to get some further training in hypnosis.

It turned out that her entire patient load was exactly the same one she originally had when she started her personal analysis some years previously.

All nine patients were being seen four or five times weekly, associating freely in the couch position, analyzing their dreams, but showing little or no improvement, some even regressing.

The therapist, maintaining the traditional passive stance, tried to listen to what they were saying, but found herself getting more and more bored and increasingly discouraged at what was happening.

She wanted to learn hypnosis, she said, to be able to get at the early memories of her patients, since few significant infantile revelations had been forthcoming.

Her patients simply were not coming to grips with their unconscious. Hopefully, hypnosis might be able to break through their resistance and bring to the surface the noxious memories and conflicts that were responsible for their complaints. In reviewing her caseload, one could see three obsessive patients—one with a germ phobia who avoided contact with people; the second, a compulsive handwasher who spent most of the day in the bathroom; the third, a salesman with an obsession of death and killing, who, when he came across the number 23, had to engage in elaborate counting and other rituals to neutralize torturous preoccupations.

In all of these cases analysis had succeeded merely in providing some answers regarding the sources and meaning of these symptoms.

The other six patients ranged from severe borderline cases who went over the border periodically, to various species of schizophrenia, with one man, an engineer, [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 89 actively hallucinating.

There was not a single case, in my opinion, that could be considered a suitable candidate for classical psychoanalysis.

And yet here was an earnest, dedicated professional digging away at their unconscious in the hopes of uncovering some mnemonic treasures that would ransom them from the prison of their past.

My un verbalized hunch was that all hypnosis would do for her stockpile of patients would be to add more fascinating imaginative data to the huge store of information that the therapist had already accumulated, without budging their afflictions a single whit.

To expose all of these cases to one technique would be like a surgeon who, because he specializes in appendectomies, removes the appendix in every patient who comes to him with stomachaches, abdominal pains, and diarrhea.

No one technique can serve to ameliorate all maladies that burden humankind.

This is the best argument for a balanced and conservative eclecticism toward which modern psychotherapy has gainfully been moving in the past decades.

But there are still a few diehards who, loyal to the traditions of their chosen theoretical school, try to force a circumscribed method on all patients.

Such a tactic, following an analogy once proffered by Freud, usually proves no more effective than trying to appease victims of a famine by passing out menus of a French cuisine.

For reasons difficult to justify, some earnest students are unable to break away from the strictures of cherished theoretical systems and virtually become trapped along with their patients in its ineffectualities.

The only way some can escape from their stagnant caseloads is by moving out of town!

To return to my student, another dilemma confounded her.

When she started working with her patients, most of whom were therapeutic failures referred by other therapists only too happy to get them out of their offices, she had, as a beginner, accepted them at a low fee.

Considering that they came four and five times weekly, and could scarcely afford paying the accumulated sum each month, the therapist could not, despite the economic havoc inflation imposed on her, bring herself to raise their fees to even the standard minimal level charged by colleagues of her rank.

This added to her dilemma and undoubtedly created resentments that I felt enhanced depression.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 90 My first task, prior to teaching her hypnosis, was to teach tactics of psychotherapy with sick patients.

We chose as our first prospect the obsessional patient with a germ phobia and a detached borderline case with masochistic fantasies.

She was to get them off the couch and allow them to sit up facing her.

She was to reduce their sessions to twice weekly and charge them the same fee monthly as before.

Most importantly, she was to stop acting like a “phantom therapist” by dropping her anonymity, with little digging and more relating.

Because she seemed bewildered at my unorthodoxy, I had to give her the exact words to say to her patients, which were to the effect that a point had been reached where it was no longer necessary to freely associate, where only two sessions weekly were needed, where only reality problems in the here-and-now were to be the focus.

She was enjoined to interest herself in what her patients were doing, smiling naturally, interchanging ideas and, if necessary, giving advice and support.

Curiously, the student showed no resistance to accepting my advice.

In two weeks a remarkable change developed in both the experimental patients and the therapist.

The patients, for the first time, spoke about how much better they felt.

And for good reason—they were relating to the therapist as a real person who was interested in them as people rather than as puppets of their unconscious.

The therapist found herself liking the patients, and her resentment resolved as the per session fee approximated that of her colleagues.

What occurred then was that she got all of her patients off the couch.

Sensing that she was losing her confidence in the analytic method, I had to work on her mistaken belief that the techniques I taught her were standard for all patients.

While they happened to be suited for the sick caseload she was currently carrying, they might not be right for other cases.

Indeed, classical psychoanalysis could be a boon for some patients carefully selected for the procedure.

Her supervision with me lasted one year, during which time she acquired new cases, one of which was a patient for classical analysis.

**Skill and Experience in the Implementation of Techniques** The history of science is replete with epic struggles between proponents of special conceptual systems.

Contemporary psychotherapists are no exception.

In a field as elusive [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 91 as mental health it is little wonder that we encounter a host of therapies, some old, some new, each of which proposes to provide all the answers to the manifold problems plaguing mankind.

A scrupulous choice of techniques requires that they be adapted to the needs and learning capacities of patients and be executed with skill and confidence.

Understandably, therapists do have predelections for certain approaches and they do vary in their facility for utilizing them.

Faith in and conviction about the value of their methods are vital to the greatest success.

Moreover, techniques must be implemented in an atmosphere of objectivity.

To function with greatest effectiveness, the therapist should ideally possess a good distribution of the following: 1.

Extensive training.

Training, in many parts of the country, has become parochial, therapists becoming wedded to special orientations that limit their use of techniques.

Accordingly, patients become wedged into restricted interventions and when they do not respond to these the therapeutic stalemate is credited to resistance.

Over and over, experience convinces that sophistication in a wide spectrum of techniques can be rewarding, especially if these are executed in a dynamic framework.

Whether a personal psychoanalysis is essential or not will depend on what anxieties and personal difficulties the therapist displays in working with patients.

The fact that the therapist does not resort to the discipline of formal analytic training does not imply being doomed to doing an inferior kind of therapy.

Indeed, in some programs, where the analytic design is promoted as the only acceptable therapy, training may be counterproductive.

Nevertheless, if a therapist does take advantage of a structured training program, which includes exposure to dynamic thinking and enough personal therapy to work out characterologic handicaps, this will open up rewarding dimensions, if solely to help resolve intrapsychic and interpersonal conflicts that could interfere with an effective therapeutic relationship.

Irrespective of training, there is no substitute for management under supervision of the wide variety of problems that potentially present themselves.

It is important that therapists try to recognize their strong and weak points in working with the various syndromes.

No matter how well adjusted therapists may be, there are some critical conditions that cannot be handled as well as others.

They may, when [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 92 recognizing which problems give the greatest difficulties, experiment with ways of buttressing shortcomings.

2.

Flexibility in approach.

A lack of personal investment in any one technique is advantageous.

This requires an understanding of the values and limitations of various procedures (differential therapeutics), experience in utilizing a selected technique as a preferred method, and the blending of a variety of approaches for their special combined effect.

Application of techniques to the specific needs of patients at certain times, and to particular situations that arise, will require inventiveness and willingness to utilize the important contributions to therapeutic process of the various behavioral sciences, accepting the dictum that no one school has the monopoly on therapeutic wisdom.

**SOCIAL AND ENVIRONMENTAL VARIABLES** Anyone who believes that the innate lenity of humankind transcend some of the abuses and indignities that society heaps on one is a victim of Utopian self-deception.

Social and environmental variables are probably the most neglected of factors in psychotherapy and sometimes among the most important.

If in doing therapy we do not consider the environment in which the patient will have to live and function, we will run the risk of annulling therapeutic gains.

An environment that does not support and encourage the patient's newly developed patterns or that punishes the patient for their constructive behavior will tend to reverse the gains brought about by the therapeutic process.

On the other hand, an environment that rewards for constructive behavior will reinforce therapeutic gains.

Treatment may be considered incomplete if it does not prepare the patient for contingencies that will have to be faced when treatment is over.

Adolescents who belong to gangs, for example, who learn to control delinquent behavior, may find themselves rejected by their peers for abiding by the law.

A young adult living at home under the yoke of domineering parents may not be permitted to assume an independent role after the therapeutic resolution of a pathological dependency drive.

An alcoholic helped to give up drink may not be able to remain dry so long as membership in a wine-tasting club continues.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 93 During therapy a thorough review of what the patient will be up against after termination will be urgently needed.

Either the patient will have to modify a destructive environment, if this is possible, or will

have to separate from it.

Thus the adolescent and the alcoholic will need to find new friends.

The young adult will have to get a job and take up residence in a more permissive atmosphere, that is, unless parents are willing to enter into family therapy and respond sufficiently to permit greater freedom.

Too frequently it is assumed that the patient will somehow get along once the treatment sessions have ended.

Because the environment will rarely take care of itself, its future impact on the patient must be studied as part of the treatment program.

**TRANSFERENTIAL AND COUNTERTRANSFERENTIAL VARIABLES** In therapy the initial positive relationship often serves during the first few sessions to quiet the patient's tensions and temporarily to restore a sense of mastery.

A good deal of the responsibility for this happening resides in the patient's need for an omnipotent idealized authority, which urgency is projected onto the therapist.

Some therapists advocate ending treatment abruptly when the patient has achieved a windfall of symptom relief, encouraging the patient to resume the customary threads of life and providing some awareness of the circumstances that contributed to this present disorder.

Where therapy is terminated after a few sessions the patient may continue to retain the initial image of the therapist as a powerful, benevolent and perhaps magical figure, having utilized this image, however unrealistic it may be, as a vehicle for restoring customary stability.

Where the therapist is more ambitious, or the patient requires more sessions to get well, around the eighth session a change often occurs in the image of the therapist that can precipitate a crisis in treatment.

The patient begins to realize that the therapist is not a god, does not have all the answers, and even possesses feet of clay.

This disillusionment may exhibit itself in a forceful return of symptoms, and a crediting to the therapist of ineffectual or evil qualities.

They draw their substance from a deep reservoir of fear and hate into which malevolent attitudes toward authority, some dating to childhood, have been stored.

This transference pollution may go on unconsciously and be manifested

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 94 solely in dreams or acting-out away from the therapeutic situation.

The only sign the therapist may detect from the patient's manifest behavior is in the form of resistance to treatment.

The patient will complain about not getting well while breaking appointments or coming late for sessions and will delay the payment of bills for therapy.

A dynamically oriented therapist searches for transference signs realizing both the potential for helping the patient resolve some of the deepest problems through the insightful understanding of transference, and the destructive effect that unrecognized capricious transference can have on the therapeutic process.

A therapist who has no awareness of transference will be truly handicapped in managing patients whose reactions to treatment become paradoxical or inappropriate.

Understandably, the more serious the early problems with authority have been, the more likely will transference become apparent, and the more tumultuous its manifestations.

Where the therapist shares the initial illusions with the patient to the effect that he or she is a demigod, changing into a devil will be highly disturbing.

Such therapists may try to avoid trouble by confining themselves to the briefest forms of short-term therapy, terminating all treatments before the sixth session.

But sometimes this does not help, especially in vulnerable borderline or schizophrenic patients

who develop transference reactions toward the therapist even before seeing the therapist at the initial interview.

It is far better that professionals who wish to do good therapy work through their godlike image, if it is at all resolvable, by themselves or in personal therapy.

At any rate, a good degree of stability is required on the part of the therapist in order to handle transference reactions when they occur.

An unstable therapist finds it difficult, because of countertransference, to control responding negatively when challenged or unfairly accused by a patient in transference.

Impulsively discharging or furiously cowing the patient into submission will obviously rob the patient of an opportunity to work through problems with authority.

Whether personal psychotherapy is mandatory for all therapists in training as a [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 95 means of preventing obstructive countertransference is a question about which there is much debate.

If one is a good therapist, personal therapy will probably help make one a better therapist but will not accomplish miracles.

There are certain problems that are so deep that personal therapy may not be able to budge them.

For example, intense childish distortions developed in very early infancy may resist correction.

The therapist may get an awareness of these distortions through personal therapy, yet be unable at times to control their surfacing.

Nevertheless, therapy may have enabled the management of reactions sufficiently so that they do not interfere too much with functioning with patients.

Personal psychotherapy will accomplish its mission if it can control the therapist's use of the patient for personal designs and projections.

Destructive countertransference is probably present to some extent in all therapists, but this need not necessarily be hurtful if the therapist is aware of its presence, recognizes how it is manifesting itself, and takes steps to resolve it.

Countertransference is, of course, not always harmful.

It may alert therapists to traits and maneuvers of the patient that arouse important feelings in other people.

The important thing is how therapists utilize countertransference toward bringing out the unconscious needs and conflicts of the patient while describing the effect they can have personally as well as on others.

The patient may learn something important from this.

For example, a great deal of tolerance is required on the part of the therapist in adjusting to the habits of some patients.

Most therapists are punctual in appointments (as they should be) even though certain patients are lax in appearing promptly at the scheduled hour.

These patients are prone to subject the therapist to a bit of delinquency as a vehicle of testing or defiance.

The therapist may particularly be irritated by patients who come to clinics, paying little or nothing yet seeming unappreciative of what is being done for them.

There is no reason why the therapist should not focus on the patient's offensive behavior, not as a way of reprimanding the patient, but to clarify the meaning of what is going on.

There are certain patients we relish working with and others who are less than a joy to treat.

There are therapists who are completely unable to handle adolescents while others do their best work with young people.

Schizophrenic, violent, obsessional, paranoidal, psychopathic, hypochondriacal, suicidal, and

delinquent patients stimulate aversive [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 96 responses in many therapists.

Yet other therapists not only tolerate these syndromes but enjoy handling them.

Countertransference elements encourage therapists to project onto patients aspects of their own inner needs of which the therapists are partly or wholly unaware.

One therapist whom I was supervising in a class reported that his patient was getting progressively more depressed.

An audio cassette of a session brought out a repetitious theme voiced by the patient that nobody cared for her, nobody paid attention to her, nobody liked her; that she was the neglected child in the family whose destiny was to spend her life in misery as an isolate.

She insisted that she could never command respect or attention from anybody.

She went on and on in this depressive vein, and the therapist from his conversation seemed to be responding correctly to what the patient was saying.

We decided then that the therapist should interview the patient behind a one-way mirror with the class observing.

The therapist was instructed to set up the furniture similar to the arrangement in his office.

When the patient entered the room she seemed fairly animated, but as she talked she appeared to get progressively more and more depressed, the content of her verbalizations centering around feeling rejected by her family, by people, and by the world.

What was startling was that the therapist, without realizing it, was actually playing into her theme of rejection.

He had placed his chair at an angle so that he did not face her, and while he would from time to time fire an interpretation at her, he was constantly busy writing or looking away toward the opposite corner of the room.

Periodically the patient would glance at the therapist, who by all appearances was off in space.

One got the impression that she was being treated like a scientific specimen, not like a needy human being.

Having been reared by a schizoid mother who in later life was admitted to a mental institution, she undoubtedly interpreted the therapist's manner as rejection.

The interesting thing was that a sophisticated professional in his last year of postgraduate training did not realize that he was providing the patient with a stimulus that activated her habitual rejection theme.

On questioning, the therapist admitted that he was losing interest in the patient because she was beating at him constantly with her griping and complaining and getting no better.

He was unaware that the placement of his chair was a gesture that signaled his disinterest, nor did he realize that the sessions were as traumatic for the patient as they were for him.

He was sufficiently advanced in his training and personal analysis to explore his feelings toward the patient and affiliate them with attitudes toward a hypochondriacal mother who drove his own father to distraction.

In this way his countertransference interlocked with the transference projections of the patient.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 97 What the patient was doing with her therapist she did with all people with whom she became intimate.

She expected rejection so much that in testing their sincerity she did exactly the things that resulted in her being rejected.

People then responded by avoiding her.

I suggested that the next time the patient came for a session the interview be conducted face to face.



The therapist was to put aside his pad and just talk about the patient's interests and experiences without probing her feelings—in other words to work on building a relationship. In a very short time the whole nature of the therapy sessions changed. The patient became livelier, more interested in what she was doing, and more able to joke and smile.

And the therapist developed greater enthusiasm about the patient. Eventually the patient's depression lifted and the patient was able to manage the termination phases of therapy without too great difficulty.

Not all of a therapist's reactions are countertransferential. They may be prompted by deliberate, destructive, and outlandish conduct. I recall one patient whose behavior was so provocative as to challenge my capacity for disciplined objectivity.

It was all I could do during some sessions when she acted particularly nasty and insulting to stop myself from responding defensively or punitively.

The patient was a married woman in her late thirties who came to me not of her own free will but because of the pressures imposed on her by her friends and family, who realized that she was seriously depressed and disturbed.

She was the only child of a wealthy couple who adulated, pampered, and spoiled her so that she soon ruled the household like a tyrannical princess with an iron fist without the traditional velvet glove.

Screaming tantrums forced her parents and private tutors to yield to her slightest whim. When she grew up she transferred these tactics to people around her, responding to not getting her way with violence, headaches, and paranoid-like projections.

Her marriage, she revealed, started off sizzling on a King-sized bed. But soon, after she had succeeded in verbally whip-lashing her husband into partial impotence, the couple retreated into twin beds, and, following the birth of her two children, they sought refuge in separate rooms from which they sauntered out to combat.

Added to this the inanities of suburban life were more, she claimed, than human flesh could endure.

Her initial contacts with me were organized around exploratory maneuvers to determine how much she could win me over and manipulate me.

Interpretations fell on deaf ears.

She was certain that I was siding with the enemy at home who blamed her for the prevailing mess she was in.

Hostility was expressed in subtle and not so subtle ways.

On one occasion, she asked me to refer her to the best [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 98 dermatologist in town.

She appeared at his office with her dog whom she brought into the consulting room. It turned out that she wanted treatment not for herself but for her dog "who deserved the best." The dermatologist, who was a dear friend of mine, winked to his nurse and they proceeded to put the dog on the examining table and to treat him like a regular patient, right to the rendering of a prescription with the dog's name on it.

Fortunately, the doctor, a dermatologic authority, had a great sense of humor and he went along with the "gag." On another occasion, being more careful to explore her complaint of backaches, I referred her to an orthopedic surgeon who on walking through the waiting room found her sitting in a chair with her feet on a new expensive coffee table.

In not too gentle tones, he commanded her: "Won't you take your feet off my table?" Haughtily she turned on him with "It took me four months to hate Wolberg.

You I hate right away." With this as a background, I want to describe an incident where my loss

of objectivity resulted in a significant therapeutic gain.

During an interview, as she sat facing me, I confronted her with her responsibility in promoting a quarrel with her best friend.

Furiously the patient removed a diamond ring from her finger and fired it at me.

As the ring, a huge eight-carat gift from her father, whizzed by my ear, I tried to conceal my surprise and dismay by acting nonchalant and by not commenting on her behavior.

I could see that she was nonplussed and irritated by my lack of concern.

"Give me back my ring," she commanded imperiously.

"You threw it," I replied, "You find it." After several such exchanges, she stormed over to my chair and began to search for the ring.

It was nowhere in sight!

"You better find my ring," she shrieked, but she got no response from me.

However, after several minutes had passed in futile search, the ring remained undiscovered, and I leisurely proceeded to help her.

But the ring was nowhere to be seen.

By this time, I too was concerned.

Yet a minute search of the room produced nothing.

The patient burst into tears and I then tried to reassure her, utilizing the incident to accent my previous interpretations that her loss of temper hurt her more than it did other people.

After the patient left my office ringless, I went over the room minutely and finally I found the ring, which apparently had fallen on the couch and bounced off to the side, becoming wedged in between the mattress and the wooden side.

The patient was relieved at my telephone call and thanked me.

My victory, however, was short-lived.

The next day when the patient came for her ring she burst angrily into my office and slammed the door shut with her foot, registering a dirty footprint on my newly painted door.

I could feel my anger bubbling up.

"You wash that footprint off that door," I ordered.

"Ha, ha, ha," she retorted defiantly, "You make me." Reflexively, I grabbed her by the back of her neck and marched a frightened patient into the bathroom, stuck a wet soapy washcloth in her hand, and firmly marched her back to the door.

She obediently washed the door, then quietly sat down; then we had our first constructive talk.

She acted contented and even smiled.

What the patient seemed to have done was to force me to set limits on her behavior that her father had failed to do.

What she wanted and needed was some discipline.

The positive effect on our relationship was amazing and we were able to achieve changes in her life that earned for me the gratitude of the patient and her family.

If through the relationship the patient is able to modify the introjected image of authority, the therapeutic process will have scored a great gain.

Such modification comes [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 99 about by a replacement of the patient's imprinted authority figure, which is often harsh or overprotective, or negligent, or distorted, with a new, more rational and constructive figure as vested in the therapist.

An opportunity for this may come about through transference on the therapist of feelings or attitudes that relate to the authoritative introject.

Manifestations of the transference appear in direct or disguised form, in oppositional resistance to the therapist or to the techniques being employed, in unreasonable demands for favors or affection, in fantasies or dreams, or in acting-out away from treatment with persons other than the therapist.

The ability of the therapist to recognize transference when it appears, particularly in its disguised forms, and to deal with it through interpretation and proper management of the relationship will have a determining effect on the direction and results of treatment.

RESISTANCE VARIABLES Shorr (1972) cited Saul Bellow, who in *Herzog* describes the common resistance to normality so often encountered in therapy: "To tell the truth, I never had it so good ... but I lacked the strength of character to bear such joy.

That was hardly a joke.

When a man's breast feels like a cage from which all the dark birds have flown—he is free, he is light.

And he longs to have his vultures back again.

He wants his customary struggles, his nameless empty works, his anger, his afflictions, and his sins." Paradoxically, some people are loath to give up the very chains that bind them to neurotic slavery.

Reluctance to accept normality is merely one of the many resistances that precipitate out in the course of therapy.

Some resistances, inspired by lack of motivation and refusal to give up a stereotype of a nonrealistic therapist, occur at the start of therapy.

Some, such as transference resistances, convert the therapeutic alliance into a battlefield of archaic projections and interfere with the treatment process itself.

Others, like regressive dependency, mobilize anger and grief, and obstruct proper termination of therapy.

Resistances can take many forms, often following defensive maneuvers customary for the individual.

Thus patients may become evasive or forgetful, breaking, cancelling or [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 100 coming late for appointments, or they may engage in prolonged silences during sessions.

They may indulge in superficial, rambling talk.

They may try to disarm the therapist with praise, or become aggressive, argumentative, and accusatory.

Women may become sexually seductive toward their male therapists and men toward their female therapists.

Destructive as they are, certain resistances protect the individual from catastrophic helplessness and anxiety.

They are means of preserving important neurotic coping mechanisms.

A phobia, for example, may disable the individual, but it still has a protective quality.

A hysterical arm paralysis can shield the individual from awareness of murderous impulses, while blandly protesting the inconvenience of being unable to utilize the limb.

Frigidity may mask overwhelming fear in a woman of assuming a feminine role.

In all of these cases the yielding of important defenses promises exposure to dangers far greater than the torments the patient already suffers.

Moreover, certain secondary gains of a positive nature may accrue to the indulgence of a neurosis.

In industrial accidents the victim who is on disability payments may in giving up pain and physical illness lose not only financial security, but also sympathy, freedom from responsibility, and the opportunity of occupying center stage with repetitive tales of what was endured at the hands of doctors.

Having been referred to a therapist by an insurance company, which insists on the victim getting treatment, or brought in by family who tire of complaints, the victim is exposed to the threat of health, which is a barren bounty compared to the advantages of disability.

Experience with large numbers of patients convinces that three common dynamic problems most often initiate emotional difficulties and also create resistance to psychotherapy.

They are: 1.

inadequate separation-individuation; 2.

a hypertrophied sadistic conscience; and 3.

devaluated self-esteem.

These are never isolated units.

Rather, they coexist and reinforce each other, and [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 101 they create needs to fasten onto and to distrust authority, to torment and punish oneself masochistically, and to wallow in a swamp of hopeless feelings of inferiority and ineffectuality.

They frequently sabotage a therapist's most skilled treatment interventions, and when they manifest themselves, unless dealt with deliberately and firmly, the treatment process will bog down in a stalemate.

The most the therapist may be able to do is to point out evidences of operation of resistance saboteurs, to delineate their origin in early life experience, to indicate their destructive impact on the achievement of reasonable adaptive goals, to warn that they may make a shambles out of the present treatment effort, and to encourage the patient to recognize personal responsibility in perpetuating their machinations.

The frightening hold a self-devaluating resistance can have on a patient is illustrated by the following fragment of an interview.

The patient, a writer, 42 years of age, who made a skimpy living as an editor in a publishing house came to therapy for depression and for help in working on a novel that had defied completion for years.

Anger, guilt, shame, and a host of other emotions bubbled over whenever he compared himself with his more successful colleagues.

He was in a customarily frustrated, despondent mood when he complained: Pt.

I just can't get my ass moving on anything.

I sit down and my mind goes blank.

Staring at a blank piece of paper for hours.

I finally give up.

Th.

This must be terribly frustrating to you.

Pt.

(angrily) Frustrating is a mild word, doctor.

I can kill myself for being such a shit.

Th.

You really think you are a shit?

Pt.

(angrily) Not only do I think I am a shit, I am a shit and nobody can convince me that I'm not.

Th.

Frankly, Fred, I'm not even going to try.

But you must have had some hope for yourself; otherwise you never would have come here.

Pt.

I figured you would get me out of this, but I know it's no use.

I've always been a tail ender.

Th.

(confronting the patient) You know, I get the impression that you've got an investment in holding on to the impression you are a shit.

What do you think you get out of this?

Pt.

Nothing, absolutely nothing.

Why should I need this?

Th.

You tell me.

(In his upbringing the patient was exposed to a rejecting father who demanded perfection from his son, who was never satisfied with his even better than average marks at school, who compared him unfavorably with boys in the neighborhood who were [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 102 prominent in athletics and received commendations for their schoolwork.

It seemed to me that the paternal introject was operating in the patient long after he left home, carrying the same belittling activities that had plagued his existence when he was growing up.) Pt.

(pause) There is no reason, (pause) Th.

You know I get the impression that you are doing the same job on yourself now that your father did on you when you were a boy.

It's like you've got him in your head.

(In the last session the patient had talked about the unreasonableness of his father and his inability to please his father.) Pt.

I am sure I do, but knowing this doesn't help.

Th.

Could it be that if you make yourself helpless somebody will come along and help you out: (I was convinced the patient was trying to foster a dependent relationship with me, one in which I would carry him to success that defied his own efforts.) Pt.

You mean, you?

Th.

Isn't that what you said at the beginning, that you came to me to get you out of this thing?

You see, if I let you get dependent on me it wouldn't really solve your problem.

What I want to do is help you help yourself.

This will strengthen you.

Pt.

But if I can't help myself, what then?

Th.

From what I see there isn't any reason why you can't get out of this thing—this self-sabotage. (The patient responds with a dubious expression on his face and then quickly tries to change the subject.) In the conduct of treatment one may not have to deal with conflicts such as those above so long as the patient is moving along and making progress.

It is only when therapy is in a stalemate that sources of resistance must be uncovered.

These, as has been indicated are usually rooted in the immature needs and defenses inspired by dependent, masochistic, self-devaluating promptings.

At some point an explanation of where such promptings originated and how they are now operating will have to be given the patient.

This explanation may at first fall on deaf ears, but as the therapist consistently demonstrates their existence from the patient's reactions and patterns, the patient may eventually grasp their significance.

The impulse to make oneself dependent and the destructiveness of this impulse, the connection of suffering and symptoms with a pervasive need for punishment, the masochistic desire to appease a sadistic conscience that derives from a bad parental introject, the operation of a devalued self-image, with the [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 103 subversive gains that accrue

from victimizing oneself, must be repeated at every opportunity, confronting the patient with questions as to why he or she continues to sponsor such activities.

When we consider the many patient, therapist, environmental, transference and resistance variables that have been described above, and that are parcels of all therapies, irrespective of type, it becomes apparent that empirical research into their effects may do a great deal in promoting more effective practice and in advancing psychotherapy to its rightful place in the family of scientific methodologies.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 104 6 Educational, Casework, and Counseling Approaches Versus Psychotherapy Educational, casework, and counseling approaches all aim at an expansion of the potentialities of the individual and a betterment of adjustment.

In this way they pursue some of the goals of psychotherapy.

They possess, however, certain distinctive features that should be differentiated from psychotherapy to avoid a confusion of role and function among the disciplines identified with these methods.

**EDUCATIONAL APPROACHES AND PSYCHOTHERAPY** Education has been defined as a process of “progressive or desirable changes in a person as a result of teaching and study” (English & English, 1958) and “the systematic instruction, schooling or training given to the young [and, by extension, to adults] in preparation for the work of life” (Oxford Univ. Dictionary, 1955).

Even though education has focused on the development of the intellectual capacities of the individual and the acquisition of knowledge, a new direction in education has been fostered by the recognition of the vital significance of human relationships in the learning process.

The purpose of education has been extended to include the emotional growth of the individual and more constructive relationships with people.

This progressive movement within the field of education, founded by John Dewey, “emphasizes the needs of the individual and the individual’s capacity for self-expression and self-direction” (Hinsie & Campbell, 1960).

The discipline of education has accordingly broadened its goals to include “the inculcation of social attitudes,” “the development of social sensitivity,” and the evolution of “better personal-social adjustment” (Smith & Tyler, 1942; Trecker, 1946).

Educational objectives have been widened to foster personal security, and to expand assertiveness and self-direction.

A principal aim in education “is to provide rich and significant experiences in the major aspects of living so directed as to [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 105 promote the fullest possible realization of personal potentialities” (Giles et al, 1942).

More and more schools are including in their curricula material “concerned with the total personality—not merely with the intellect but with emotions, habits, attitudes.

...

It [general education] consists of preparation for efficient living, no matter what one’s vocation” (N.

Central Assn., 1942).

Among the most interesting experiments along this line were those of the Bullis, Force, Ojemann and Forrest Hill Village projects (Comm.

Prev.

Psychiatry, 1951).

An additional factor has been recognized to the effect that emotional blocks may prevent the student from accepting or integrating educational offerings.

The traditional pedagogic techniques are usually unable to handle such impediments.

Interviewing and group work methods that are continually evolving in educational circles

(Cantor, 1946; Slavin, 1950) attempt to deal with learning blocks.

Both in objectives and techniques, some of these procedures resemble those in psychotherapy. The question, however, still has not been answered satisfactorily as to whether these techniques are forms of psychotherapy or pure aspects of educational intervention. Behavior modification, for example, dealing as it does with habit, learning, and adjustment and practiced in schools, hospitals, prisons, and clinical and community settings, appears to fall into both areas.

The confusion perhaps lies in the fact that therapeutic techniques often turn out to be educational for the patients in that they discover new things about themselves and learn different ways of behaving.

On the other hand, education can register a significant psychotherapeutic impact on some individuals, helping them to control their symptoms and fostering a better life adaptation.

The success of education in promoting the development of growing and plastic personality structures of children has aroused hopes in utilizing educational approaches toward reshaping attitudes, altering values, reorganizing feelings, and refashioning behavior.

Recent years have witnessed the popularization of mental health concepts in current periodicals, books, movies, radio, and television.

Under certain conditions the simple imparting of the proper kind of mental health information has geared the recipient [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 106 toward a more wholesome adjustment.

Such simple concepts as that an infant requires the loving presence of a mother and adequate stimulation during the first year of life, or that rebelliousness during adolescence is

universal and perhaps a necessary component of the child's reaching for independence, may soften parental anxiety and alter ways of handling problem situations in both parent and child.

Attempts are being made also to educate persons in more wholesome attitudes toward themselves, their families, and the community.

Materials are available that are intended to ameliorate the effects of emotional illness on adjustment.

This "mental health education" or "psychoeducation" in which there are promulgated precepts of normal personality growth, of psychodynamics and psychopathology, data on the health, adjustment, social and economic hazards of emotional illness, and details on the adoption of mental health information to various problems in the community has sponsored a profession of "Mental Health Educator." The impact on the public has in general been a questionable one, even though the substance of what is taught and the communication techniques have been correct.

The widespread preoccupation with psychological concepts has introduced a flood of writings and lectures formulating doctrines on how to regulate one's life most efficiently.

The intent is partly to educate people into the proper handling of their relationships, and partly to mediate their personal conflicts.

The effect of such writings and lectures is difficult to assess.

Where they do not oppose basic defenses, the person may integrate the teachings with a reeducational result.

Where they conflict with basic defenses, they may either wield no influence whatsoever or inspire guilt feelings induced by a realization that one is unable to abandon attitudes or patterns of behavior potentially hurtful to oneself or others.

Group seminars oriented around discussion and clarification are much more effective than writings, provided that they are headed by a skilled group leader.

Given the proper motivation, adequate educational media, a congenial setting, an intelligent educator who inspires respect and confidence, and a friendly group bent on [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 107 learning objectives with which the individual can identify, an individual may acquire a prodigious amount of knowledge.

Some change may also be expected in patterns of adjustment as a by-product of the learning experience, principally due to the relationship with the educator and the group.

There are limitations, however, as to how extensively attitudes, values, feelings, and behavior, in which the individual has a deep emotional investment, may be influenced.

Although these may seem senseless and are in opposition to a reasonable and happy adjustment, the individual continues to adhere to them with bullheaded persistence.

They are impervious to logic, common sense, and scientific argument.

The obese young woman, bloated with avoirdupois, may be better versed in the rationale and methods of calorie control than a trained dietician.

She knows every reason why she must diet, yet she is unable to stop stuffing herself with food.

The man with a recent coronary attack has read the latest antitobacco literature, yet he invites devastation by chain smoking.

The irate business executive, constantly disciplined by her superiors for her spleen, and in spite of a brush-up course in public relations, continues to attack her subordinates and to alienate her peers.

The promising young salesperson with a flying phobia has learned all about the safety of aircraft, yet prefers the risks of bus travel to the ease and comfort of a jet plane.

These are but a few examples of how limited an educational approach may be in dealing with patterns that reflect deep-seated fears and needs.

Educational techniques are valuable when the individual is able to countenance and to absorb the content they communicate.

They are a parcel of all good psychotherapies.

But they are unable in themselves to overcome resistances to change, nor can they readily handle the anxieties that inspire the resistances.

They usually deal with dimensions peripheral to those that have initiated the individual's neurotic disturbance and that continue to activate that person's mechanisms of defense.

In summary, while educational approaches are important and useful, they are no substitute for psychotherapy.

They have a growth potential, but one should not overestimate their impact on firmly conditioned and neurotically structuralized [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 108 behavioral patterns.

**CASEWORK AND PSYCHOTHERAPY** Psychiatric social work traditionally "is social work undertaken in psychiatric agencies and mental health programs." It has as its aim a contribution "to those services and activities within the community which promote mental health and are conducive to the restoration of the health of individuals who are suffering from mental and emotional disturbances." The social work process employed to promote this aim is "social casework," which is applied in the "identification, diagnosis, and treatment of persons with personal and social maladjustments caused or aggravated by mental and emotional problems" (Knee, 1957).

By means of a "person to person helping relationship through individual interviews or group process, the social worker can assist the individual to determine and resolve specific problems in the environment and interpersonal relationships which interfere with adequate functioning" (Am.

Assn of Psychiatr.

Soc.

Workers, 1955).

Among the varied services of the social worker are those that deal with family social work, family life education, adoption, child welfare, foster care for children, day care, homemaker activities, legal aid, public assistance, school social services, youth services, community organization and coordination, rehabilitation, protection of rights, and correctional work



involving the broadest aspects of law enforcement, detention, probation, parole, and crime control and prevention.

Problems in employment, housing, education, living arrangements, finances, recreation, and health come under the social worker's aegis.

In the course of work the social worker must make contact with a wide range of humanity, including the physically handicapped, mentally ill, mentally retarded, indigent, chronically ill, alien and foreign born, aged, unmarried mothers, alcoholics, juvenile delinquents, criminals, drug addicts, and generally unhappy and maladjusted individuals.

The enabling, problem-solving process the social worker performs to help individuals and their families resolve social difficulties they are unable to manage by themselves is "casework."

With the expansion of government responsibility for financial assistance, private family agencies particularly have employed casework in order to [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 109 counsel family members with reference to intrafamilial relations, so that marriage counseling, counseling of parents regarding their children, and counseling of adults regarding their aged parents constitute a substantial part of work in a family agency.

Conventionally, the trained worker has executed these functions through the rendering of services of which the client is in need.

The focus has been on the external problem or social situation and not, as in psychotherapy, on the individual's inner distress or illness.

In rendering social services, no deliberate attempt is made to alter the client's basic personality patterns, the object being to handle situational problems on a purely realistic level.

This classic role in casework has, during the past decades, undergone considerable modification.

Operationally, caseworkers have found it difficult or impossible to limit their area of work to the client's external life situation.

Indeed, a conviction has evolved that, unless certain capacities are mobilized or developed in clients, they will be unable to utilize the social services offered or the community resources made available to them toward a better life adjustment.

Consequently, much more extensive goals have developed in the practice of casework than are implicit in the early definition.

This broadening of objectives was largely the product of psychiatric influence, particularly that of Freud and Rank, and of better understanding of the use of sociologic theory and techniques.

Thus, Towle (1947) described the caseworker as one who handles persons "experiencing some breakdown in their capacity to cope unaided with their own affairs." This breakdown, she added, may be caused by external factors, "or it may be partially, largely, or entirely due to factors within the individual." Services may be geared toward reality needs or may be "oriented to feelings and to ways of responding." Bowers (1951), reviewing definitions of casework, adds this one: "Social casework is an art in which knowledge of the science of human relations and skill in relationship are used to mobilize capacities in the individual and resources in the community appropriate for better adjustment between the client and all or any part of his total environment." Other definitions are (1) "casework is a method of helping a troubled person to [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 110 understand what is causing his personal or family problems and to find inside himself, in the home, or in health and welfare agencies the resources to rebuild his or her family's life" (U.S.

Soc.

Sec.

Adm.

1949) and (2) casework is “the function of professional [social] workers who, through social services and personal counseling, attempt to help individuals and their families improve their personal and family adjustments” (Hinsie & Campbell, 1960).

When we examine these definitions carefully, we find that some of the goals toward which casework is directed are identical with those in our definition of psychotherapy.

Since the casework process involves an interpersonal relationship, similar emotional phenomena develop between the caseworker and client as occur between a psychotherapist and patient.

The relationship has often served to release forces within the individual that are essentially psychotherapeutic in effect.

But can we say the casework process is psychotherapy?

Some authorities insist that it is and that “when a psychiatric social worker says she is doing case work, within the interviewing room, she does psychotherapy” (Grinker, 1961).

Davidson cites Rennie’s account of the casework process: “People who have been interviewed by caseworkers ... say that they like to be talked with in this way; that they gained insight, came to understand some of the reasons for their problems, and got some inklings of the way out of their difficulty... .

The caseworker observes moods, hesitations, and emotions ... expressed in subtle ways.

He tries to trace problems to their roots.

He accepts the person as he finds him, without blame.... Often the client’s anxiety is drained off and his hostility and guilt are lessened.

He is freed from the blinding effects of these emotions and sees, for the first time, psychological connections of which he had formerly been unaware.” Davidson adds, “Now here, in the words of a distinguished psychiatrist, is a definition of casework— and it’s as good a definition of psychotherapy as you can find” (Perlman, 1960b).

When we examine the effects of casework, and scrutinize the dynamics of the caseworker-client relationship, we find a number of similarities to those of psychotherapy.

There are, however, differences.

First, the casework process is not geared toward the resolution of emotional problems as such, but rather toward a bringing of the [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 111 client to a recognition of those problems that interfere with the client’s social adjustment and utilization of services.

“The particular purpose of social casework is to help people who are suffering from some impairment or breakdown in their adequate social functioning and to restore, reinforce, or enhance the performance of their daily lifetasks” (Perlman, 1960b).

Second, resistances to the use of services, and fears and anxieties about changes that are occurring in the life situation and patterns of relatedness, are managed on a realistic level with active guidance and advice giving.

Interpretation of unconscious conflicts and a delineation of the origin and minute operations of mechanisms of defense are avoided.

Third, the usual education of the social worker provides counseling procedures, but not the more extensive psychotherapeutic techniques.

While there is general recognition of the fact that psychological disturbances are operating to initiate or reinforce problems in the milieu of the individual or family unit, these disturbances are handled on a different level than in psychotherapy.

The procedures are focused on enhancing the client’s own problem-solving capacities to cope more effectively with social difficulties or to execute actions to modify or resolve them.

The establishment of a working relationship with the patient, the making of a social diagnosis, the understanding of the client’s personality workings and resistances, the analysis of the client’s assets and liabilities, and the maintenance of an effective communicative climate are

important elements in the methodology of the caseworker.

Guidance, reassurance, emotional support, opportunity for emotional catharsis through verbalization, environmental manipulation, and interpretation of the client's feelings and resistances are freely employed in the counseling process that develops.

While the contributions of Freud and Rank are readily acknowledged, their influence on social casework practice has not been as vigorous as in previous years.

There are instead tendencies to employ the principles of ego psychology (Upham, 1973), systems theory (Hollis, 1974) and behavior modification (Fischer & Gochros, 1975), trends that are also preoccupying contemporary psychotherapy.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 112 Unfortunately, modern casework, for many years the dominant method of practice, is undergoing an unfair attack as too expensive, and that it focuses on getting people to conform to oppressive social conditions.

The field of social work itself is in a state of ferment as practitioners seek new, more lucrative roles and values to keep up with the temper of the times (Rein, 1970; Payne, 1972; Vigilante, 1972).

In the face of this many social workers are striving for changes in professional identification, moving closer to the clinical field.

The social worker, usually a psychiatric social worker, who has had specialized postgraduate training in psychotherapy, may, in addition to casework, function with clients on a psychotherapeutic level, dealing with their intrapsychic mechanisms as they influence the total adjustment of the individual.

When the trained caseworker does this, casework is no longer being practiced; psychotherapy is the modus operandi.

Toward this expanded function advanced clinical training is being offered to social workers in training centers around the country on a postgraduate level.

With such training, the designation of psychiatric social worker is being rejected by many social workers as a term connoting subservience to and imitation of the medical profession.

The title clinical social worker is favored, which better describes their practice while recognizing their independence of functioning.

**COUNSELING AND PSYCHOTHERAPY** Counseling is customarily defined as a form of interviewing in which clients are helped to understand themselves more completely in order that they may correct an environmental or adjustment difficulty.

Guiding and helping people to make rational decisions, to organize plans for constructive pursuits, to seek out the best available community resources to satisfy immediate and future needs, and to overcome reluctancies toward and fears of action are among the tasks of the counselor.

A wide variety of professional and paraprofessional paid and volunteer workers function in this way as counselors.

Clients seek help for a host of problems, some real, some projections of inner distress.

And it is the duty of the counselor to distinguish between the two.

The relationship between client and counselor, which is considered of prime

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 113 importance in counseling, is used in different ways—from the offering of suggestions as to available resources to the interpretation of the client's attitudes and feelings.

The directiveness of the counselor varies.

In directive counseling (Thorne, 1950) the counselor assumes the role of an authority offering the client an evaluation of the particular problem and defining courses of action.

In nondirective counseling (Rogers, 1942) the counselor functions as an agent who encourages the client's expression of feelings, reflecting these and helping the client to assume

responsibility for them.

In this way the client thinks things out, develops goals, and plans the course of action.

Other forms of counseling draw from the field of dynamic psychology, seeking to utilize the counselor-client relationship to demonstrate the operations of the client's personality structure either in creating the situations for which help is being sought or in blocking the client from finding appropriate solutions.

Counseling programs have advanced rapidly, particularly in the educational, industrial, social work, health, and military fields.

Counselors in progressively larger numbers have been utilized for guidance activities and personal counseling in secondary schools, colleges, and universities, for "employment counseling" toward selection, placement, and morale building in industry, for "rehabilitation counseling" to enable handicapped persons to make a transition from disability to productiveness, for job relocation services for returning veterans, for "counseling psychology" in Veterans Administration hospitals and various community agencies.

Family agencies offer counseling services on matters of family relationship and social adjustment such as the following: 1.

Difficulties in interpersonal relationships manifest within the family, between the client and patient, siblings, spouse, children, and others, resulting in anxiety, symptoms, or deviant behavior.

2.

Difficulties in personal adjustment in relation to inner functions, such as sexuality, educational achievement and learning, and employment and work abilities.

3.

Environmental problems, such as economic hardships and housing deficiencies.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 114 4.

Health problems, such as disabilities, physical handicaps, mental retardation, heredity illness (genetic counseling), etc.

The helping methods in the counseling services rendered include (1) advice giving, environmental manipulation, and ego support, such as reassurance and encouragement, (2) guidance in the client's roles in family and society and in what can be done about a specific situation, and (3) clarification regarding the client's feelings and attitudes and their deviations.

Services are given not only to individuals and couples (marital counseling), but also to the total family (family counseling) and to groups (group counseling).

Because of the increasing need for counselors, various training courses have been organized on different levels of sophistication.

They range from those that require only a few hours at an undergraduate level to those that lead to a doctor's degree.

No unified curriculum exists, but a body of information is gradually being organized from fields of psychology, psychiatry, sociology, social work, and anthropology that are helpful in understanding personality development and structure, human relationships, the vicissitudes of adjustment, and the interviewing process.

Goals in counseling have been expanded from simple testing and advice giving to managing the individual's general adjustment problems with the realization that a situational difficulty may be a mere surface manifestation of a more widespread disturbance.

For example, the attempt to broaden a person's occupational perspective or to outline a curriculum in line with the person's abilities may fail because of opposing personality forces.

This has tended to shift the emphasis of the interview in both individual and group discussions.

The client's perceptions, goals, and values as well as feelings about the self and the environment are reflected in behavioral choices.

Therefore, they need to be a part of the counseling operation (Cottle, 1973).

The counselor may thus act as a catalyzer to a growth process within the client.

The term counseling has become diffuse, covering information and advice giving, and merging imperceptibly into psychotherapy (Patterson, 1966; Truax & Carkhuff, 1967; Osipow & Walsh, 1970).

Training for a modern counseling role, accordingly, requires, in [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 115 addition to supplying the counselor with special knowledge and information about resources, a recognition of symptoms of emotional illness, some comprehension of dynamics, discernment of the forces of transference and resistance that are apt to be released during the counseling relationship, knowledge of how cultural factors influence value systems, and, finally, an understanding of one's own emotional shortcomings and prejudices (including destructive countertransference) that are apt to release themselves during counseling.

Particularly important is sufficient diagnostic skill to discriminate depression, paranoidal projections, and psychotic manifestations in their early stages.

The counselor should know when, how, and where to refer clients for psychotherapeutic help when their conditions require more than counseling and the counselor is not equipped to function in a psychotherapeutic role.

In summary, one may easily discern from the discussion how difficult it is to separate the goals of counseling from those of psychotherapy.

Some attempts have been made to distinguish the methodologies.

Counseling requires a relationship between a helping agent and a client.

In this relationship emotional intercurrents operate that may have a psychotherapeutic effect on the individual, with an influence far beyond the purposes for which help was sought.

But while personality change may be the outcome of the counseling relationship, as it is in psychotherapy, there are certain differences between the two processes.

The reason a counselee seeks help is because there is generally some situational difficulty for which specialized knowledge is required or because the individual is unable to cope with a problem through personal resources.

The counselor then executes specialized knowledge of the area of concern to aid the counselee.

Thus, the area may be educational, vocational, or behavioral, as in marital maladjustments.

Objective instruments, such as psychological tests, may be employed.

The counselee is then guided toward adequate courses of action.

The approach here is supportive, the counselor making suggestions, offering guidance, presenting to the counselee opportunities for ventilation of feelings, and encouraging the counselee toward the proper actions.

Sometimes the approach may, in addition, be reeducative, with an attempt at explaining [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 116 the meaning of destructive behavior patterns, helping the counselee to clarify feelings, and fostering an awareness of how the counselee behaves in relationships with family and other people.

If emotional factors seem to be responsible for the counselee's inhibitions, this may be pointed out to the counselee as well as ways in which the counselee may overcome blocks.

The counselor neither handles resistance and mechanisms of defense in terms of the total psychodynamic operations of the individual nor focuses on early conditionings and the unconscious forces that play upon the person.

However, where counselors have received appropriate postgraduate specialized training in psychotherapy, they may be qualified to add to the battery of counseling methods the operations of psychotherapy.

They will then function as psychotherapists rather than as counselors.

Although counseling borders closely on the domains of psychotherapy, unless some demarcation is made in boundaries and responsibilities, interprofessional communication and cooperation are apt to suffer.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 117 7 Basic Ingredients of Psychotherapy Emotionally upset people are constantly being assisted in achieving homeostatic equilibrium through a variety of approaches.

Taking a vacation, changing jobs, confiding in a concerned and wise friend, consulting a minister, swallowing tranquilizing substances, adopting a different philosophical outlook, and talking to a professional consultant all seem to bring relief.

Both informal approaches and formal psychotherapy are helpful.

The soothing embraces of a human relationship, the automatic arousal of a magical placebo element, and the releasing powers of emotional catharsis are parcels that may bring a person to an adaptive equilibrium.

The method is non-specific and diverse: it could be amulet, pill, environmental change, homely philosophy, systematized dogma, or scientific method.

But the fact that any contact between two human beings or that any device, appliance, or technique seems to bring relief does not justify our applying the label of psychotherapist to the healer and psychotherapy to the tactic.

People are abidingly achieving relief from symptoms in a propitious environment.

But only rarely—and this is most fortuitous—do they acquire a significant enrichment of their behavioral or creative potentials.

What we are concerned with is the studied manipulation of forces in a professional relationship that can, in addition to restoring homeostatic equilibrium, bring about behavioral and personality change with greater frequency than would occur by chance or through the activities of nonprofessional “helping” agencies or professional counseling.

Can we identify properties of psychotherapy that can bring about deeper and more permanent change than other forms of helping?

One of the great bewilderments in appraising the virtues of psychotherapy is the difficulty of assigning to it specific processes and effects apart from the non-specific instrumentalities of “helping” and the subsidies of casework and counseling.

In practice, the techniques of helping, counseling, and psychotherapy merge imperceptibly; as to [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 118 effects, it is generally impossible to apportion the degree of improvement brought about by non-specific and specific moieties.

Nevertheless, it is of more than heuristic value to attempt to distinguish aspects of relating to which we may affix the term “psychotherapeutic” as differentiated from counseling and helping.

Psychotherapeutic-like services are often rendered without intent by persons with no training whatsoever.

For example, what would one call the ministrations of an individual who is visited regularly—sometimes as often as six times weekly—by a steady “clientele” suffering from a wide range of psychiatric syndromes, a person who serves the purpose of relieving their emotional symptoms by (1) dispensing a tranquilizing substance more effective than the most powerful psychotropic drug and (2) relating with the clientele, variably reassuring, guiding, advising, and interpreting.

The “clients” in turn interact with this administrative individual as well as with the souls around them, participating in the boons associated with placebo influence, suggestion, and group dynamics.

Under the influence of the “tranquilizing drug” their resistances are softened, and they are

apt to express themselves volubly, often with free associations, experiencing emotional catharsis, exhibiting transference reactions (sometimes an actual transference neurosis), revealing aspects of their unconscious, and occasionally exhibiting acting-out tendencies that are usually dealt with firmly by the individual in charge.

No patients are more dedicated to their "sessions" than are these clients.

The individual to whom I refer practices the skills daily in every local bar in the country; his steady clients are among the sickest individuals in our society.

There are probably more bartenders functioning in the role of psychotherapists in this country than there are psychiatrists, psychologists, and psychiatric social workers combined.

Yet they dispense their medicaments without prescription, and they go through their interviewing maneuvers with no psychiatric supervision whatsoever.

To call such an individual a psychotherapist is obviously preposterous, and to dignify his activities as a form of psychotherapy would be a disservice to the art.

This is only one example (and there are many) of untutored helping agencies to whom an emotionally upset individual may turn who may serve a therapeuticlike function.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 119 Helpful intervention is also the intent of many professionals whose training equips them to deal with special segments of behavior and to enlist in this process community resources.

Human beings in trouble are constantly consulting such professionals to aid them in resolving their distress.

Generally the agency selected is authorized by social sanctions to manage a particular complaint; the assessment by the client of the problem will determine the choice of professional.

Thus, for marital difficulties a lawyer may be the counsel; for economic hardships, a social worker; for educational failings, a teacher or educational psychologist; for moral quandaries, a minister; and for physical troubles, a physician.

The goals of such consultations are (1) to assuage the prevailing tension, (2) to correct remediable disturbances responsible for the individual's present predicament, (3) to rectify deviant behavior, and (4) to prevent the outbreak of more serious disorders.

The responsibilities of both counselor and client are more or less explicitly defined, the role expectations of the counselor being structured by training and experience.

The effect of such counseling may be psychotherapeutic in essence, but the techniques employed and the objectives approached vary from those of psychotherapy.

Moreover, these counselors in other professional areas generally do not have the training or experience to deal definitively and correctly with emotional difficulties, although they may be able to mediate their effects.

Perhaps the most significant way psychotherapeutic relationships (other than supportive psychotherapy, which has a kinship with counseling) differ from nonpsychotherapeutic ones is that in the latter the helping agency or counselor enters into collusion with the neurotic forces to achieve an immediate objective.

In psychotherapy, there is an opposition to, and a direct attack made on, the neurotic forces in the hope of disposing of them and of reconstituting new and more adaptive defenses.

Table 7-1 attempts to distinguish helping, counseling, and psychotherapeutic situations in reference to a number of important variables.

Perhaps the main reason that psychotherapy has so often been considered affiliated with casework and counseling is that no attempt has been made to classify supportive psychotherapy in a category apart from re-educative and reconstructive psychotherapy.

Unless this is done we are unable to separate psychotherapy from other forms of helping.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 120 All helping, counseling, and psychotherapeutic situations

embrace automatic healing elements that are released during the relationship between client-patient and helper-counselor-psychotherapist.

These include the positive accruals of a projected idealized relationship along with the bounties of placebo influence, emotional catharsis, suggestion, and group dynamics.

A vital aspect of the prevailing interaction is identification with the helper-counselor-psychotherapist.

More or less, all clients-patients will regard the person to whom they relate as a model to pattern themselves by.

They will incorporate, consciously or unconsciously, that individual's ideas, attitudes, and values into their reality-testing and problem-solving activities.

The possession by the helper-counselor-psychotherapist of appropriate personality characteristics and attitudes will enhance this therapeutic dimension, while their absence may interfere with it.

Present also in all helping, counseling, and therapeutic relationships are elements of transference and countertransference, the understanding and management of which may constitute the difference between a successful and unsuccessful outcome.

In helping situations, counseling, and supportive psychotherapy, positive aspects of transference and countertransference are cultivated to enhance the operations being promoted; negative aspects, if recognized, are reasoned away or avoided.

In some reeducative and all reconstructive psychotherapies, positive and negative transference and countertransference are examined and analyzed as a means of understanding the patient's behavior and of aiding him or her in altering it.

**COMMON ELEMENTS IN ALL PSYCHOTHERAPIES** Let us now, then, attempt to delineate some processes that are inherent in effective psychotherapy, which while perhaps present to some extent in helping and counseling are not deliberately nurtured.

All good psychotherapeutic systems—irrespective of their theoretical underpinnings and while they manifest some differences—employ these processes to a greater or lesser degree, whether they involve conventional dyadic insight approaches, manipulations of the patient-therapist relationship, or selected reinforcement of special aspects of behavior.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 121 Interviewing Procedures Communication is the channel of interchange between patient and therapist.

Practitioners of different methods are usually taught principles of interviewing consonant with their theoretical systems.

In the main, the practitioner must be able to subject the patient's communications to selective scrutiny, directing comments toward facilitating and constructively utilizing verbalizations.

This involves an ability to employ language that is understandable to the patient.

It includes an awareness of nonverbal behavior, an index of some of the most important defensive operations.

It entails knowledge of techniques of maintaining the flow of significant verbalizations either toward free association or toward selective focusing on pertinent themes.

It embraces methods of understanding or inculcating insight by various techniques, including interpretation.

It encompasses an understanding of how to terminate the interview.

These formalities are often left to chance during training, and it is only through experience that the practitioner gains the interviewing skills that are most helpful to his patients.

**Establishment of a Working Therapeutic Relationship** Unless a cooperative empathic contact is established with the patient, the therapeutic process may come to naught.

An effective system must maintain this as a prime objective during the first part of therapy.

The techniques of achieving a relationship are rarely formalized, but usually they involve



gaining the patient's confidence, arousing expectations of help, accenting the conviction that the therapist wishes to work with the patient and is able to do so, motivating the patient to accept the conditions of therapy, and clarifying misconceptions.

Without a working relationship, in reconstructive therapy, there can be no movement into the exploratory and working through phases of therapy; the patient will be unable to handle anxieties associated with the recognition and facing of unconscious conflict.

In supportive and reeducative therapy, a good relationship expedites progress immeasurably.

Determination of the Sources and Dynamics of the Patient's Problem

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 122 Cognitive learning is present in all therapies.

The different psychotherapies attempt to search for and to explain the patient's emotional difficulties in varying terms, such as discordant elements in the environment that mobilize stress, distorted interpersonal relationships that prevent the individual from self-fulfillment, conditionings that rigidly dragoon the patient to destructive behavior, and unconscious conflicts that mobilize anxiety and interfere with a realistic adjustment.

All psychotherapies attempt such explorations within the framework of special theories about human development and adaptation that include to a greater or lesser degree some of Freud's monumental discoveries and refinements of Pavlovian concepts.

It is generally considered essential in the resolution of a problem toward reconstructive change for the individual to become aware of the fact that one is being victimized by repetitive patterns that force one to actions opposed to a productive life.

These patterns are rigid and compulsive; they defy logic and common sense; they are both supported and opposed by ambivalent value systems that have been incorporated within the self; they make for an undermining of security and self-esteem, and for helpless expectations of injury that are registered in reactions of anxiety.

The physiologic and psychologic manifestations of anxiety, and the marshalling of defenses against anxiety create various symptoms of neurosis.

Much of this dynamic turmoil goes on below the level of awareness, and its recognition is opposed by the mechanisms of denial and repression that both safeguard the individual against anxiety and help to retain the neurotic gains residual in their preservation.

A variety of resistances operate to maintain this denial-repression.

The individual who is being treated under the aegis of this dynamic model is, through a number of techniques, taught to recognize personal offensive patterns and their consequences, the repudiated conflictual aspects of the psyche, and the origins of difficulties in destructive past conditionings.

Awareness of stress sources and conflicts hopefully enables the individual better to challenge current maladaptive patterns, to be liberated from old values, to rectify the disturbed life situation with new modes of relating to people, and to develop a more wholesome and realistic conception of the self.

Supportive and reeducative therapies deal more with provocative reality factors in the here and now, while attempting to correct faulty past learnings in line with goals of symptom alleviation and problem solving.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 123 Many contemporary psychotherapeutic systems utilize some of the fundamental principles of Freud, though they affix to these their own labels.

To a greater or lesser degree, concepts of the unconscious, repression, transference, and resistance are acknowledged.

The means by which the patient is brought to an awareness of problems and the extent of exploration of the unconscious, will depend on the type of theoretical orientation to which the therapist has been exposed.

The focused interview, free association, dream interpretation, analysis of the transference,

exploration of genetic material, and the buildup of a transference neurosis will thus be employed in varying degrees.

Behavioral therapies, rooted in conditioning theories, do not put much credence on insight acquisition; rather they focus more on tactics of relearning.

But inherent in the techniques employed is a relationship between therapist and patient and the inevitable derivation of some insight as part of the corrective therapeutic experience.

Utilization of Insight and Understanding in the Direction of Change Effective psychotherapies acknowledge that understanding is not enough, that conditioned patterns of behavior do not allow themselves to be displaced so easily, and that various techniques must be implemented to produce change.

Techniques, therefore, are put into effect to create incentives for change, to deal with forces that block action, to promote problem-solving and reality testing, to help the patient to master anxieties investing normal life goals, to correct remediable environmental distortions, to encourage adjustment to irremedial conditions, and to accept personal limitations and handicaps while fulfilling creative potential to the highest degree.

Behavior therapies focus on this relearning dimension almost exclusively.

Resistance and the Readiness for Change In all forms of therapy resistance will appear in stark or disguised forms and may block or destroy therapeutic progress.

Despite the fact that suffering is intense and symptoms disabling, the patient may resist changing a preferred way of life.

Toward this [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 124 end the efforts of even experienced therapists may be blocked.

The bounties derived from pursuing a course that must inevitably result in anxiety and turmoil may not be apparent on the surface.

The patient seems frozen into unreasonable bad habits that refuse to dissolve.

And credit for failure may be ascribed to the impotence of the therapist and the worthlessness of the latter's methods.

Resistance to change can paralyze all forms of therapy, and the capacity to recognize their subtleties and to deal with them constructively spell the difference in any psychotherapeutic endeavor between a therapeutic triumph or a debacle.

What we seem to be dealing with in all of our patients is their readiness for change, which apparently involves the degree to which they have spontaneously or with professional help resolved their resistance to change.

An individual with reasonable readiness to move forward will seem to benefit from almost any situation or tactic that can be used constructively.

For years there may have been silent building either through spontaneous insights and propitiously reinforcing life experiences or in formal therapy, with few apparent signs of improvement.

Should more psychotherapy later be sought, improvement or cure may then unjustifiably be entirely credited to the second treatment experience, however brief or coincidental it may be, or to some dramatic event in life that actually served as a convenience that was successfully manipulated.

Multiple obstructions in the form of resistance are apt to present themselves at every phase of treatment.

They may oppose the establishing of a working relationship, the acceptance of explanations of the therapist, the full cooperation with the therapist's techniques, the search for provocative conflicts, the probing into genetic material, the facing of reality, the abandoning of the pleasure values and secondary gains of neurotic tendencies, the acceptance of maturity, and the giving up of the treatment situation when termination is necessary.

Obstructions appear in various masquerades, as transference, as “acting out,” as forced “flight into health,” as self-devaluation, and as innumerable other disguises.

The skill of the therapist is revealed by dexterity in recognizing and managing the resistive maneuvers of the patient.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 125 Patient Variables There are an endless number of variables that the patient brings into therapy that will augment or negate the direction of psychotherapy.

The expectations of the patient, the kinds of symptoms possessed, the attitudes and reactions to the therapist as an authority as well as to the techniques being employed, and the intensity and persistence of childish distortions are among the most common factors that must be taken into account.

Perhaps of greatest importance is whether or not the patient will utilize the relationship with the therapist for objectives inimical to therapeutic goals.

Thus if residual dependency needs exist, the patient may override idealize the therapist and project personal aspirations for magic onto the therapist.

Basking in the sun of the therapist’s celestial power, the patient will establish a satellite position insisting that the therapist cure him or her even in the absence of any personal effort.

We are all victims of past conditionings and habit patterns, some of which interpose themselves subtly on our present-day adaptations.

If a person as a child has been able to maintain identity only by resisting or fighting parental authority, there is no reason why we should not suspect that the individual will attempt to treat the therapist with similar defensive tokens.

These may never interpose themselves in outright defiance; rather they may take the more subtle form of an inability to respond to remedial promptings.

An insidious pattern possessed by some patients who seek to enhance their independence and emancipation is a detachment that separates individuals from others and from themselves and accordingly tends to rob them of many of life’s pleasures.

The presenting complaint may be depression and generalized anhedonia.

We may find that a meticulous application of techniques fails to register marks on the patient’s indifference.

When we realize that our patient has an investment in maintaining detachment, that it has always served the patient as armor against being controlled and manipulated, that it dulls threatened anxiety and a thousand imagined hurts, we can see that efforts toward its maintenance have greater reinforcement value for the patient than the rewards we as therapists can proffer.

There may be nothing faulty in our techniques, but psychological obstructions act as impenetrable barriers to our efforts.

This is why a high level of motivation is so important in all therapies.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 126 Therapist Attitudes and Operant Conditioning The proper therapist attitudes are therefore crucial for effective psychotherapy as they are probably important for all kinds of learning.

They constitute powerful reinforcers that strongly influence the patient’s behavior.

Attitudes of empathy, warmth, and understanding tend to promote positive feelings in the patient; they relieve tension and lower the anxiety level.

In such an atmosphere learning is enhanced.

Interviewing, focused by the therapist on anxiety-laden content, may then prove rewarding.

Thus, if dynamically oriented, the therapist will pursue and encourage the patient to explore zones that are usually resisted or repressed.

Approbative responses, verbal and nonverbal, from the therapist reward the patient when repudiated material is prosecuted.

In addition to the temporary benefits of emotional catharsis, the patient learns that this material can be tolerated, and when placed in the context of the historical past, a reevaluation may occur.

Schedules of selected reinforcement foster the extinction of anxiety-provoking past experiences and their present-day associations.

In behavior therapy the patient is also exposed, in the medium of a rewarding emotional climate, to reinforcers that help extinguish certain reactions that have been self-defeating and accentuate others that have an adaptive potential.

Symptom relief and the acquisition of constructive behavior patterns occur without the formality of insight.

Apart from specific reinforcing maneuvers that are implemented in dynamic and behavioral approaches, the therapist-patient relationship itself serves as a relearning experience from which the patient may generalize constructive responses toward other relationships.

This gratuity may occur in any helping or therapeutic relationship.

Dynamic approaches have the advantage of working with transference contaminants that can effectively block this happening.

Where interfering transference is not bypassed, but dealt with firmly in terms of its genetic roots, and the patterns and defenses that it embraces are skillfully analyzed, it will tend to undergo negative reinforcement and extinction.

The therapist relationship will then become a powerful corrective experience for the patient.

This does not mean that cure is automatically guaranteed, since in some cases psychic damage is so profound, the secondary gain benefits so intense, the masochistic need so

www.freepsychotherapybooks.org 127 great, that inner rewards for the perpetuation of transference exceed those the therapist can supply by approving-disapproving tactics.

Nevertheless, in a considerable number of patients the developing and unravelment of transference can be most facilitating of extensive personality alterations.

Behavior therapies, while remarkably effective in promoting symptomatic improvement and behavior change, cannot approach the depth of reconstructive personality change possible in selected patients exposed to dynamic therapy with trained psychotherapists whose personality structures contain the proper ingredients of warmth and understanding, and who know how to deal with transference and countertransference.

A question immediately poses itself.

Is not the proposed climate for some types of psychotherapy, for instance classical psychoanalysis, a neutral, detached one, and, if so, would not the patient then respond in an antitherapeutic way to the traditional detached manner of the therapist?

The answer to this question lies in the simple fact that effective psychotherapists, including psychoanalysts, are not really neutral and unconcerned.

They communicate, in spite of practiced noninterference and passivity, an understanding of and empathy toward their patients.

The patient quickly discerns from nonverbal cues the underlying true emotional feeling of the therapist.

Noneffective therapists (including psychoanalysts), on the other hand, who personality-wise are detached, cold, uninvolved, or lacking in empathy will stimulate negative therapeutic reactions in their patients.

Extensive training and experience will not compensate for the absence of positive personality qualities, without which no technique can truly be productive.

Psychotherapists generally practice preferred methods that over the years have yielded enough

triumphs to reinforce faith in their powers.

What we do in therapy is tempered constantly by how we do it.

We have an affinity for some techniques and prejudices toward others.

Not all procedures make sense, nor will they work for all therapists.

A highly discriminating process generally takes place as therapists gain experience and find that certain theories and special techniques seem effective in their hands.

A problem that plagues our field, of course, is the tendency to apply one's personal experience to the world at large.

The fact that a therapist finds a particular approach of [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 128 great value for her or him does not mean that other therapists will do likewise.

Countertransference An effective psychotherapeutic system recognizes negative damaging consequences of countertransference.

The prejudiced responses of the therapist to the patient, positive or negative, may interfere with the latter's getting well.

The nature of countertransference projections onto the patient will depend upon the specific problems of the therapist that are being activated by the patient at the time.

These may be unique to a single case, or they may occur in different forms with various patients.

Where a therapist is victimized by feelings over which he or she has little control, such an individual may not be able to apply techniques with a proper measure of disciplined objectivity.

For instance, a therapist repulsed by homosexuality is not the preferred resource for an individual pursuing a nonheterosexual life style.

A therapist who is fearful of aggression may display anti-therapeutic behavior when verbally attacked by a disgruntled soul.

The passive, ingratiating, helpless patient may arouse overprotective attitudes in the therapist who may act as a crippling shield, isolating the patient from the realities of life.

Such attitudes will interfere with the working relationship, the womb in which personality change and other benefits are propagated.

In contrast, countertransference may be, if utilized correctly, an important indicator of nonverbalized attitudes and feelings that are being projected onto the therapist and thus prove helpful in understanding the patient's conflicts and needs.

Environmental Variables In considering what ingredients enter into psychotherapy, we cannot neglect social forces.

Existing cultural trends and the prevailing life style may motivate patients to seek out special types of therapy and actually influence their learning patterns.

In our presentday rock-loving, drug-dominated culture, members of the younger generation are especially attracted either to the expressive types of therapy characterized, on the one hand, by acting-out, screaming, and shedding superego restraints and, on the other hand, to an escape from tensions and responsibilities through meditation, psychotropic

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 129 substances, and indulgence in Eastern philosophies.

It may be futile to try to impose variant therapeutic techniques on such individuals.

They may be more attuned to therapists with unconventional styles, particularly therapists labeling themselves as "avant-garde" who practice original and unorthodox methods that border on the irrational.

The environment itself in which the individual functions will influence therapeutic change both during and following treatment.

Thus, a milieu that reinforces destructive behavior will neutralize and one that rewards healthy behavior will encourage the success of the therapist's efforts.

Recognition of the environment in which the individual functions and will be forced to live in after therapy will permit the therapist to focus on elements that need to be altered or, if irremediable, adapted to without compromising the gains achieved in psychotherapy.

**Termination of Therapy** The termination of all types of therapy is best handled in a planned way and not left to chance.

An analysis of any obstructive dependency elements in the therapist-patient relationship is part of this process.

The patient is generally induced to shoulder the bulk of probings into personal problems and to take total responsibility for his or her plans and activities.

Independence and assertiveness are goals toward which the patient is encouraged.

The patient is prepared for possible relapses and reminded that should any symptoms return, the tool of self-understanding acquired in therapy should help him or her regain equilibrium.

**SUMMARY** Having delineated the important aspects of process, can we reasonably assume that these will bring good results?

In the main, yes; but, as has been indicated, there are important qualifications.

There are certain limitations to change in all people; there are certain potentialities for change in all people.

If the psychotherapist applies himself or herself to the task with disciplined process, he or she will be best equipped to foster in [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 130 patients a successful outcome.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 131 8 Is Psychotherapy Effective?

In some circles the idea still prevails that psychotherapy is a swamp of marshy theories imbedded in a quagmire of metapsychological slogans and convoluted methodologies.

This perhaps was the sentiment behind the Congressional queries several years ago regarding the effectiveness of psychotherapy while asking for demonstrated proof of its value.

Unfortunately, it has been extremely difficult to establish, without question, a causal relationship between techniques and methods of any psychotherapeutic system known today and the changes that have been brought about through the expedencies of that system.

Both the futility of all forms of therapy in altering neurotic processes (Eysenck, 1952, 1954, 1955, 1960a, 1964, 1965, 1966, 1967; Levitt, 1957, 1963) and arguments against these conclusions (DeCharms et al, 1954; Rosenzweig, 1954; Bergin, 1971) have been voiced.

Skeptics insist that neither clinical studies nor ordered observation and experiment have established beyond reasonable doubt the virtuosity of psychotherapy.

This does not mean that psychotherapy is unproductive; on the contrary, the experience of "effective" psychotherapists is testimony to its potentialities.

However, because present-day propositions that exist in the field of psychotherapy are not of a high order of empirically tested probability, it is difficult to demonstrate the consequences of treatment by any concrete methods and operations.

Attempts to apply probability theory to the events of psychotherapy are blocked by formidable difficulties that have up to the present time defied resolution.

This has encouraged some iconoclastic research psychologists to apply themselves to the evaluation of psychotherapy with the dedication of assassins.

Present-day outcome studies have yielded impressive statistics about the effectiveness of psychotherapy that contradict published negative reports (Smith et al, 1980; Andrews & Harvey, 1981; Epstein & Vlok, 1981; Am.

Psychiat.

Assn.

Com., 1982).

However, the skeptics insist that evidence from statistics is, upon close analysis, vastly

www.freepsychotherapybooks.org 132 misleading since we have few criteria upon which to gauge the quality of improvement or the specific parameters of personality that are being influenced by psychotherapy.

Thus it has been estimated that two-thirds of all patients suffering from emotional difficulties, who turn to and relate themselves with helping agencies other than psychotherapists, will, if the agencies are reasonably mature, experience "cure" or "improvement" purely as a product of the relationship.

It has been posited also by some observers that a similar proportion of cures or improvements will be registered should the same kinds of patients come under the care of psychotherapists or psychoanalysts.

With non-specific therapeutic measures, principally rest, sedation, and reassurance, Denker (1946) discovered a recovery rate of 70 percent, while Landis (1937) reported a recovery rate of 68 percent in patients who were not exposed to any therapy.

If these findings are true, psychotherapy would seem to be a fraud.

It would scarcely be worthwhile to expose oneself to the rigors and expense of psychotherapeutic treatments if at the end the results were no better than one could obtain with less elaborate procedures.

On the other hand, if one could demonstrate that the quality of the two-thirds cure or improvement was of a better grade, or if the total improvement rate with psychotherapy could be increased by at least 20 percent, the effort and financial outlay might be justified.

No matter how strong our conviction may be about the positive effectiveness of psychotherapy that is reinforced by some of the modern studies on outcome, we cannot, with a wave of hands, disregard the negative convictions of the skeptics nor the tenets of past published data pointing out the absence of irrefutable documentation that psychotherapy is more potent than spontaneous cure or counseling (Appel et al, 1953; Teuber & Powers, 1953; Barron & Leary, 1955; Frank, 1961; Eysenck, 1962).

Indeed, there are studies that seem to indicate that patients who apply for therapy and are merely put on waiting lists, receiving no further treatment, reveal after a six month's follow-up, a 40 percent rate of improvement (Endicott & Endicott, 1963), and five to six years after the initial evaluation, a spontaneous improvement rate of 65 percent (Schorer et al, 1966).

The latter rate is held to be superior to that following exposure of patients to an extensive and carefully designed treatment program.

www.freepsychotherapybooks.org 133 In the Cambridge-Somerville Youth Study (Powers, 1949; Powers & Witmer, 1951; Teuber & Powers, 1953) 2 equal groups of 325 boys were matched. The first group received therapy from adherents of both the psychoanalytic and Rogerian schools.

The second group served as controls.

Follow-up studies over a period of years disproved the expectation that the treatment group would be less delinquent than the other.

Indeed there was a slight difference in favor of the control group.

Brill and Beebe (1955), working with soldiers who had experienced a breakdown in the army, also found that no difference was scored in remission of neurosis between those who did and those who did not receive psychiatric treatment.

Barron and Leary (1955) treated a group of psychoneurotic patients and compared the end results with an untreated control group.

They discovered that "for the most part...the changes tended to be in the same direction for treatment and non-treatment groups, and of about equal magnitude." Three groups of patients were subjected to a follow-up study by Barendregt (1961).

The first division of 47 patients had been given psychoanalysis, the second of 79 patients received psychotherapy other than analysis, and the third of 74 was exposed to no form of psychotherapy.

The results showed little difference among the different groups.

Gliedman et al.

(1958), on the basis of their work, insist that placebos are as effective as psychotherapy in psychiatric cases exposed to both.

Walker and Kelley (1960), working with male schizophrenic patients, reported that short-term psychotherapy brought about no greater improvement than ordinary custodial care; indeed, it seemed to delay the discharge of patients.

The quoted results of psychotherapeutic treatment of over 70,000 cases reviewed by Eysenck (1952, 1965) concluded that these “fail to prove that psychotherapy, Freudian or otherwise, facilitates the recovery of neurotic patients.” Approximately two-thirds of patients will improve with or without psychotherapy.

Exposed to psychoanalytic treatment, the cure-improvement rates average 44 percent, variously being reported as 39 percent (Fenichel, 1920-1930), 62 percent (Kessell & Hyman, 1933), 47 percent (Jones, 1926—1936), 50 percent (Alexander, 1932-1937), and 67 percent (Knight, 1941). With “eclectic” psychotherapy the cure-improvement rate was cited as higher, averaging 64 percent.

This figure was drawn from the following: 46 percent (Huddleson, 1927), 41 percent (Matz, 1929), 55 percent (Luff & Garrod, 1935), 77 percent (Ross, 1936), 58

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 134 percent (Yaskin, 1936), 61 percent (Curran, 1937), 54 percent (Masserman & Carmichael, 1938), 73 percent (Landis, 1938), 53 percent (Carmichael & Masserman, 1939), 63 percent (Schilder, 1939), 66 percent (Hamilton & Wall, 1941), 51 percent (Hamilton et al, 1942), 50 percent (Wilder, 1945), 58 percent (Miles et al, 1951).

The data of the Central Fact-finding Committee of the American Psychoanalytic Association, according to Brody (1962) and Masserman (1963), appear to reveal that of 210 “completely analyzed” cases, 126 were “cured” or “greatly improved.” Of the remaining 385 “incompletely analyzed patients,” it is estimated that about half achieved some improvement.

Levitt (1957), summarizing many studies on the psychotherapeutic treatment of children, arrived at a figure of 67.05 percent of cases who improve at the end of therapy and 78.22 percent at follow-up.

Eysenck (1965), in examining this mass of data, concluded that psychotherapy registers a small effect, if any, on patients, with the exception of therapies based on modern learning theory.

His conclusions, while upheld by Astrup (1965), Zubin (1965), Meehl (1965), Davidson (1965), and Wolpe (1965), have been vigorously challenged by Zetzel (1965), Frank (1965), Glover (1965), Barendregt (1965), Matte-Blanco (1965), Strupp (1965), Handlon (1965), and Bergin (1971).

To all of these criticisms, Eysenck (1973) has replied that there is not one single study that indisputably demonstrates that psychotherapy succeeds better than no treatment, or behavior therapy, or any other alternative.

Indeed, he avows that Rachman (1972) in his book, which exhaustively reviewed the literature, substantiates his own conclusions made in 1952.

He states, “I believe that psychotherapy of the usual interpretive kind is simply the premature crystallization of spurious orthodoxy, a verbal exercise without any proof of effectiveness.”

Bergin (1971) reviewing the data that Eysenck cites in his 1952 review comes up with a different statistic by taking another point of view.

For example, Eysenck quotes a study by Fenichel evaluating the work of the first ten years of operation of the Berlin psychoanalytic clinic and shows that the improvement rate was



two-thirds.

However, Eysenck obtains this rate by including those patients who dropped out of treatment after brief contact with the clinic.

If these patients are not included, then the improvement rate jumps to 91 percent.

Scientifically, however, one can validly argue for either including or

www.freepsychotherapybooks.org 135 excluding the dropouts in computing the percentage rates as improved.

Bergin has also demonstrated that the spontaneous improvement rate of two-thirds shown by control groups supposedly receiving no treatment is fallacious.

The reason that one cannot have an adequate control group in an outpatient psychotherapy study is because patients who are suffering and are refused help will usually look elsewhere for assistance.

They will go to friends, neighbors, or practitioners of various sorts (Gurin et al, 1960).

They thus do not constitute a scientific control group.

At best we must compare the results of professionally trained therapists with the results of nonprofessional operators in the community.

Some of the latter may influence patients toward improvement as or more effectively than professional therapists, largely perhaps because they are adept at impressing on their clients an unshakable optimism and thus stimulating non-specific elements of the helping relationship. Published reports of spontaneous recovery or improvement rates on emotionally ill persons in whom no formal therapy had been validated are below the two-thirds figure and range from zero upward: for instance, negligible (Orgel, 1958; Cappon, 1964; O'Conner et al, 1964; Koegler & Brill, 1967); 30 percent (Shore & Massimo, 1966); 25 percent (Kringlen, 1965a) and 18 to 22 percent (Paul, 1967).

These studies, which indicate a median rate of 30 percent, deal with varying patient populations with widely different syndromes and hardly satisfy rigorous empirical criteria (May, 1971).

Despite later writings disputing Eysenck's ideas (Bergin & Lambert, 1978; Vandembos & Pino, 1980), these continue to influence opinion and are upheld by some (Erwin, 1980).

To round out the statistical muddle, the different schools of psychotherapy cannot agree among themselves regarding the fruitfulness of their particular brands of therapy, published figures displaying a greater divergence among proponents of a special approach than between those of different schools.

There is general agreement, however, among non-analytic groups that results with their methods are superior to those of psychoanalysis.

Behavior therapists, for example, score their curative yield as roughly double that of psychoanalysts.

Psychoanalysts, on the other hand, label the results of nonanalytic therapy as "temporary" and "superficial." THERAPEUTIC IMPROVEMENT IN RELATION TO GOALS www.freepsychotherapybooks.org  
Comparison of the effectiveness of competitive brands of psychotherapy is meaningless without considering the goals to which they direct themselves.

Symptom relief or cure, a legitimate and important target, is easier to achieve than attitude and behavior change, which, in turn, is more readily attainable than reconstructive personality change.

The reason for this is that habit patterns constitute for the individual a way of life.

They contain vital defenses and security operations, interference with which is bound to provoke anxiety.

The painful confrontations to which the ego must inevitably be exposed will promote resistance that may take diverse forms, one of which is flight from therapy.

Where a patient interrupts treatment before the goal of reconstruction has been reached, the initial symptoms, kept alive by resistance and transference, may still be present.

Therapy may then be graded as a failure.

Paradoxically, had treatment been discontinued during the early treatment phases when the salubrious glow of the placebo effect was still felt, and before resistance and transference had restored some symptoms, therapy may have been considered successful by the patient. However, more extensive goals would not have been reached where potentially achievable. Whereas an effective psychotherapist can obtain 80 to 90 percent of symptom cure or improvement, the positive results will be reduced where behavior change is the goal. In the event the therapist seeks reconstructive changes in patients, a figure of only 40 percent will probably be as high as can be attained, even where the therapist is highly trained.

There are many reasons why so small a percentage of patients can be influenced beyond the benefactions of symptom relief and attitude change.

With the best of intentions the therapist may succeed in tearing down, in sicker patients, their defensive structures, expectantly waiting for a new personality edifice to rear itself.

Helpless and deprived of customary neurotic resources, which have been "analyzed away," the patient will cling to the therapist with a dependent desperation that will confound both participants.

Hopeful expectations go unrewarded, the patient symptomatically being worse off than before treatment.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 137 As in major surgery, the risks are greater in reconstructive therapy than where palliative measures are employed.

The results will best justify the risks where cases are carefully selected.

This calls for diagnostic skills that enable the therapist to exclude patients who are least disposed toward extensive change (such as fragile psychotics, severe alcoholics, psychopathic personalities, drug addicts, borderline cases) profound characterologic dependencies, severe obsessive-compulsive neuroses, etc.

Reconstructive treatment necessitates sophisticated training and wide experience in the handling of stormy, long-term therapeutic relationships.

Nevertheless, with the skillful application of techniques, the rewards should more than justify its application in selected patients as the preferred treatment method.

**THERAPEUTIC IMPROVEMENT IN RELATION TO TREATMENT PHASES** At the beginning of any treatment effort, irrespective of type, extratherapeutic helping agencies operate to bring about improvement in symptoms.

Counteracting these positive non-specific influences during early treatment phases are (1) defective motivation, (2) continuing conflict that, sponsoring anxiety, revitalizes symptoms, and (3) the defensive dividends and secondary gains that make the retention of symptoms advantageous for the patient.

These interferences must be dealt with energetically by therapists as part of their technical pursuits.

Assuming that negative forces are not too prominent, or that they have been handled with proper therapeutic craftsmanship, the patient will register symptomatic improvement.

This boon is the conjoint product of such forces as the placebo influence, emotional catharsis, the projection of an idealized relationship, suggestion, and group dynamics (see Chapter 4).

Abatement of tension and anxiety and restoration of a sense of mastery will then promote a better outlook toward life.

If therapy is stopped at this period, the patient may be able to continue improvement, particularly if the sources of difficulty are dealt with constructively.

This can best be accomplished during the brief therapeutic effort after existing troubles have been explored and especially when a continuity has been established between the [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 138 immediate complaints, habitual personality operations, and determining childhood experiences and conditionings.

If, however, therapy continues beyond this early phase, initial benefits may soon expend themselves in the wake of the patient's realization that the idealized properties with which he or she has invested the therapist are truly nonexistent.

Faith, hope, and trust no longer will temper the therapeutic climate.

It is, of course, possible where the patient's need is sufficiently great—as in certain characterologically dependent personalities—for the patient to continue to endow the therapist with godlike qualities, particularly where the therapist narcissistically shares the patient's omnipotent delusions.

Under these circumstances the patient will bask in the therapist's sun, soaking up the power of celestial exposure, and feel protected and continue symptom free as a result.

On the other hand, neither the patient nor the therapist may be capable of keeping alive such a sainted image.

Indeed the therapist may purposefully retreat from playing a protective role and, particularly where striving for reconstructive goals, may even challenge the patient's defenses by pointing out existing behavioral improprieties.

An inescapable increment of the protracted therapeutic time period to which patients are exposed for purposes of extensive personality alteration is the furtive or dramatically explosive obtrusion of resistance and transference.

Where the patient's inherent personality strengths are sufficient to reconstitute personal defenses in a new climate of strife, and where the therapist is sufficiently skilled and by disposition equipped to handle the patient's insurgency, the patient will best be enabled to proceed toward a remodeling of relationships and toward values beyond the benefactions of symptom relief.

At any rate, we may expect that by-products of most therapeutic endeavors that extend themselves in time are an eruption of symptoms and a mobilization of tension and anxiety. Stopping treatment at this middle phase will usually expose a patient who insists that therapy has brought little benefit.

And yet, had treatment halted earlier, at the crest of the non-specific improvement wave, the effectiveness of the effort might have been endorsed.

In reconstructive therapy, therefore, we may expect a recrudescence of symptoms.

Challenge of resistance, maneuvers toward insight and working through, are aspects of [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 139 the therapeutic process, however painful they may be, that force patients to learn new modes of adjusting.

It is to be expected also that prolonged therapy will tend to promote dependency.

This silent saboteur may interfere with the patient's efforts toward self-actualization.

It may keep the patient reduced to an infantile status, violently promoting a return of original symptoms when the patient perceives a threat of termination of therapy.

As the therapist works through this "separation anxiety," the patient may be able to rely more and more on himself or herself and gradually to expand the feelings of assertiveness.

Table 8-1 outlines some of the foregoing processes.

**THERAPEUTIC IMPROVEMENT IN RELATION TO THE THERAPIST'S PERSONALITY** One of the most important variables in psychotherapy is the helping or therapeutic personage, whose character traits and technical skills are bound to influence results.

In Table 8-2 an attempt has been made to grade rates of cure or improvement that we expect in relation to desired areas of change (goals), the kinds of processes to which the individual is

exposed (techniques), and the quality of the agency administering help.

Starting with the baseline of what we might anticipate should the individual spontaneously exploit random avenues of help, we may then compare these rates with what could happen if the individual related himself or herself to some "helping" person, such as a minister, physician, teacher, friend, or authority who, while trained in a particular field, has had no special schooling in counseling or psychotherapy.

"Effective helpers" are those who possess personality qualities that inspire in the subject with whom they are working hope, faith, trust, liking, and freedom to respond.

Such helpers generally have characteristics of sincerity, honesty, a capacity to respect people, confidence in what they are doing, positiveness of approach, and what Rogers (1946) has called "genuineness," "empathic understanding," and "non-possessive warmth." Whitehorn and Betz (1960) and Betz (1962) substantiate the vital role the personality of the therapist plays in securing results with schizophrenics in [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 140 psychotherapy.

Effective therapists, they discovered, see a patient as a person and not a problem, stress assets of the patient not liabilities, challenge self-depreciatory attitudes, are reasonably permissive, behave naturally with the patient, and focus on securing a trusting relationship.

Ineffective therapists are too permissive, focus on the patient's mistakes, evince the qualities of an aloof teacher, and are more passive and permissive.

Betz (1967) suggests, however, from research studies that the outcome of therapy depends on the quality of the relationship that develops as a result of the blend of the personalities of therapist and patient.

Truax and Carkhuff (1967) have derived three scales of traits of effective therapists derived from ratings of typescripts: (1) positive regard for the patients, (2) accurate empathy, and (3) congruence.

The accurate empathy scale would seem to be related to the concept of accuracy of interpretation of what is going on within the patient, although no studies have been done to date to relate these two concepts.

Congruence refers to therapists being in touch with their own feelings.

If therapists assert that a patient is "liked," yet by tone of voice and previous statements show anger toward the patient, they are rated to be in low congruence.

Truax and Mitchell (1972) have reviewed the results of 10 years of such studies and have found that the results are very consistent across many studies employing different diagnostic groups, with therapists of different theoretical persuasions and with differing lengths of therapy.

Rogers et al.

(1967) reported an extensive controlled study of psychotherapy with schizophrenic patients who were hospitalized at a state hospital.

They found that patients whose therapists offered high levels of non-possessive warmth, genuineness or congruence, and accurate empathic understanding achieved significant positive personality and behavioral changes on a wide variety of indexes; while patients whose therapists offered relatively low levels of these interpersonal skills during therapy exhibited deterioration in personality and behavioral functioning.

These studies point to the existence of a "therapeutic personality" that effective therapists possess irrespective of their operational modes (techniques).

Ineffective [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 141 helpers do not possess therapeutic qualities and traits, and their absence, as may be seen from Table 8-2, will damage their capacity to render proper help.

Parenthetically, every helper (or counselor or psychotherapist for that matter) will relate differently to special persons.

Warmth and liking may be shown toward some individuals with whose personalities and problems

the helper identifies and with whom there is a feeling of security.

On the other hand, detachment and hostility may be manifested with other persons toward whom the helper feels alien and who tend to light up countertransference.

All helpers will consistently be more effective with certain individuals than with others.

Counselors are those who have been trained in casework and counseling techniques.

If they have the personality qualities outlined above, such training will enable them to function more expertly than non-trained helpers.

Effective counselors should accordingly score higher in results than effective helpers.

However, irrespective of how thoroughly trained, if counselors lack appropriate personality qualities, they may be classified as ineffective counselors, and the results will be no better than those of ineffective helpers.

When we consider trained psychotherapists, we view a somewhat complex picture since their theoretical and methodologic orientations vary so greatly.

We may expect that the average trained psychotherapist whose personality contains the positive units described above (i.e., an effective therapist) will be able to achieve symptom relief, as well as behavior change, above that brought about by an effective counselor.

Additionally, if the psychotherapist is trained psychoanalytically to do "depth therapy," we may expect that the most extensive and difficult to achieve goal of reconstructive personality change, potentially possible in patients selected for this approach, will at least be double that seen with therapists and counselors who do not employ insight techniques, and who depend on the adventitious operation of constructive relearning alone, which, sometimes, in a favorable atmosphere, may result in some reconstructive change.

Again, though a psychotherapist has had exhaustive training, should that therapist lack the proper personality qualities, the results will be no better than those of an ineffective counselor or an ineffective helper.

Patients may temporarily improve on the basis of non-specific extratherapeutic agencies, but soon these dividends will expend themselves as the [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 142 patients find themselves locked in a frustrating and unrewarding relationship situation.

Where a therapist with suitable traits is not too well trained or skilled in executing therapeutic maneuvers, the results will be no better than those of an effective helper who has had no training.

They will, however, eclipse those of the trained ineffective therapist who may have spent many years in exhaustive postgraduate studies that, though sharpening the cognitive processes, have made no dent on the individual's antitherapeutic personality.

The reason why the patient is impeded rather than benefited by an ineffective helping agency, counselor or psychotherapist is that the patient is blocked, by being trapped in an unrewarding situation, from spontaneously seeking out other helping resources that may bring homeostasis.

Moreover, the relationship with the ineffective helper-counselor-therapist will in all probability promote hostility and release transference distortions that will activate regressive defenses, like hostile dependency, lighting up new and galvanizing old symptoms.

We are inevitably drawn to the awesome and unpleasant conclusion that a person suffering from a psychological problem is better off with no treatment at all than if that person enters an emotionally inadequate helping or therapeutic situation, whether this involves an untrained helper, a trained counselor, or an intellectually sophisticated psychotherapist.

A crucial question is whether the appropriate personality ingredients essential for therapeutic change may be taught those who seek to administer help.

There is some evidence that helpers, counselors, and psychotherapists who do not inherently possess such traits, may be trained to communicate warmth, empathy, and genuineness without themselves entering into depth therapy.

However, this would probably apply only to those personality structures that were not too rigid, hostile, or detached—in short, to basically healthy individuals.

There is some evidence, too, that certain inflexible, hostile, and detached individuals may, with properly conducted personal reconstructive psychotherapy, work through extensive flaws in their personalities and acquire the [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 143 qualities important for functioning as effective psychotherapists.

This development, however, is not at all guaranteed, and we see evidences repeatedly of professional persons who have undergone extensive training, including personal didactic psychoanalysis, who are as arrested in their growth as when they first exposed themselves to treatment, and whose contributions to the ailing masses the world could very well do without. The consequences of their education and training have become the target of critics who are constantly sniping at the results of psychotherapy.

Available statistics do not convey an accurate picture of the potentialities of psychotherapy since they lump together the results of effective with those of ineffective therapists that because of the “deterioration” impact (Bergin, 1963, 1967; Truax & Carkhuff, 1967) cancel each other out and reduce the score to a figure not too much higher than that of the spontaneous cure.

Psychotherapy has vast potentials as a healing force, but it is the product of a complex equation, the elements of which require careful scrutiny, contemplation, and unravelment. It can be executed properly only by selected individuals who inherently possess or have acquired appropriate personality characteristics that will enable them to relate constructively with their patients.

It is enhanced by scholarly grounding and sophisticated postgraduate schooling best available through disciplined educational resources specialized to teach an extremely complex skill.

THE MEASUREMENT OF THERAPEUTIC IMPROVEMENT General procedures for the measurement of therapeutic improvement have been detailed by a number of authorities, including Waskow and Parloff (1975) and Gottman and Mark-man (1978).

Of primary concern is identifying the specific variables that are significant to measure and that give us reliable and valid data.

Of concern also are the research designs that can best provide answers to our questions about outcome.

The instruments that are used for the gauging of outcome must be selected carefully, recognizing that no one instrument is suitable for different patient populations and for varying forms of psychotherapy.

Rather, multiple outcome instruments are indicated.

Among the measures in use today are: self-reporting that deals with the patient's daily functioning (Cartwright, [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 144 1975; Imber, 1975); broad anamnestic material as in the popular Minnesota Multiphasic Personality Interview (Payne & Wiggins, 1972; data from family and friends (Hargreaves et al, 1975; Waskow & Parloff, 1975); a “Community Adjustment Scale” (Ellsworth, 1974); therapist assessment scales (Green et al, 1975; Endicott et al, 1976; Newman & Rinkus, 1978; Mintz et al, 1979); material from community agencies or members (Cummings & Follett, 1968; Halpern & Biner, 1972; Cummings, 1977); and changes in economic and creative output (Riess, 1967; Yates, 1980).

Insofar as research designs are concerned, a number of authorities have offered their ideas, for better or worse, including Glass et al.

(1973), Luborsky et al.

(1975), Bandura (1978), Cronbach (1978), Kazdin (1979), and Cook and Campbell (1979).

Formidable problems exist in any attempt to measure the results of psychotherapy or to verify its empirical propositions (Pumpian-Mindlin, 1956; Kubie, 1960).

Paradigms for psychotherapy evaluation that will identify and possibly quantify clinically relevant parameters that consider the uniqueness of each patient require further development (Glass, 1984).

Ideally, we should like to observe exhaustively what takes place in the course of treatment, to study the results by subjective and objective means, to erect comprehensive hypotheses concerning the relationships between events, to deduce the consequences of such relationships, and to elaborate methods of testing inferences under controlled conditions in order to construct scientifically valid concepts.

Most efforts in this direction, however, applied to the staggering uncertainties of the clinical situation, have merely accented the fallibility of our present research tools and techniques.

The methodologic problems encountered in studies on the outcome of psychotherapy are compounded by semantic befuddlements.

Hazy language and even hazier concepts permeate the field of psychotherapy.

When we attempt to put into words what treatment aims have been achieved, we are handicapped in translating the complex formulations of the different psychotherapeutic approaches into abstractions that possess reasonable unity.

We discover that each school has its particular way of talking about and emphasizing essentially similar elements in human behavior.

The nuances that are being stressed in these ideas are often not as important as the advocates of the particular [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 145 schools would make them out to be.

Another confusing thing is that the same labels may be employed to designate markedly diverse ideas, for instance the words relationship and ego mean entirely different things when used by select theoretical schools.

Furthermore, therapists may inject into words their own private meanings that may not at all resonate with the concepts under examination.

Another pitfall is that the bewildering number of variables, many of which are not manipulable, tempt one to grapple with only a few that can be handled with relative ease.

When these are lifted out of the context of the tremendously complex physiologicpsychologic-sociologic continuum that constitutes human adaptation, they often lose their significance.

We are rewarded with a catalogue of beautifully compiled categories that mean little, particularly when we try to generalize conclusions beyond the material with which we are dealing.

Before proceeding, let us review the outstanding problems in applying scientific method to evaluation studies: 1.

There is disagreement as to which observable phenomena are worthy of observation.

2.

The data available for study are difficult to manipulate and control, interfering with conditions ripe for experiment.

3.

It is cumbersome to qualify the quantitative data of psychotherapy due to the complexity of the variables involved.

4.

Available units of measurement are ill-defined, interfering with comparisons and with the synthetization of similarities and differences into a homogeneous unity.

5.

Theoretic prejudices and personal biases make for a loss of objectivity and an interference with the ability to utilize imagination in hypothetic structuring.

6.

The absence of an accepted conceptual framework that can act as a basis for communication obstructs the formulation of inferential judgments regarding order in the observed phenomena—blocking the deduction of [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 146 valid analogies justified by the available facts, and hindering the exploration of causal connections between antecedents and consequences.

7.

The reliability of our results is distorted by a variety of other difficulties that are related to special problems of the therapist, the resistances of the patient, the amorphous status of diagnosis, the prejudiced selection of the sample, the involvements of outside judges, coders, and raters, the inability to employ adequate controls, the interferences of adventitious non-specific changes, and certain complexes inherent in the psychotherapeutic process itself. With advances in computer technology, the development of treatment manuals to standardize therapeutic operations, the use of video tape recordings to enable independent assessors to monitor adherence to the prescribed techniques, the greater employment of adequate controls, the development of more sophisticated research designs, and improved follow-up studies on research, findings should become more reliable.

The question is how much will such findings enhance our techniques for the better.

The answer to this is still uncertain because individual interpretations of research findings can be different, and styles of operation vary so much that one therapist may completely foul up a prescribed method that another has utilized with great effectiveness.

This is one problem that a prepared “Manual of Operations” poses.

How the manual fits in with the preferred style of a therapist will determine its usefulness.

It may put some therapists into a straight jacket and may totally cripple their spontaneity and flexibility.

On the other hand, it may structure operations for some and make them more precise.

What a therapeutic manual may be able to do is to define more precisely what enters into a specific form of treatment (behavioral, psychoanalytic, eclectic, etc.).

When we examine what therapists do under the banner of a specific form of psychotherapy, we sometimes find that their operations deviate from what is commonly accepted as standard (Strupp, 1978).

Problems Related to the Therapist Not the least of the sources of error are the individuals who supply the data on [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 147 which we are dependent, particularly the therapists and the patients (Lambert & Utic, 1978; Parloff et al, 1978b).

A psychotherapist's assessment of what is accomplished in treating a patient is apt to tell us more about the therapist's narcissism than it does about what actually happened to the patient. One pointed example of how therapists may contaminate the data is their direct or indirect influencing of the verbal content.

This is significantly affected by comments and even indications of approval or disapproval as contained in nonverbal cues (Greenspoon, 1954a,b).

The material that is brought out may consequently be a facsimile of the therapist's ideas and values that have been subtly communicated and are now being regurgitated by the patient upon proper stimulation.

We repeatedly are confronted with productions from patients that seem to validate the specific theoretical systems espoused by the therapist.

Most psychotherapists have a tremendous investment in their theories and methods.

By the time they have completed training they have some firmly set ideas about human behavior. As students in training, affirmation of the approach in which they are being groomed offers rewards of approbation, of completion of training, of admission to the special graduate society



with status privileges, and of economic security in the form of patient referrals.

Denial of the verity of the system poses the hazards of being accused of hostile resistance, of exclusion from the ranks of the privileged, of dismissal from training, of excommunication and possible financial doom.

Against these odds, the preservation of the thinking integrity is put under a strain that few students can resist.

Glover (1952) has remarked, "There is a tendency in the training situation to perpetuate error." This error may be carried over to the patient who is the recipient of the therapist's values, no matter how passive and nondirective the therapist imagines himself or herself to be. The very nature of the psychotherapeutic process demands that the therapist have a conviction about what he or she is doing.

As.

F.

C.

Rhodes Chalke remarked in his presidential caveat at the Canadian Psychiatric Association (Psychiatric News 9:1, 1974): "It has been demonstrated that much of the therapeutic effectiveness of psychiatrists resides in an inherent faith in one's curative powers.

This can be significantly diminished if one were to be confronted in measured terms with one's failures or successes." [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 148 Dedication to one's system is an important constituent in the therapist's mental set.

If the therapist evinces a scientific attitude of skepticism, it may be interpreted by a patient as unsureness and lack of belief in the method.

This may influence adversely the patient's expectations of help and reduce faith in the therapist.

The therapist, therefore, may find that dogma tends to stabilize some patients as well as himself or herself.

While it may help one area of functioning, it does introduce errors that may be fatal in the experimental evaluation of therapeutic activities and results.

The therapist's bias also extends itself to patient selection, so that our examining the results may be handicapped by a restricted sample.

Therapists are inclined to select patients who will, according to their experience, respond best to their methods.

A not too inconspicuous screening process may be employed to eliminate a wide spectrum of patients and problems.

In intensive long-term therapy, an economic factor also enters into the picture, an important criterion of selection being, not the syndrome or personality, but the ability of the individual to afford the financial drain that will have been made.

Thus the patients who avail themselves of long-term treatment may belong to a different subcultural group than those who apply for short-term therapy, with value systems and social pressures that make for distinctive reactions and singular responses to psychotherapy.

Economic and status factors also interfere with the reliability of a therapist's accounting of results.

As a recording apparatus, the therapist possesses many defects.

Bias may divert from presenting factual data, leading the therapist to some erroneous conclusions.

Berg (1952) has humorously commented that when a patient loses his or her job, the therapist will find outside factors responsible; however, when the patient gets a salary increase, the therapist is willing to take credit for singular clinical competence.

A therapist is generally unwilling to stand by passively and offer an unprejudiced account of

why patients have failed to get well.

He or she has, as Greenacre et al.

(1948) pointed out, "too great a stake in the patient's recovery." When a researcher seeks to examine records or to listen to session recordings, the therapist most likely will refuse to deliver them.

Why become a sacrificial victim?

Why should one be a party to one's own discredit as a [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 149 therapist?

What constitutes a well-trained professional person who is qualified to do good psychotherapy is another delicate matter around which many heads have been shattered.

Avoiding the ever annoying topic of medical-non-medical, we can say that an individual who has been graduated from a reputable postgraduate school run by qualified and trained psychotherapists, which has an adequate screening process for candidates, a wellformulated program of didactic instruction, competent and elaborate supervision of a varied caseload, and provisions for personal psychotherapy or psychoanalysis of its students, can arbitrarily be considered a psychotherapist.

A study of cases handled psychotherapeutically by persons whose qualifications are questionable is certainly not a reliable index of the value of psychotherapeutic techniques.

After all, not every person who drives a nail can be considered a cabinetmaker.

Among the graduates of postgraduate schools for psychotherapy will be those with relatively fair, average, and excellent degrees of competence; there will be differences in the results of the individual practitioners with special kinds of patients and problems; there will be varying personality difficulties and aberrant characteristics among them that will determine, as has been indicated in an earlier part of this chapter, whether they will be effective or ineffective psychotherapists.

Of importance also is the ability to utilize methodologies that have proven valuable for special syndromes.

Many therapists attempt to squeeze all patients into the kinds of technique with which they are familiar.

This is understandable, but the patient may become a sacrificial victim as a consequence.

**Problems Related to the Patient** There are many sources of error in the patient's statements about results achieved in therapy.

Notoriously, many patients are poor judges of what actually is going on, but unfortunately, they may be the only witnesses at our disposal to testify.

They may, however, prove to be reluctant witnesses.

Patients are motivated to seek help for their suffering; their objective is not to offer themselves as research vehicles.

Asking them to subject themselves to psychological studies, to interviews with observers and research [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 150 workers, and to follow-up manipulations may mobilize attitudes and feelings that negate therapeutic aims.

It is always necessary to validate, by objective criteria, subjective reports by patients, good or bad, since their judgments are obviously colored by resistances.

These may operate either in the service of a false report of well-being, as may be the case where the patient conceives of further therapy as a threat; or they may evoke a depreciation of legitimate developmental progress, for instance, where such progress does not conform with personal value systems or those of the subcultural group to which he or she belongs.

The patient may have firm convictions of what is desired out of therapy that do not parallel the objectives of the therapist or meet the goals of reasonably good mental health.

For example, Mary Jones may be interested merely in resolving her tension, neutralizing her

anxiety, and reducing her symptoms so that she can function comfortably.

She may be willing to restrict specific life functions, adopt protective or precautionary defenses, or forego important measures of self-fulfillment in order to achieve peace and relief from suffering.

From her standpoint her therapy will be a success, if she achieves these goals, and if she symptomatically feels better she may consider herself cured.

From the standpoint of the therapist, Mary Jones may be only moderately improved, since she has not altered any of her basic life patterns.

From the yardstick of ideal mental health objectives, she may have retrogressed, in view of her abandonment of vital human functions.

The Problem of Diagnosis Another factor that interferes with good evaluative research is that our present-day psychiatric nosologic systems are still in need of reordering.

They embrace conglomerate labels that depict anxiety, symptomatic manifestations of anxiety, defenses against anxiety, character traits, and disintegrative phenomena.

Often a diagnosis is made of that symptom complex that is most disturbing to the individual even though coordinate pathologic elements exist that are more fundamental and more serious, but happen to be less annoying to the patient than the complaint factor.

Or the therapist may favor a special [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 151 group of manifestations and focus attention on these.

Even well-trained therapists examining the same case may, therefore, arrive at different diagnoses.

This is, to say the least, confusing to the researcher who attempts to establish some unity among the diagnostic categories.

The sharpening of diagnostic systems would certainly lead to a greater clarity about goals in therapy and help in our evaluative effort.

The authors of DSM-III and DSM-III R have made estimable progress toward this end.

New attempts are now in process, of working out more suitable classifications.

The problem is a most complex one since emotional difficulties influence every facet of functioning—physiologic, intrapsychic, interpersonal, and community relationships.

What distinguishes one individual from another is the unique configuration of tendencies and traits in the form of residues of faulty conditionings, fingerings of immature personality promptings, defective modes of dealing with aggression and sexuality, manifestations of anxiety and its neutralized derivatives, and defensive and characterologic distortions.

A person is qualitatively different from others because each individual is quantitatively constituted and disposed differently.

What we perhaps need to do in establishing a more serviceable classification is to determine which clusters of traits and problems, or combinations of the latter, respond best to special therapeutic approaches.

The ability to define significant clusters that have responded to special treatment attempts presupposes that we have a way of identifying them.

Of great handicap is improper, and even downright irresponsible, recording at the time treatment was begun.

It is manifestly impossible to gauge what has been accomplished for an individual if we have no starting point from which we can begin our measurements.

Too often we find ourselves desperately grasping for clues, attempting to reconstruct from memory or from the feeble jottings in our notebooks the approximate status at the beginning of treatment.

The fact that so many areas are pathologically afflicted in any emotional problem makes inadequate our simple designations of change.

Labels of “cured,” “much improved,” “moderately improved,” “unchanged,” and “worse” mean little in view of the complex systems with which we are dealing.

Rarely is the patient influenced similarly throughout the physio-psycho-sociologic spectrum. The patient may have experienced great [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 152 symptomatic improvement and even cure of a disabling condition, yet the patient’s interpersonal difficulties may continue in force.

The patient may have improved in work capacities and community relations, but marital and sexual adjustment may have deteriorated.

When we utilize ratings of change, it would seem important, therefore, to assign these to the areas to which they relate.

This would enable us better to appraise the specific zones of disturbance that respond to special approaches and techniques.

In group therapy, for example, we may find that the patient derives great help for interpersonal problems, while entrenched intrapsychic defenses have scarcely budged.

Among the areas we may consider for our ratings are the following: 1.

Changes in manifest symptoms 2.

Changes in interpersonal, family, work, educational, and community adjustment 3.

Changes in physical health 4.

Changes in feeding, eliminative, sexual, rest, sleep, and activity patterns 5.

Changes in consumption of alcohol, sedatives, tranquilizers, and other medications A fruitful area of research is in the development of scales by which we can assess the various outcome criteria.

We may then better be able to compare the results of varying kinds of therapy, performed by different therapists, with diverse types of problems and patients, and then contrast these results with what happens when no formal treatment has been administered.

Some years ago attempts in this direction were made by defining change in descriptive and operational terms without evaluating the changes (Witmer, 1935; Hunt & Kogan, 1950; Watterson, 1954).

How an evaluative scale may then be employed to utilize such descriptions has been described by Malamud (1946).

The Problem of the Sample In evaluating a psychotherapeutic system one may tend to neglect how the sample was selected.

Because many therapists screen their patients, choosing those whose [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 153 problems, in their opinion, are best suited to their methods and skills, a prejudiced weighting of the caseload is possible.

Our conclusions regarding the worth of a therapy, therefore, may have to be restricted to a special group of patients and problems.

For instance, an individual applying for formal psychoanalysis may be rejected if that individual does not possess a balance of characteristics that are lumped under the designation of “ego strength.” Other therapies may practice no partisanship in choosing candidates for their methods.

Attempts to contrast results between different therapies must obviously take into account the fact that we may be dealing with varying populations.

Thus, O’Connor and Stern (1972) studied the effects of psychoanalysis (4 sessions weekly for a minimum of two years) and psychotherapy (semiweekly for no longer than two years) on 96 patients with functional sexual disorders.

They reported an improvement rate of 77 percent with psychoanalysis and only 46 percent with psychotherapy.

This might sound significant except that the group selected for psychoanalysis “evidenced less

illness than those who received psychotherapy.” We are dealing then, at least in the area of this sample, with two dissimilar groups.

Some workers, in judging the results of treatment, include all individuals who have been interviewed and accepted for therapy, even though they do not continue beyond a few sessions. Others exclude persons who have discontinued therapy against advice.

Obviously this discrimination alters the percentage of individuals improved.

It is very easy to manipulate statistics to substantiate almost any bias, such partiality being not always unconscious.

Another error commonly overlooked is failure to mention the cultural and subcultural background of the persons composing the sample.

Therapeutic results are influenced to a marked degree by conditions in the social environment to which the patients must adapt themselves after they have completed their treatment.

Indeed, certain neurotic defenses may be mandatory if the individual is to survive in a predatory environment.

Value systems regarded as constructive from the standpoint of ideal mental health may be a source of victimization where one must adapt to groups perpetuating erratic conventions and folkways.

It is essential in doing comparative studies to include a [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 154 description of the individuals who are being evaluated, how they were selected, their background, their socioeconomic status, the length of treatment, and other information that might influence the treatment results.

What is important also is when in time evaluations are made “whether repeatedly, during the course of, or immediately afterwards, or at variable and increasing intervals after it is over” (Kubie, 1973).

These considerations will help reduce the error involved in generalizing beyond the sample to the population at large.

In our effort to select a homogeneous sample, we must keep in mind the fact that no two persons are alike even though their diagnosis is similar.

Thus individuals in a sample of agoraphobic patients will vary in their physiologic makeup, developmental conditionings, educational backgrounds, environmental experiences, intrapsychic structures, personality organizations, living milieu, values, and sundry other personal determinants, including age, severity of symptoms, and motivation.

These will make every patient a totally unique human being, who in a pool of other human beings, looks and behaves in certain ways distinctively from the rest.

In responses to the same kind of psychotherapy, we may expect differences.

In measuring change we need to know what area or areas are to be assessed.

The selection of different criteria by which to estimate improvement or cure has led to a great deal of confusion.

Thus the measure of successful problem-solving, symptom relief, or behavior change accepted by behavior therapists may not be acceptable to psychoanalysts who gauge improvement by reconstructive personality alterations.

Neither may be considered basic by therapists practicing humanistic approaches whose criterion would be inner peace, happiness, and creative self-fulfillment.

What has been suggested is the establishment of multiple measures that cut across the different kinds of psychotherapy.

Therapeutic changes involve many facets of an individual's functioning, not all of which are easily measurable.

Among these are symptoms, relationships with people, values, self-esteem, work capacity, self-image, economic status, creativity, etc.

No patient progresses equally along all dimensions of possible change.

To assess these we would [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 155 require a variety of instruments. The usual pronouncement of "improvement" or "cure" generally connotes merely a relief of symptoms, and does not indicate what sacrifices are being made to achieve this.

For example, a patient with depression initiated by loss of a loved person who served as a maternal object may find his or her depression cured when a new maternal companion is found. Should the patient coordinately be in therapy, the cure falsely may be ascribed by the patient to the effect of treatment.

In estimating change, we are confronted with the dilemma that the multiple change criteria with which we deal are not standard and that situation-specific behaviors (like efficiency at work) are more easily assessed than personality traits.

To bring some order to this muddle attempts have been made to establish a battery of measuring devices.

Waskow and Parloff (1975) have recommended as a standard test battery: the Minnesota Multiphasic Personality Inventory (Dahlstrom et al, 1972); the Hopkins Symptom Checklist (Derogatis et al, 1973); the Psychiatric Status Schedule (Spitzer et al, 1967, 1970); Target Complaints (Battle et al, 1966); and a choice of either the Personal Adjustment and Role Skills Scales (Ellsworth, 1975), or the Katz Adjustment Scales (Katz & Lyerly, 1963).

The value of these or any other proposed batteries will require further evaluation.

For the most part, outcome assessment will rely on the patient's divulgements, on disclosures of the therapist, as well as on the reports of family and friends, all of which may be highly biased.

Nevertheless, we may have no other way of assessing change than through these declarations and through the use of instruments such as the battery of tests cited above by Waskow and Parloff (1975).

Lambert (1979) has written an excellent review of measurement batteries, and useful measures have been described by Miles et al, 1951; Lorr and McNair, 1965; Strupp et al, 1969; Malan, 1976a; and Meldman et al, 1977.

Insofar as personality tests are concerned, they have not been found too useful (Mischel, 1977).

The Use of Outside Judges, Coders, and Raters Assuming that we are able to define concretely the variables that we wish to [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 156 observe, we would heighten the reliability of our observations to employ a number of competent and trained judges to go over the data on which we will base our estimates of therapeutic change.

A number of problems arise here related to the confidential nature of the material, the need on the part of the patient to retain anonymity, the belief that observational intrusions alter the behaviors of patients and therapists, and the fear of the therapist that his or her therapeutic competence may come under challenge.

During the training process in an outpatient clinical setting, the patient may be prepared to accept outside adjuncts as part of the treatment and the therapist may be motivated to work with raters and coders.

In private practice, however, this is practically impossible.

In the event we are able to employ qualified accessories, the reliability of the measuring and rating instruments that we have at our disposal at the present time is probably high enough to permit consistency in our results (Herzog, 1959).

It is essential, however, that we have dependable data, that we be explicit in the definitions of what is to be coded and rated, and that the accessory workers be properly qualified and trained in the use of the selected categories.

Whether or not accessories are used, once we have settled on the categories to be rated and are

satisfied that our methods of rating are reliable, we must still question the validity of what we are doing.

Let us say that we have accurate statistics about changes in symptoms and adaptive patterns; does this necessarily give us accurate information about the emotional status, whether the individual has been cured, is improved or unimproved?

An example may make this clear.

In going over the record of a patient who has completed therapy, the patient's work performance will come up for appraisal, work being one of the factors in evaluation.

We discover that shortly after leaving therapy, the patient was demoted.

This does not seem to be a good indication of the patient having made progress in treatment.

Yet when we examine the circumstances closely, we see that the basis for the patient's demotion is that he or she has become less obsequious and masochistic; hence the patient is more capable of resisting unfair demands.

This independence surely will not endear the patient to a domineering and exploitative foreman or employer.

On the other hand, it is conceivable that an increase in salary or job advancement may be the product of [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 157 a neurotic acquisition of overambitiousness and self-exploitative character traits that drive a person to be dedicated to the job with a merciless grimness that is so often rewarded by money and position.

A safeguard to the faulty assessment of environmental data is that each item is not viewed in itself, but rather is related to the totality of the patient's adaptation.

Not only must reports of environmental adjustment be questioned for their validity, but other indices of change must also be considered with caution, and only in relationship to the entire range of indices.

The relative weights to be given to each variable in the adjustment picture is a matter that needs to be decided individually after all of the elements have been put together.

Obviously, variables of change cannot be scored on an even basis, nor can the emphasis in rating extended to a set of criteria in one individual be transferred to any other individual.

The total constellation of forces that operate will determine the unique emphasis, if any, that is given to each variable.

Ratings of adjustment following a point scale system are consequently invalid unless the scores are considered of unequal weight, depending on special circumstances.

Because the weighting of so many of our items is based on the opinion of the observer, the element of prejudice cannot be eliminated from our results (Miles et al, 1951).

The sole safeguard we have is the background, training experience, and reputation of the researcher, which gives us some indication as to the researcher's reliability.

Yet we cannot be at all certain that a researcher for emotional and other reasons, consciously or unconsciously, may not abandon objectivity for the triumph of verifying a preconceived idea.

A good researcher is one who not merely can do an analysis of variance or compute a chi square, but more importantly, perhaps on the basis of personal experience, can blend common sense with the nuances of therapy.

**The Problem of Controls** The problem of controls in psychotherapy is perplexing.

Let us suppose that we have a clinic staffed by trained, competent psychotherapists who employ accepted psychotherapeutic techniques, and that we reject every other patient applying to the clinic for help and utilize the rejects as controls.

Assuming that we have developed proper [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 158 criteria for evaluating change in the direction of mental health, that we have evolved efficient ways of data gathering, and a methodology that enables us to deal expertly with this information, we still would not be able to say with reasonable certainty that psychotherapy was the only

dimension of difference between the experimental and the control groups.

Our quandary is that there is no such thing as exactly the same kind of an emotional problem. Subtle and oft undetectable cognitive, emotional and behavioral differences within the same syndrome may actually be the determining factor.

By no stretch of the imagination can we say that the environmental pressures, or the healing adventitious situational elements, that impinge themselves on any two people are the same, nor can we keep them anywhere near constant throughout the period of our differential study.

Experience with control groups shows that it is difficult to regulate the lives of human beings so that they behave the way a research sample should.

The matching of similar cases is, therefore, a haphazard process, based more upon hunches than facts.

Indeed any attempt to use controls may introduce new errors.

It is conceivable that if we followed thousands of cases that had been treated over a period of years, as well as a roughly similar number of control subjects, our errors would be reduced. However, this could not be guaranteed even though we could afford the formidable costs that such research would entail.

From a practical standpoint we are limited in using the sound experimental method of pairing individuals as a means of equating experimental and control groups with respect to relevant variables.

The proposals of utilizing each individual as his or her own control in the tactic of "wait" control (Dymond, 1955), the analysis of variance (Miller, 1954), the use of statistical control through covariance methods (Dressel, 1953), the dealing with control phenomena through "control in data" (Gordon et al, 1954), casual visits but no real treatment ("attention" or "placebo" control) (Kazdin & Wilson, 1978), treatment as needed ("PRN contact") (Weissman, 1979b), and the employing of the principles of "randomization" (McNemar, 1949) are ingenious, but they do not resolve all of the essential problems, no matter how we manipulate the sophisticated statistical devices we have at our disposal.

It is difficult to obtain a large number of homogenous patients who randomly can be assigned to special treatments for a set period of time.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 159 The use of dropouts ("terminator controls") of those who fail to keep any appointment or stop visits early in treatment also has many flaws (Gottman & Markman, 1978).

As a way out, it has been suggested that, instead of using a control group of untreated individuals, we employ two groups of the same population being treated by different methods. Our results would surely then be dependent upon the skill and experience of the therapists, accepting the contention that good therapists get approximately the same results, irrespective of their theoretical and methodologic differences (Fiedler, 1950a,b, 1951).

We would have to make sure that therapists practicing different methods be comparable in ability.

Zubin's (1953) idea of establishing a "standard control group" that would act as a basis for comparison with treated groups is interesting, but, as he puts it, still somewhat idealistic.

It is impossible to match patients on any more than a few personality variables.

Frequently a "no-therapy" group is set up as a control after an initial interview.

No-therapy is an invalid concept.

An initial interview is a form of therapy and patients can benefit significantly from a single contact with a trained professional person.

After the initial interview, the patient who is not accepted for therapy does not exist in a vacuum.

That patient will exploit many measures to relieve his or her symptoms or to resolve problems.



These range from tranquilizers, to self-help measures, to relationships with sundry individuals through whom the patient may work through some of the difficulties.

The idea that the patient is receiving no therapy then is not true, even though the therapy is non-formal.

Other problems relate to informed consent in order to protect the rights of patients, and to the ethical issue of withholding treatment from a needy patient who happens to fall into the control group.

To disclose to a patient that he or she is being used as a guinea pig, and an untreated one at that, may not meet with universal acceptance and may thus compromise the research design.

To deny a patient treatment we know is needed constitutes a dilemma that the lofty principle of research for the sake of science cannot resolve.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 160 The Problem of Adventitious Change One of the bewilderments of evaluation is that we have little against which we can compare our results.

Statistics, as has been previously indicated, generally uphold the dictum that approximately two-thirds of all patients improve irrespective of the kind of psychotherapy to which they have been exposed.

They also attest to the fact that approximately two-thirds of persons with emotional problems also improve by arranging for their own destinies.

These figures mean next to nothing because we have no idea as to what is connoted by the words "improvement" or "psychotherapy" or "no treatment whatsoever." We have no data on the practitioners who have presumably rendered therapy, nor on the constituent problems and syndromes, as well as their severity, that have been treated in contrast with those that have not been treated.

We have no idea of the specific parameters of personality influenced in the patients who have received psychotherapy, as opposed to those who have achieved stabilization through the circumstance of extratherapeutic forces.

Are persons who apply for professional help those who have in greater proportion failed to achieve benefit through extratherapeutic elements?

It would seem more than coincidental that individuals who have exploited every device and resource in quest of relief from anxiety begin to improve with the institution of psychotherapy.

Problems in Dealing with the Statistical Data It is not irrelevant to point out that research studies in mental health can be flawed by an improper analysis and reporting of the quantitative results.

Spitzer and Cohen (1968) describe three common errors: inability to distinguish between statistical significance and magnitude of association, measurement of reliability, and neglect of statistical power analysis.

Because professional people in the field of therapy are generally untrained in quantitative techniques, it is essential that a researcher skilled in statistical methods be consulted whenever a serious research study is contemplated.

Before data is collected, it is important carefully to review the hypotheses to be tested, the kinds of data to be accumulated, and the statistical techniques to be employed.

This emphasis does not justify a deification of statistical methods as ends in themselves, but rather as tools that [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 161 can order massive data and lead to reasonably valid inferences.

**CONCLUSION** Concern with the rising costs of health care has focused the spotlight on the safety and cost-effectiveness of psychotherapy.

Governmental authorities and insurers are asking for proof regarding the usefulness of the various kinds of psychotherapy.

Can we verify the worthwhileness of an expensive project of psychiatric or psychological treatment?

The difficulty of supplying scientific evidence of merit is complicated by the fact that, no matter how good a species of psychotherapy may be, it will not prove costeffective in the hands of a bad therapist.

But even if we accept what the most dubious researchers now concede, that psychotherapy is at least minimally effective and better than no treatment or the use of a placebo, most impartial observers would have to consider it a beneficial enterprise.

But can we say it is cost-effective and that the benefits justify the expenditure of time, effort, and money?

This depends on how we rate the tangible and intangible costs of emotional disturbance and how much monetary value we put on human suffering and the misfortunes psychological illness foists on the community.

When we consider the misery wrought by neurotic symptoms—the awesome damage to families, the wrecked marriages, the derailed lives, and the shattered productivity that follow in the wake of a neurosis—and add to these calamities, crimes, delinquencies, rapes, arsons, murders, suicides, violence in the streets, and the ravages of alcoholism and drug addiction that are neurotically or psychotically inspired, we may ask: “How costly is it to society not to try to prevent these tragedies through some kind of corrective procedure?” Is not even a minimally effective solution better than no solution at all?

The motive on the part of governmental authorities for inquiring into the safety and cost-effectiveness of psychotherapy is understandable.

The various procedures employed today in treating mental and emotional problems, such as dynamically oriented psychotherapy, behavior therapy, family therapy, group therapy, marital therapy, pharmacotherapy, and others, are substantially safe and effective when executed by trained, experienced, and skilled professionals.

What makes a procedure unsafe and [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 162 ineffective is not the technique itself, but how it is applied.

A scalpel in the hands of an unskilled surgeon can be a dangerous and useless instrument.

Pardes, while Director of the National Institute of Mental Health, pointed out that the question of solid proof of treatment effectiveness extends across the entire health care field.

In a 1978 report from the Congressional Office of Technology Assessment, only 10 to 20 percent of all health care technology had been proven effective by formal methods.

Many of the commonly employed medical procedures have never been satisfactorily evaluated.

Controlled studies in the mental health field definitely show that psychological treatments rate at least no worse than treatments in medicine and surgery.

But further research in psychotherapy is necessary.

Research in psychotherapy is still burdened by many handicaps.

Yet the literature is replete with studies flaunting impressive statistics that “prove” the superiority of one brand of psychotherapy over others or that downgrade all forms of psychotherapy as worthless or limited at best.

We still do not possess a model of psychotherapy research that we can consider uniquely applicable to the special problems and conditions existing in psychotherapy (Frank, 1979; Karasu et al, 1984).

Nevertheless, in the opinion of the majority of practitioners, of patients who have received treatment, and of unprejudiced observers, psychotherapy, properly instituted, is the most effective measure available to us today for the treatment of emotional problems and for the liberation of potential adaptive and creative resources in the individual.

The fact that research in psychotherapy to this date has had surprisingly little impact on

contemporary clinical practice, should not discourage future attempts to substantiate the effect of psychotherapy by scientific means.

There is evidence that with recognition of the complexity of the variables involved and vitality and sophistication that is currently being manifested by researchers in the field, the outlook is an optimistic one.

Malan (1973) in a historical review terminates his paper with the prediction that we will eventually find "that there are particular techniques appropriate for particular types of patients, which give outcomes for which words written very long ago by Kessel and Hyman (1933) are appropriate: 'this patient was saved from an inferno, and we are

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 163 convinced this could have been achieved by no other method.'"  
" [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 164 9 Supportive Therapy The object in supportive therapy is to bring the patient to an emotional equilibrium as rapidly as possible, with amelioration of symptoms, so that the patient can function at approximately his or her norm.

An effort is made to strengthen existing defenses as well as to elaborate better "mechanisms of control." Coordinately, one attempts to remove or to reduce detrimental external factors that act as sources of stress.

There is no intent to change personality structure, although constructive characterologic alterations may develop serendipitously when mastery has been restored and successful new adaptations achieved.

There are times when supportive therapy is all that is needed to bring about adequate functioning.

This is the case where the basic ego structure is essentially sound, having broken down under the impact of extraordinary severe strains that sap the vitality of the individual.

A short period of supportive therapy will usually suffice to restore equilibrium.

On the opposite end of the pathological scale are those victims so deeply scarred by childhood experiences that the radical surgery of intensive psychotherapy can only increase the disfigurement.

The only practical thing we can do for some of these casualties is the topical cosmetic repair of symptom alleviation so that they can live more comfortably in spite of their handicaps.

While we do not yet possess the diagnostic tools to assess accurately which patients will benefit most from supportive expedients, some therapists rely on the pragmatic principal of resorting to supportive therapy where more extensive measures fail to produce an adequate response.

On the other hand, where case loads are overwhelming, therapists may be inclined to utilize supportive measures as a routine, reassigning patients for educational or reconstructive approaches where results are not satisfactory.

Since supportive therapy attempts the achievement of symptom relief or symptom removal, the question presents itself regarding the value of symptomatic cure.

Among the [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 165 most solecistic of legends is the notion that elimination of symptoms is shallow if not worthless.

This notion stems from the steam engine model of psychodynamics that conceives of energy in a closed system, bound by symptomatic defenses, which when removed, releases new, more dreadful troubles.

This is in spite of the fact that physicians from the time of Hippocrates have applied themselves to symptom removal in both organic and functional ailments with little or no baneful consequence.

Yet, legends survive from one generation to the next.

The presumed dangers of symptom removal are now as threatening to some younger therapists as they were to their teachers.

Little proof is offered of a causal relationship between the fact of symptom relief or removal and any pathological sequelae.

The evidence persuades that supposedly precipitated disasters are either coincidental or the product of inept therapeutic interventions (Spiegel, H. 1966).

The complaint, then, that symptom removal is an arbitrary, incomplete, irrational, and unsatisfactory approach in psychotherapy is apocryphal.

Yet, we cannot entirely dismiss the anecdotal accounts and experimental studies (Szasz, 1949; Browning & Houseworth, 1953; Seitz, 1953; Jones, HG, 1956; Paneth, 1959; Crisp, 1966; Bruch, 1974a) of infelicitous effects, but on the whole the empirical evidence for symptom substitution is not consistent.

I have, in my own consultative practice, seen occasional untoward results with supportive approaches.

However, I have also seen baneful consequences with reeducative and reconstructive approaches. My impression is that it is not the method that produces bad results; rather it is how the method is applied, as well as the fragility of the ego structure on whom it is applied.

A callous therapist who batters away with any technique without adapting tactics to the immediate reactions and sensitivities of patients may become a greater instrument of harm than of help.

The transference reaction can negate any therapeutic benefits; indeed, it may in itself be responsible for a therapeutic debacle.

Experience convinces that supportive measures, carefully and selectively applied, can, as a result of desirable symptom alleviation, promote substantive behavioral benefits.

In summary, symptom relief or removal is an essential goal in any useful psychotherapeutic program.

It may occur "spontaneously" or be brought about by a variety of methods, such as by drugs, by conditioning techniques, by faith and prayer, and by insight.

While one may not agree with Eysenck (1960a) that elimination of a symptom [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 166 cures the neurosis, there is evidence that it contributes to a better adjustment and to the elimination of auxiliary symptoms clustered around the original complaints.

Any therapy that leads to enlightenment and greater self-understanding without symptom relief may legitimately be regarded as a mediocre success if not a failure.

But this does not mean that symptom control should constitute the sole objective of therapy.

Undermining of the basis of the individual's symptoms, the resolution of past conditionings and current conflicts that nurture more symptoms expands one's potentials for happiness and creativity.

While such an objective may come about as a byproduct of symptomatic change, its studied achievement should be an important target.

In many patients receiving supportive therapy, consequently, an effort should be made to motivate them toward some kind of reeducative or reconstructive change in order to insure greater permanence of results.

Supportive measures thus may be utilized as the principal treatment or as adjuncts to reeducative or reconstructive psychotherapy.

They are employed as: 1.

A short-term exigency or expedient for basically sound personality structures, momentarily submerged by transient pressures that the individual cannot handle.

2.

A primary long-term means of keeping chronically sick patients in homeostasis.

3.

A way of “ego building” to encourage a dedication to more reintegrative psychotherapeutic tasks.

4.

A temporary expedient during insight therapy when anxiety becomes too strong for existing coping capacities.

An understanding of how and when to do supportive psychotherapy is, therefore, indispensable in the training of the psychotherapist.

Supportive therapy does not work in many cases where the problems with authority are so severe that the patient automatically goes into competition with the helping individual, depreciating, seeking to control, acting aggressive and hostile, detaching [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 167 himself or herself, or becoming inordinately helpless.

These reactions, appearing during therapy, may act as insurmountable resistances to the acceptance of even supportive help.

Therapies with designations of “palliative psychotherapy,” “social therapy,” “situational therapy,” and “milieu therapy” fall into the supportive category.

Many of the tactics utilized are similar to those in casework and counseling.

Among procedures employed in supportive therapy are guidance, tension control and release, environmental manipulation, externalization of interests, reassurance, prestige suggestion, pressure and coercion, persuasion, and inspirational group therapy.

**GUIDANCE** Guidance is the term given to a number of procedures that provide active help, in the form of fact giving and interpretation, in such matters as education (educational guidance), employment (vocational guidance), health, and social relationships.

Many casework, counseling, and educational operations come under the category of guidance.

The extent to which the patient’s life is manipulated varies with the patient’s condition and with the system of guidance employed.

Most of the guidance schemes are patterned after those described years ago by Payot (1909), Vitoz (1913), Walsh (1913a, b), Barrett (1915, 1925), Eymiew (1922), Traxler (1945), and, Erickson (1947).

The role that the therapist plays may be that of a directing authority who arranges for a planned daily regimen and allows no time for idleness and destructive rumination.

A balanced program may thus be organized, relating to the time of arising, bodily care and grooming, working schedule, diet, rest periods, recreation, sleep, and other activities that will account for every hour of the day.

Such complete control of the patient’s routines, however, is rarely necessary.

Usually, guidance is aimed at a specific disturbing problem that interferes with adjustment.

Instruction is given in ways of detecting, examining, and avoiding stressful situations.

Courses of action realistically suited to the problem are then outlined.

Recommendations may also be made toward specific adaptive goals, like enhancing one’s career, furthering one’s education, etc.

This advice is particularly helpful to individuals whose emotional problems are not too severe.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 168 Guidance is based on an authoritarian relationship established between therapist and patient.

One of the problems inherent in such a relationship is that a dependent patient may tend to overestimate the capacities and abilities of the therapist to a point where the patient’s reasoning abilities and rights to criticize are suspended.

Under these circumstances any doubt regarding the strength or wisdom of the authority will arouse strong insecurity.

Hostility and guilt feelings, if they develop at all, may be rigidly repressed for fear of

counterhostility or disapproval.

One may recognize in such irrational patterns the same attitudes that the child harbors toward an omnipotent parent.

Actually, the emotional helplessness of the neurotic individual resembles, to a strong degree, the helplessness of the immature child.

The neurotic person may project the original authority that was invested in parents, and may be seeking from the therapist extravagant evidences of support and love.

Characterologically dependent persons particularly demand demonstrations of infallibility.

Should the therapist display human frailties or appear to lack invincible qualities, the faith of the person may be shattered, precipitating helplessness and anxiety.

Mastery may then be sought by annexing oneself to another agency in whom magical and godlike features lacking in the previous host are anticipated.

The life history of such dependent individuals shows a flitting from one therapist to another, from clinic to clinic, from shrine to cult, in a ceaseless search for a parental figure who can guide them to paths of health and accomplishment.

Because they have so often been disappointed in this search, some persons will resent guidance, even though they feel too insecure within themselves to direct their own activities.

Others will reject guidance because of previous experiences with an authority who has been hostile or rejecting or who has made such demands on them for compliance as to thwart their impulses for self-growth.

Acceptance of advice may, to certain individuals, be tantamount to giving up their independent claims on life.

In spite of its disadvantages, guidance may be the only type of treatment to which some patients will respond.

Desperately helpless in the grip of their neurosis, such individuals have neither the motivation nor the strength to work with a technique that [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 169 requires self-direction.

Resistance to self-assertiveness is so strong that a parental figure must prod them into performing their daily tasks.

A guidance toward religion is sometimes deemed expedient (Holman, 1932; Blanton & Peale, 1940; Poole & Blanton, 1950).

An attempt is made here to convince the patient that health and self-fulfillment can be achieved by self-devotion to prayer.

In yielding to a stronger power, the patient can crush dread, overcome fear, and even achieve peace of mind.

Christian Science (Bates & Dittmore, 1932) and other "faith cures" draw on these principles.

Tormented by self-doubt and riddled by anxiety, the individual may reach for a solution through salvation.

By confessing his or her wickedness the patient becomes a candidate for forgiveness and a future blessed existence.

Attitudes of defeatism change to those of hopeful anticipation.

As one of God's chosen instruments, a sense of dignity is acquired.

Competitiveness may then be abandoned for compassion and hate for love.

Some patients, comforted by religious forms of therapy, benefit also through participation in church activities.

Within its limited orbit, therefore, guidance has utility in therapy.

One does not employ it under the illusion that any deep changes will occur in underlying conflicts or the dynamic structure of the personality.

Causal factors are usually whitewashed, and the person is encouraged to adjust to problems

rather than to recify them.

The patient may be taught many methods by which emotional blind alleys can be avoided. The patient may learn to correct certain defects or to adapt to circumstances that cannot be changed.

However, where guidance is not supplemented by other therapies calculated to render the person more self-sufficient and independent, fundamental difficulties in interpersonal relationships will probably not be altered.

**TENSION CONTROL** Tension activates many disturbing physiological and behavioral tendencies. More subdued than anxiety, of which it is undoubtedly a component, it often registers its effects subversively through the autonomic nervous system, influencing the functioning of various organs.

It is one of the earliest signs of emotional disturbance, and once mobilized [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 170 it may continue to torment the individual even after neurotic defenses have been established.

Tolerance of the effects of tension varies.

There are some persons whose repressive mechanisms work so well that they are unaware of how tense they are even though their physical health is affected by resultant physiological imbalances.

There are others whose sensitivity to tension is so extreme that they are in a constant state of uneasiness and discomfort.

Individuals with poor impulse control may release their tension in passionate outbursts even though this leads to violence, sexual acting-out, and sundry behavioral improprieties.

Because tension is so discomfiting, it is little wonder that escape from it constitutes a chief preoccupation of human beings, who will eagerly utilize assorted devices in pursuit of peace of mind.

Some of the contingencies exploited have a destructive potential.

Alcohol, for example, is often employed as a potent tranquilizer; its use is universal, a part of our social tradition.

It narcotizes the brain and easily, while its effects last, subdues awareness of tension.

Most people are capable of controlling the intake of drink, but for those who attempt to utilize it as a prime calmative resource, it poses serious hazards.

In recent years other tranquilizing substances (such as Valium and Xanax) have entered the scene.

Enthusiastically prescribed by physicians (by far, they constitute the most commonly sold drugstore medicaments), they are fervently utilized by patients, some of whom after several months may become addicted.

Barbiturates and nonbarbiturate hypnotics continue to enjoy a risky popularity.

This is not to deprecate the value of tranquilizers, sedatives, and hypnotics as temporary expedients, but caution must be exercised in their use since they cannot be employed as a way of life.

Among the less noxious modes of regulating tension are exercises in self-relaxation, self-hypnosis, meditation, and biofeedback.

There is no reason why these devices cannot be employed as part of a comprehensive program where tension is too unsettling.

Their influence is palliative and can be helpful, but their value is greatest where they are utilized with psychotherapy (see Chapter 56).

It is to be expected that many practitioners, with and without proper qualifications, [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 171 will offer unique schemes for tension release, including bodily massage, tapping, slapping, touching, intimate games, screaming exercises, spiritual

discipline, and self-induced altered states of consciousness.

Popular volumes flood the market and have varying virtues for individuals seeking self-help direction for tension control.

The professional person may derive some good ideas from a few of these volumes (e.g., Krippner & Kline, 1972) that can be adapted to one's own technique and style of operation.

Muscle relaxation exercises have been used for many years.

Many of these are founded on the system of the nurse, Annie Payson Call (1891), which combined muscle relaxation, rest, and "mind training" for purposes of repose.

The best known modern exercises are those of E.

Jacobson (1938) and Rippon and Fletcher (1940).

D.

H.

Yates (1946) and Neufeld (1951) have described a series of exercises that enables the individual to gain voluntary control over tension.

Massage enhances muscle tone in addition to encouraging relaxation (Jensen-Nelson, 1941; Mennell, 1945).

Enforced rest also has a relaxing effect on the individual's muscular system.

In part, this was an objective of the old Wier-Mitchell (1885) "rest cure," which combined isolation, diet, massage, and electrical stimulation.

These measures were often reinforced by prescribed isolation from relatives.

Weir-Mitchell's method gained wide repute, although beneficial effects were probably as much induced by psychologic as by physiologic factors.

Schultz's "autogenic training" (Schultz & Luthe, 1959) provides the dividend of muscle relaxation during self-hypnotic exercises.

The influence of muscular relaxation on the individual's tension is purely palliative and should be accompanied by some form of psychotherapy.

Unlearning of dysfunctional habits that escalate tension will require daily practice of a chosen method (Benson, H., et al, 1974).

This may involve nothing more than E.

Jacobson's simple progressive relaxation of muscle groups (1938) from scalp to toes or vice versa.

Such self-relaxation techniques can quiet major muscle groups and ultimately lead to substantial tension control.

It is estimated that on the average approximately 2 months of daily practice are required for satisfactory results.

As the subject experiences a sense of awareness about his or her bodily reactions, the subject may be able deliberately to reproduce at will an adequate state of relaxation without needing to go through all the [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 172 prescribed exercises.

Autogenic training (Schultz & Luthe, 1959) strives for a reorganization of subconscious thinking patterns through the use of a technique of "passive concentration," an unstructured relaxed form of cogitation and association.

No formal muscle relaxation maneuvers are utilized, yet the technique promotes a sense of warmth and lightness over the entire body.

Suggestions emphasize peacefulness and quietness and enjoin the subject to allow such feelings to develop without forcing them.

Meditation has a long history, most frequently being associated with the Buddhist religion (particularly the Zen cult) and with Yoga.

More recently Transcendental meditation has attracted large groups of people, particularly the young (Forem, 1973).



Experimental studies have shown that meditation can produce striking psychophysiological effects, including alleviation of tension, lowering of oxygen consumption and metabolic rate, and decrease of cardiac output (Wallace, RK, 1970; Glueck & Stroebel, 1975).

Many techniques exist for the production of meditation, ranging from practiced suspension of thinking to concentration on monotonous environmental stimuli, to repetition of certain sounds or words (mantras), to special forms of physical exercise.

The association of meditation with mysticism is understandable.

As in hypnosis, a change of body image and the evocation of weird fantasies may strike some as evidence of preternatural worlds.

But meditation may be practiced apart from mystical union with the absolute and without achievement of "enlightenment" and "universal wisdom," should these essences not suit the philosophical bent of the subject.

Faithful practice of meditation from 20 to 30 minutes daily is usually required for proper practice toward mastery of tension (Carrington & Ephron, 1975).

Biofeedback training is a recent entry into the arena of self-regulated tension control (Stroebel & Glueck, 1973; Glueck & Strobel, 1975).

There is considerable evidence that one may gain conscious command over involuntary bodily functions by receiving sensory information from visceral organs.

There gradually develops a type of operant conditioning that enables a person to monitor certain physiological functions.

The most common instruments employed are those that record changes in skin potential (psychogalvanic meter), brain-wave activity (EEG machine), muscular function (electromyograph www.freepsychotherapybooks.org 173 machine), and skin temperature (thermal machine).

By learning to diminish or increase the auditory signals or amplitudes on a visual scale, one may be able to produce greater quantities of alpha waves (a phenomenon associated with lessened anxiety), to reduce activity of the sympathetic nervous system, and to achieve profound muscle relaxation.

Lowering of blood pressure in hypertension, healing of stomach ulcers, and relief of migraine are among the conditions that have been successfully managed.

Relatively easily mastered instruments are available for biofeedback training through which most persons are able to develop an ability to regulate subconscious physiological activities. This kind of visceral learning holds out great promise for the future in treating psychosomatic ailments.

Therapists may profitably explore this field to see whether it may enhance their therapeutic repertoire, should their practice involve the treatment of many patients with psycho-physiological ailments.

In the event the therapist does not possess the instruments, the patient may be referred for adjunctive biofeedback therapy to a behavior therapist skilled in the method.

Most patients can be trained in about 10 sessions.

Some practitioners utilizing tension control as the primary treatment method employ a combination of techniques, for example, progressive relaxation, skin galvanometry, hypnosis, and meditation.

In my own experience, I have found that most patients do well with self-relaxation alone, utilizing an audiotape, the making of which is described later in the book.

Where this does not suffice, I have sometimes employed an inexpensive psychogalvanometer with electrodes that attach to the fingers as a way of measuring skin resistance.

Some patients do better if they possess a means of overtly gauging their progress, and a biofeedback apparatus in this way serves them well.

Patients with tension headaches often benefit from learning how to relax the frontalis muscle

with the electromyographic (EMG) machine, while migraine may respond to learning to direct the blood flow from the head to a hand, employing a thermal biofeedback apparatus.

MILIEU THERAPY (ENVIRONMENTAL ADJUSTMENT, SOCIOTHERAPY) A vast number of stimuli from outside impinge on the individual daily.

Some of [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 174 these are clearly responsible for an existing acute emotional upset.

Some are habitual aggravations that keep chronic troubles alive.

In milieu therapy we attempt to define and eliminate provocative environmental irritants or to remedy deficits in the living situation that create problems for the person.

Long considered an exclusive province of the social worker and rehabilitation counselor, many psychotherapists are recognizing the importance of dealing with defective environmental contingencies as part of the treatment process.

Broadening the scope of their concerns beyond their usual intrapsychic and interpersonal focus acknowledges that the patient is subject to influences away from the therapist's office that can sabotage the treatment effort.

Perhaps more important are the preventive aspects of a regulated environment that regard such things as nurseries as more than a dumping grounds for the children of working mothers, and hospitals as more than a warehouse for the storage of the mentally disabled.

The writings of Aichorn (1948), Bettelheim and Sylvester (1948), Main (1946), M.

Jones (1952, 1956, 1957, 1959), Stanton and Schwartz (1954), Cumming and Cumming (1962) have documented the growth of the movement outlining the history, rationale, and value of milieu therapy for preventive and therapeutic purposes.

Therapeutic nursery schools, residential treatment centers for children, psychiatric hospitals, halfway houses, daycare centers, rehabilitation centers, nursing homes, and other institutions have attempted to apply principles from education, psychoanalysis, and behavior therapy to the organization and management of therapeutic units (Greenblatt et al, 1955; Cumming & Cumming, 1962; Sherwood, 1975; Kramer & Kramer, 1976; Gunderson et al, 1983; Noshpitz et al, 1984).

Dominating the thinking has been the concept that supplying patients with the benefits of a scientifically organized therapeutic community has, in practice, displayed both benefits and limitations (Greenblatt et al, 1955; Knobloch, 1960, 1973; Rapaport, L, 1962; Karasu et al, 1977; Talbott, 1978; Gunderson, 1980; Oldham & Russakoff, 1982; Gunderson et al, 1983).

Great diversity of design are obviously possible varying with the specific facilities and resources available in a community.

Their efficacy, no matter how ingenious they may seem, is crucially dependent on (1) the dedication and skill of the involved personnel, (2) how completely the environment accords with the demands of the patient, and (3) the singular reactions of the patient to what is being done for and to him or her.

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 175 No matter how benevolent a controlled environment may seem, the attitudes and reactions of administrative and clinical professionals will determine its therapeutic impact.

Many programs have been wrecked by negative countertransference reactions of the persons in charge.

The treatment and rehabilitative facilities must also be vigorous and varied to fit in with the requirements of different patients.

Unless a program is flexible and possesses many alternative resources, it will not be suitable for all assigned individuals.

Finally, some patients respond adversely to certain environments no matter how therapeutic they may seem.

For example, detached and schizoid patients may not be able to tolerate the overload of stimuli

characteristic of some programs (Van Putten, 1973).

Very often patients are so bound to an existing life situation, because of a sense of loyalty or because of a feeling that they have no right to express demands, that environmental distortions are tolerated as unalterable.

They may be unaware that tension and anxiety are generated by specific conditions, blaming their difficulties on things other than those actually responsible.

This may act as a deterrent to psychotherapy.

The therapist may have to interfere actively with environmental aspects that are grossly inimical to the best interests of the patient.

This may necessitate work with the patient's family, for it is rare that the patient's difficulties are self-limited.

The various family members may require some kind of help before the patient shows a maximal response to therapy (see Family Therapy, Chapter 52).

Social work more than any other discipline has evolved the most complete and best organized system of environmental manipulation (Atkinson et al, 1938; Bruno, 1948; French, LM, 1940; Jewish Board of Guardians, 1944; Lowrey, 1946; Menninger, WC, 1945; Pray, 1945; Ross & Johnson, 1946; Towle, 1946; U.S.

Children's Bureau, 1949; Van Ophuijsen, 1939).

Among services rendered are mediation of financial, housing, work, recreational, rehabilitative, marital, and family problems.

Many of the environmental manipulative techniques elaborated in social work may be employed by the therapist when situational difficulties are so disturbing to the individual that he or she is unable to live with them.

Or the therapist may utilize the services of a social worker as an adjuvant

[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 176 toward this end.

Home treatment is a modality through which inaccessible patients may be reached before a crisis makes hospitalization inevitable (Becker & Goldberg, 1970; Goldberg, HL, 1973).

It is especially applicable to disturbed individuals who are unable or unwilling to seek psychiatric help.

For such people a team effort is best.

That a visiting team sees the patient in the family and customary social setting facilitates diagnosis and treatment planning.

In many cases, the visit promotes greater confidence and trust and elicits far less anxiety than would be the case in a strange setting like a hospital clinic.

The interdisciplinary team with the different backgrounds of its constituent members can improve the quality of the total evaluation.

The interaction of the entire family at the initial home visit permits a better understanding of the dynamics that have led to the upset in the identified patient.

Although home treatment may seem an expensive process, the cost to the community is probably less than maintaining the patient in an institution and providing aftercare.

When several visits have been made, sufficient confidence may be inspired in the patient to be motivated to continue contacts with one or more members of the team in the central clinic.

Family therapy and drug treatment are particularly suitable in home treatment: paranoid patients have benefited from the use of long-acting injectible phenothiazines at intervals of 2 weeks (Goldberg, et al, 1970).

Sometimes the results of minor environmental adjustments are most gratifying, constituting all that the patient will need (Duncan, 1953; Suess, 1958; Murray & Cohen, 1959; Redl, 1959; Ytrehus, 1959; Lander & Schulman, 1960; Stanton, A, 1961; Wilkins et al 1963).

At other times, considerable psychotherapy will be required before the patient is able to take

advantage of improved environmental adjustments.

We should never lose sight of the fact that environmental difficulties, while accentuating the patient's problems, may merely be the precipitating factors.

The basis for the maladjustment exists largely in the personality structure.

As a matter of fact, most people have a tendency to objectify their problems by seeking out conditions in their environment that can justify feelings of upset.

For example, if a person has a problem associated with the fear of being taken advantage of by others, that individual will relate this fear to almost any situation in which  
[www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 177 the person is involved.

Environmental correction may, therefore, have little effect on the basic difficulty that has been structuralized in such widespread character disturbance that problems in interpersonal relationships appear to perpetuate themselves endlessly.

Indeed, the individual seems to create adversity toward which one is capable of reacting with customary defenses.

More confounding is the desire for martyrdom, a disturbed atmosphere actually being needed.

The individual may, for instance, seek to be victimized by others in order to justify feelings of hostility that could otherwise not be rationalized.

In cases such as this, the correction of environmental stress may produce depression or psycho-physiologic illness due to an internalization of aggression.

One of the most discouraging discoveries to the therapist is that in liberating a patient from a grossly distorted environment, the patient may promptly become involved in another situation equally as bad as the first.

The dynamic need for a disorganizing life circumstance will have to be remedied first before the patient responds adequately to environmental manipulation.

However, there may be no alternative in helping some patients (particularly nonmotivated, psychotic, psychopathic, or mentally defective persons) than to substitute for their habitual setting a radical change in the conditions of life even while they resist psychotherapy.

This may be the only practical means of management.

Even where there is an avoidance of intimate involvement, a milieu that taxes coping capacities minimally and supplies some gratifying experiences may bring about homeostasis and lead to greater self-fulfillment.

Hospitalization will occasionally be needed.

When considering hospitalization for an acutely disturbed, depressed, or psychotic patient, it is important to recognize the disadvantages and advantages of removing the individual from the situation to which that individual will have to adjust on returning home.

Most patients can be adequately cared for with partial hospitalization such as a well run day hospital.

Research studies show that this resource is to be preferred in the majority of cases and yields better results than full [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 178 hospitalization (Herz et al, 1971; Washburn et al, 1976).

However, if the patient is too disturbed or uncooperative, or if suicide is likely, admission to an institution may be advisable for the protection of that person and others.

What must be considered then is whether emotional stabilization is the preferred goal.

Under these instances, a short-term crisis-oriented period of patient care of 1 to 3 months may suffice.

However, where deeper structural personality changes are the objective for persons who have the capacity for such change but who resist altering their behavior, a longer period of hospitalization—1 year or more will be required.

In the latter case, the referring therapist should ascertain that the staff of the hospital

contains professionals trained in approaches that aim for deeper changes. Instead of hospitals, many rehabilitation units exist that may be ideally suited to some patients.

It may be productive for a therapist to visit some of the rehabilitation facilities in the area to see what services they render.

Some patients may be better suited for sheltered workshops than for other programs.

Still others will need halfway houses that act as a bridge to community living.

What is essential is the formulation of a plan both for immediate care and for therapy after the critical situation that necessitates referral has subsided.

Guidelines are suggested in the book by Frances et al, 1984.

A day hospital may, in many cases, serve as a substitute for hospitalization (Budson, 1973).

The cold impersonal institutional setting is replaced by a small intimate familylike atmosphere in which the individual may find a suitable identity.

In a controlled study, investigators at the Washington Heights Community Service of the New York State Psychiatric Institute discovered that "on virtually every measure used to evaluate outcome, there was clear evidence of the superiority of day treatment." Small communitybased residential facilities in the form of halfway houses or neighborhood residences have also been increasing in great numbers for alcoholics, drug addicts, criminal offenders, maladjusted youths, and persons variously handicapped physically and psychiatrically.

These facilities may serve as intermediate stations for the mentally ill patient after institutionalization, as a preliminary step to reentry into community life.

They may also function as a form of continuous aftercare (Axel, 1959; Bierer, 1951, 1959, 1961; Boag, 1960; Cameron, DE, 1947; Cameron, DE, et al; 1958; Carmichael, DM, 1960, 1961; Cosin, www.freepsychotherapybooks.org 179 1955; Craft, 1958, 1959; Fisher, SH, 1958; Goshen, 1959; Harrington & Mayer-Gross, 1959; Harris, A, 1957; Jones, CH, 1961; Kramer, BM, 1960; Kris, EB, 1959; Odenheimer, 1965; Rafferty, 1961; Robertson & Pitt, 1965; Steinman & Hunt, 1961; Winick, 1960; Zwerling & Wilder, 1964).

Obviously, emptying out mental hospitals and throwing sick patients onto the mercies of a community without adequate alternative rehabilitative facilities can be a disaster for both the patients and the community.

What is essential is that the environment to which patients are assigned be organized as a "therapeutic community," which is more easily said than done.

In hospital, day care, and other institutional settings occupational therapy plays a most important role [Fiddler, 1957; Schaefer & Smith, 1958; Subcomm.

Occup.

Therapy (no date on publication); Wittkower & LaTendresse, 1955; Conte, 1962], In this reference it may be possible to regulate the lives of patients so that a group work project becomes part of the daily routine.

The patient's responses and distortions of reaction to the group and work experiences may then be employed for counseling and psychotherapeutic purposes, in this way improving the patient's capacities for self-observation (Greenblatt et al, 1955; Mesnikoff, 1960).

Aftercare management of mental patients may also incorporate rehabilitation house programs (Tyhurst, 1957; Brooks, 1960), night care in a night hospital unit (Harris, A, 1957); neighborhood community centers (Kahn & Perlin, 1964), halfway houses (Williams, DB, 1956; Clark & Cooper, 1960; Wechsler, H, 1960b; Wayne, 1964; Landy & Greenblatt, 1965; Wilder & Caulfield, 1966), sheltered workshops (Black, 1959; Meyer & Borgatta, 1959; Hubbs, 1960; Olshansky, 1960), family care (Muth, 1957; Crutcher, 1959; Ullman & Berkman, 1959; Patton, 1961; Mason & Tarp, 1964), aftercare clinics (Muth, 1957), social clubs (Bierer, 1943; Palmer, MD, 1958; Wechsler, H, 1960b; Fleischl, 1962), and self-help groups, such as Recovery, Inc., Alcoholics Anonymous,

Gamblers Anonymous, and Synanon (Bromberg, 1961; Wayne, 1964).

In rehabilitative planning, the focus may be on the individual's physical work problems or social difficulties rather than on total functioning.

The skills brought into play here often involve a multidisciplinary approach, including methods and techniques from medicine, surgery, psychology, pedagogy, mechanics, sociology, etc.

(Greenblatt & Simon, [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 180 1959; Schwartz, CG, 1953; Bauman & Douthit, 1966; Eustace, 1966).

Some of the beneficial emotional effects of rehabilitation come from the sense of mastery that is restored to the individual when the handicap is overcome for which help was sought.

Programs organized around restoring the individual's social responsiveness help to eliminate isolation.

Recreational activities may constitute the bulk of services rendered in a day care center or in special units such as Fountain House (Fisher, SH, et al, 1960; Goertzel et al, 1960; Fisher & Beard, 1962), The Bridge, and the Social Therapy Club of the Postgraduate Center for Mental Health, in New York City (Fleischl, 1962).

Services may also include vocational training and placement, such as those of the Altro Workshop (Meyer & Borgatta, 1959).

There is much to be said for the development of comprehensive rehabilitation centers that contain a full range of services, including psychotherapy (Gelb, 1960).

The pioneer experiment of Maxwell Jones (1952, 1953, 1956, 1959) in establishing a therapeutic community has illustrated how useful this facility can be.

Since then, many comprehensive units have been erected, organized around social, recreational, work, and educational objectives, with and without formal programs of individual and group therapy (Agrin, 1960; Bennett, LR, 1961; Blackman, 1957; Brace-land, 1957; Caplan, 1961b; Clark, DH, 1971; Cohen, M, 1957; Denber, 1960; Edelson, 1964; Greenblatt, 1961; Irvine & Deery, 1961; Kepinski et al, 1960; Knobloch, 1960; Koltes & Jones, 1957; MacDonald & Daniels, 1956; Main, 1946; Meijering, 1960; Papanek, E, 1956, 1958; Rabiner et al, 1964; Rapaport, RN, 1960, 1961, 1963; Raskin, 1971; Rees, 1957; Siegel, 1964; Stainbrook, 1967; Talbot et al, 1961; Veomans, 1961; Visher & O'Sullivan, 1971; White, RB, et al, 1964; Wilmer, 1958).

An important aspect of the therapeutic community is that it removes patients from the irritants of their customary environment, theoretically giving them an opportunity 24 hours daily to build a new and viable personality.

The hope is that sufficient personality restructuring will occur to carry over into the patient's life situation after leaving the therapeutic facility.

There is some evidence that this can occur without formal psychotherapy.

In British Columbia Knobloch (1973) and a research team set up a [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 181 therapeutic community at Haney patterned after the Lobetch community of Czechoslovakia in which great behavioral changes were produced with dramatic economy in about 15 percent of the time required for an equivalent stay in a hospital.

Fifty-four patients were housed at Haney for 6 weeks and results compared with matched samples of patients treated in a day-care program and as inpatients.

Treatment efficacy was greatest at Haney at a fraction of the cost.

The results were credited to the use of a relatively closed socioecological system with a small group, assignment of work similar to the life work situation, the use of a variety of procedures including play, games, fantasy, psychodrama, kinesthetic therapy, and dramatic and pantomimic exercises, the homogeneity of the patients who all entered voluntarily into the program, and shared leadership.

Knobloch stresses that "this confusion about leadership makes some so-called therapeutic communities in North America lamentable examples of disorganization— social slums with

unnatural reinforcement contingencies which demoralize the patients.” Follow-up studies have shown that the patients at Haney retained their low symptomatology.

The regressive impact of institutionalization has been consistently reported, but there is no reason why this has to be so.

There will always be patients who cannot adapt to any facility other than an institution.

The problem is not that institutionalization is antitherapeutic, but that the way institutions are run contributes to the illness and withdrawal tendencies of many patients.

“As long as we continue to view the purpose of institutions as lumber rooms for individuals with deviant behavior, we will continue to foster countertherapeutic atmospheres” (Jones, M, 1973).

It is not enough to load up patients with drugs and assign them to clean quarters.

There is no reason why enlightened administration cannot organize institutions as viable therapeutic communities, with activities to enhance latent growth factors in patients.

Describing one hospital organized around psychotherapeutic lines, Gralnick and D’Elia (1969) explain, “We began to believe we had created a society that had a positive effect on patients because it permitted them to discover, explore, and overcome their sick ways and learn healthier ways of relating to others—ways that became part of them for the rest of their lives.” [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 182 We undoubtedly will find in the future that institutions will become less custodial than therapeutic because they will have flexible programs that consume the interests of patients, enabling them better to relate to one another and to the personnel, permitting them to develop new adaptive skills, and broadening their stress tolerance, which may carry over into the extrainstitutional environment.

**EXTERNALIZATION OF INTERESTS** Anxiety, depression, and excessive concern with symptoms may foster a withdrawal of the individual from interests and diversions that are a healthy component of living.

Where this has occurred, it may be expeditious to encourage such patients, as part of their psychotherapeutic program, to resume activities once meaningful to them or to help them to develop new diversions for leisure time (Davis, JE, 1938, 1945; Martin, AR, 1944, 1951; Slavson, 1946b).

Among the many outlets are sports (such as golf, tennis, swimming, handball, table tennis, boating, and riding), crafts (woodwork, needlecraft, weaving, metal work, and rug making), games (bridge, chess), photography and fine arts (painting, sketching, carving, sculpture, and mosaics), and various other activities (hiking, gardening, collecting—stamps, coins, etc.—music, and dancing).

Countless hobbies and recreations may be exploited where appropriate resources are available. Apart from physical exercise and the creative stimulation and temporary distraction from neurotic concerns, leisure time activities serve to bring the individual into contact with other persons, thus invading the individual’s social isolation and exposing him or her to the influences of group dynamics.

They become a kind of bridge to a more adequate contact with reality.

Organized recreational programs may provide a consuming interest for patients that may be symbolically meaningful and perhaps serve as a way of relating themselves to the recreational workers and to their peers (Haun, 1967).

With this as a model they may then learn to act more constructively with other people.

**Occupational Therapy** Among the best organized activities, universally employed in mental institutions, is [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 183 occupational therapy, which has proved itself a vital means of restoring the individual through the relationship with the occupational therapy worker, the symbolic meaning to the therapy worker of the tools used, and the end products of his or her manipulations, toward a more adequate integration (Am.

Occup.

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Assn., 1972; Dunton, 1915, 1945; Fiddler, 1957; Haas, 1946; Linn, 1975; Linn et al, 1962; Meyer, 1922; Schaefer & Smith, 1958; Sub-comm.

Occup.

Therapy (no date on publication); Wittkower & La Tendresse, 1955).

Occupational therapy is more than merely recreational and diversionary.

It may be the sole means of entering into the inner world of the withdrawn and regressed patient.

The occupational therapy worker has become an important and respected member of the psychiatric team (Conte, 1962).

As such, the therapy worker must be capable of providing adequate support and understanding for patients whose stability is at best tenuous and who are hostile, defeated, and uncooperative.

At the same time the occupational therapy worker must be able to set limits and to apply adequate controls when necessary.

The ability to detect, understand, and help the patients to work through conflicts as they manifest themselves in the patient's performances and relationships with fellow patients requires a high degree of training, perceptiveness, and stability.

Facilitating these aims are an understanding of both rehabilitation and psychodynamic concepts.

Further objectives are improvement in work, socialization, and activities of daily living.

Accordingly prevocational guidance, work adjustment, self-care, home-making, child care, and other services are offered to patients of all ages in hospitals through sheltered workshops, public and private schools, group homes, correctional institutions, day-care centers, community mental health centers, and in private facilities with special "concern for the complexities of sensorimotor and cognitive skills, personal motivation, self-determination, and adaptation" (Fine, 1984).

Essentially what is attempted is restoration of a sense of mastery that has been eroded by the emotional illness.

Quality assurance is an objective of the American Occupational Therapy Association that has a membership of over 35,000 professionals.

The Association as well as the Committee on Allied Health Education and Accreditation set standards for training and [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 184 practice.

Certification as an occupational therapist requires completion of standard training and full-time field work.

Registration and licensure are necessary for practice in many states.

THE CREATIVE ARTS THERAPIES Among the creative arts therapies, art therapy, music therapy, drama therapy, and dance movement therapy have been developing rapidly as adjuncts to the more traditional psychotherapeutic procedures, in addition to serving as supportive and educational methods in their own right.

Recognizing the usefulness of these modalities especially with the chronically mentally ill and severely disturbed children and adolescents, legislation was formalized during the late 1970s in the Mental Health Systems Act that resulted in new federal job classifications in the creative arts therapies and thus vitalizing these fields of therapeutic activity.

Originally designed for use in hospitals and institutions, these methods are increasingly being utilized with individuals, their families, and other groups in outpatient clinics and even private practice.

Standards of training, qualification, certification, registration, and licensure have been developed by emerging national organizations that are related to each area of the creative therapies, and research and writings have been increasing steadily in the last few years as a



consequence of this ferment.

**Art Therapy** Art therapy has become more formalized over the past decades and has developed from a casual technique used with children in clinics and with withdrawn patients in mental institutions to a sophisticated vehicle, which in the words of the American Art Therapy Association, is capable of "reconciling emotional conflicts and fostering self-awareness and personal growth." In this way it may serve as a bridge to help an individual "find a more compatible relationship between his inner and outer worlds." Understandably, art therapists trained in different theoretical schools will fashion their methods and interpretations around their special orientations whether these be [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 185 behavioral, Gestalt, humanistic, psychoanalytic, etc.

They will also devise uniquely personal ways of conducting the art session and of relating themselves to a co-therapist or team, if there be one, in an adjunctive collaborative, or supervisory way.

Art therapy may be done individually, in groups, or with families.

With schizophrenic patients, art therapy helps to bring them out of their regressive state and to establish a relationship with a helping individual.

Kwiatkowska (1967) has developed a popular evaluation method for art therapy candidates that designates strengths, weaknesses, and family interactions.

The diagnostic value of children's drawings in learning disabilities has been pointed out and detailed by Levick et al. (1979).

**Music Therapy** There is an inherent quality in sound that tends to calm or disturb, contingent on its physiologic and psychological effects.

Thus, sudden loud noises may arouse fear and apprehension; coordinated musical resonance may evoke pleasure; rhythmic cadence stimulates motor activity and relieves tensions; dissonant and shrill reverberations promote tension and may actually be experienced as painful.

Sounds influence both cortical and subcortical areas, affecting the autonomic nervous system.

Harmonious and rhythmic tones can arouse emotional feelings, promoting happy, excited, and sad moods.

For this reason, increasing interest has been shown in the use of music in medicine (Gaston, 1968; Licht, 1946; Mathews, 1906; Nordoff & Robbins, 1977; Ruud, 1980; Reese, 1954; Schullian & Schoen, 1948; Stein & Euper, 1974; Walters, L, 1954; Zimney & Weidenfeller, 1978).

A Journal of Music Therapy exists with many interesting articles on the subject.

There is little doubt that music can stimulate, relax and sedate, depending upon past associations and present symbolic significances (Colbert, 1963).

Accordingly, it has been employed in various ways in both psychotic and neurotic patients (Altshuler, 1944; Blaine, 1957; Blair et al, 1960; Folsom, 1963; Gutheil, 1954; Ishiy-ama, 1963; Jenkins, 1955; Joseph & Heimlich, 1959; Masserman, 1954; Muscatenc, 1961; Pierce et al, 1964; Reinkes, 1952; Rogers, 1963; Rose et al, 1959; Simon, B, et al, 1951; Soibelman, 1948; Winick & [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 186 Holt, 1960b; Wortis, 1960).

In some cases comprehensive programs have been organized around music appreciation assemblies, rhythm bands, singing groups, and concert and community singing clubs (Van de Wall, 1936; Soibelman, 1948; Gilliland, 1961, 1962), providing the means for both personal achievement and socialization.

In hospitals, day hospitals, and other settings background music may help release tensions, allay fears, provide an escape from boredom, and encourage teamwork (JAMA, 1956).

Music may also serve as a means of opening up channels of communication among patients (Snell, 1965).

It becomes a stimulus for the verbalization of emotion and a vehicle for the encouragement of

interaction in a group (Heckel et al, 1963; Lucas et al, 1964).

From projection of feelings the individual is helped to assume responsibility for his or her feelings.

Initial comments about the musical composition and its sources are followed by verbalization concerning inner emotional stirrings.

These are at first dissociated from the self, but later are acknowledged as part of the person.

The patient talks increasingly about how different forms of music affect him or her.

In a group setting there is an opportunity to listen to others, to compare feelings, and to identify with members of the audience (Weiss & Margolin, 1953; Sterne, 1955; Shatin & Zimet, 1958).

Transference toward the music therapist and the group members is almost inevitable and provides opportunities for exploration, clarification, and interpretation.

Fultz (1966) contends that music therapy properly employed may serve the following rehabilitative goals: (1) to aid in diagnosis and treatment planning, (2) to establish and cultivate socialization, (3) to promote selfconfidence, (4) to control hyperactivity, (5) to foster the development of skills, (6) to assist in the correction of speech impairment, and (7) to facilitate transition from nonverbal to verbal codification systems.

In this way, music becomes an adjunctive therapeutic agent, and the music therapist who is properly trained may be employed constructively as part of a team in a treatment program.

The education and professionalism of music therapists is constantly being improved and monitored by the National Association of Music Therapy and the American Association of Music Therapy.

Drama Therapy [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 187 Drama therapy includes those "approaches that stress the appreciation of creative theatre as a medium for self-expression and playful group interaction and that base their techniques on improvisation and theatre exercises." (Fink, et al, 1984).

The field has developed during the past 10 years and has led to the organization in 1979 of the National Association of Drama Therapy, which has developed standards of training and competence as well as qualifications for functioning as registered drama therapists.

Theater techniques were introduced into mental health practices by a number of pioneers such as Jacob Moreno, Peter Slade, Winifred Ward, and Jerzy Grotowski, which focused on the inner experiences of the actor rather than the audience, utilizing fantasy and role playing.

Since then, creative theater methods have been introduced into hospitals, prisons, outpatient clinics, and schools and have been found helpful in disturbed and handicapped individuals as well as other populations who are not approachable by the usual psychotherapeutic methods (Jennings, 1974).

D.

Johnson (1982 a,b) has employed a developmental approach on the theory that a block in development sponsors mental illness, which may be resolved by an external organized environment with definite boundaries, expectations, and rules.

Such a structured environment is provided in drama therapy through specifying the roles of therapist and group members and designating physical and other material arrangements.

The use of movement, sound, and imagery attempt to reconstruct "sensorimotor," "symbolic," and "reflective" stages, such as depicted by Piaget and kindred developmentalists as a means of progressing from preverbal to verbal forms of expression.

Characters chosen for parts in the designed drama represent forces, objects, animals, people, etc., with which or whom the individual is neurotically fused and identified, or who are idealized or conflicted entities.

Acting out of feelings and attitudes is encouraged through a set script or by improvisations

and role playing.

The therapist participates actively, attempting to resolve any impasse and restraint in free expression.

Role playing brings out both stable patterns of behavior and styles of interaction, as well as fleeting undercurrent inhibitions and impulsive outbursts that are both constructive and destructive in their consequences.

What is especially interesting is the [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 188 relationship of the actor to the role that is being played, as well as the interactions with the other actors in the play (McReynolds & DeVoge, 1977) (see also Psychodrama, Chapter 52).

Dance and Movement Therapy Dance and movement therapists work in clinics, psychiatric hospitals, day-care centers, correctional centers, and special schools all over the world on an individual and group basis.

In this way they serve as adjuncts to primary psychotherapists.

Such technical terms as movement empathy, interactional synchrony, and replication refer to how body movements have symbolic meanings and reveal information about inner emotions and mental process.

An individual's movements—posture, carriage, gait, and muscular coordinations— reveal tensions and character traits (Birdwhistell, 1952, 1959).

The way people dance— their body direction, coordination, and use of space—communicates many of their conflicts.

Free dance improvisations often bring forth gestures and movements of special parts of the body.

Sequences of expression, hesitations, and aggressive motions have meaning for the dancer.

Thus, dance may be exploited, not only to secure emotional release, providing an outlet for energy, but also as a way of bringing out attitudes and conflicts.

Solo dance performance and improvisations, folk dancing, and ballroom dancing help externalize feelings and act as a bridge to greater social participation (Rosen, E, 1957).

As in music therapy, the greatest effect on the patient is the interpersonal involvement.

In a dancing relationship the patient has an opportunity to work through some of his or her shyness and embarrassment toward greater assertiveness and selfexpression.

Movement therapists also employ body-awareness techniques on an individual and group basis.

The techniques employed draw from various sources, ranging from Yoga and Tai Chi Chuan to Jacobson's "Progressive Relaxation." The object is not only to induce relaxation but also to promote emotional catharsis, mental and physical alertness, and greater awareness and acceptance of the body (Winston, 1966).

Awareness of muscle tension, of posture and body alignment and of freedom or inhibition in [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 189 breathing and body movements helps to focus attention on the self, its defenses, and its conflicts.

Exercises to improve muscle tone and posture help reduce anxiety, release energy and enhance self-confidence.

Focusing on select muscle groups may release an outpouring of memories and encourage the reexperiencing of affect related to past events.

As the patient is stimulated to move body parts that are held rigid, the patient often begins to work through the fantasies and fears that have sponsored the original immobilizations.

Obviously the relationship with the movement therapist, and the latter's ingenuity and sophistication, will have a determining influence on results (Smallwood, 1974).

Dance and movement therapy was endorsed by the President's Commission on Mental Health for use in federally assisted facilities.

A number of educational programs now are available leading to a master's degree, and the

American Dance Therapy Association coordinates activities in the field.

Also available is a journal, *The American Journal of Dance Therapy*.

Structural Integration A method that has been utilized in recent years is that of structural integration, developed and practiced for many years by Ida Rolf.

It has been given the name of Rolfing (Rolf, 1958, 1973; Sperber et al, 1969).

The object of this therapy, which is usually confined to no more than 10 sessions, is to regulate posture and motor control defects that are said to influence emotional factors adversely.

"Deep message" is systematically applied to different anatomical areas.

Since there is no formal interviewing, therapists need not be psychologically trained.

By producing proper muscle coordination, a psychological integration is said to be obtained, a development attested to by testimonials proffered by many clients who have undergone Rolfing.

The method is organized around a dubious and anatomical electromagnetic theory.

Carrera (1974) has stressed the need to develop a conceptual framework with clear observable referents before one is capable of judging the true value of this technique.

It is quite probable that potent factors in the helping process here are the placebo effect and the impact of the relationship with the therapist.

Poetry Therapy [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 190 A new ancillary technique, poetry therapy had its inception in Greenwich Village, New York, when Eli Greifer, a lawyer and poet, brought together groups of mentally ill persons and recited poems that he later published in many books and pamphlets, including "Philosophic Duels," "Rhymes for the Wretched," "Poems for What Ails You," "Lyrics for the Lovelorn," and "Psychic Ills and Poem-therapy" (Leedy, 1966).

Greifer was aided in his campaign by a number of Village poets, including Maxwell Bodenheim and John Rose Gildea.

Poems, according to Leedy, may incite patients to constructive action as well as contribute to the evolution of a life philosophy.

Various techniques may be employed to secure group participation and interaction, including the recitation by patients of poems from readings or through memorization, and the writing of original poems (Harrower, 1974).

Discussion of the emotional stirrings created by poems and their meaning for the individual serves to bring the individual to greater selfunderstanding.

Various techniques may be employed along with group reading, such as, if the therapist is dynamically oriented, or if acting as a co-therapist to a poetry therapist, the discussion of fantasies and the encouraging of free associations to the poems (Pietropinto, 1975).

The advocates of poetry therapy contend, with compelling examples, that poetry can be an aid in reconstructive therapy; repressed and unconscious feelings are accelerated by the reading, listening to, and writing of poetry.

Thus Arsula Mahlendorf contends that "Harmonious emotion, allows catharsis, reaches into the unconscious by rhythm, rhyme, and imagery, creates coherence, order, and insight into hidden relationships, promotes integration between the conscious and unconscious, and thereby increases selfunderstanding, self-esteem, and mastery." (*Psychiatric News*, Oct. 1, 1982).

A National Association for Poetry Therapy has drawn together therapists interested in this modality.

Social Therapy The protected atmosphere of an institution, clinic, or agency encourages the organization of groups that can participate in a full spectrum of social activities (Bierer, 1943, 1958; Palmer, MB, 1958; Wechsler, H, 1960b).

At the Postgraduate Center for [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 191 Mental Health, in New York City, a program has been in operation for years that offers the individual who is or has been

in psychotherapy expressive opportunities for emotional growth and development (Fleischl, 1962; Fleischl & Waxenberg, 1964; Waxenberg & Fleischl, 1965; Fleischl & Wolf, 1967).

An environment is provided that is favorable for the overcoming of detachment, aggression, and social isolation.

While patients may have gained insight through psychotherapy into their characterologic distortions, they may be unable to overcome these defects in their usual environment that customarily imposes harsh penalties for even minor deviations.

A therapeutic social club fosters real life experiences through interaction with other persons, directly or by means of such creative activities as arts, crafts, games, reading, dramatics, dancing, and music.

Parties, visits to museums, and attendance at plays and outings provide further possibilities for rich experiences.

The influence of group dynamics here cannot be discounted, many deviant patterns altering themselves in the relatively tolerant setting of the club.

Group psychotherapy and vocational rehabilitation are often coordinately employed.

Social therapy is particularly valuable where normal familial relationships, social activities, and work situations produce upsetting and self-defeating reactions in a patient in spite of exposure to psychotherapy.

The organization and variety of activities are limited only by the inventiveness of the staff and the creative planning of the director.

A screening of members is essential to eliminate hallucinating and delusional psychotics, chronic alcoholics, drug addicts, violent and assaultive persons, and criminal psychopaths.

While supportive in its intent, social therapy may become reconstructive in its effect as it enhances self-understanding and furnishes rewards for socially constructive behavior.

Changed attitudes and emotions lead to greater self-realization and capacities for less ambivalent relationships.

REASSURANCE Reassurance is a partner in all psychotherapies, even where there is purposeful avoidance of pacifying consolations.

The very presence of the therapist serves to conciliate the patient, apart from the auxiliary agencies of placebo and suggestion.

This is especially [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 192 the case in severely upset patients who, lacking the capacity to handle their anxiety through their own resources, seek comfort in the shadow of an idealized parental figure.

Coming to therapy, therefore, in itself constitutes for the patient an inspiration that he or she is not hopeless.

Verbalized reassurances are often given the patient in supportive therapy, particularly when doubts are voiced concerning the ability to get well.

The patient is also solaced whenever in the grip of fears conditioned by irrational thinking.

The therapist discusses such fears openly with the patient, offering explanations of how baseless they are, in the hope of diverting the patient from destructive thinking patterns.

The most common misconceptions nurtured by patients are those that are related to a fear of going insane, of being blemished by a hereditary mental taint, of harboring an undetected malignant disease or abnormality, of having sustained irreparable damage through early masturbatory excesses, or of being overwhelmed by murderous impulses or perverse sexual fantasies.

Such fears lead to brooding and self-recrimination.

Where the patient is convinced of the sincerity of the therapist and accepts the therapist's authority at face value, that patient may be helped, by verbal placating, to master some misconceptions.

Reassurance is least successful when it is directed at basic, egosyntonic personality difficulties, particularly devaluated self-esteem and its derivatives.

**PRESTIGE SUGGESTION** Among the oldest of techniques is prestige suggestion, which is still employed extensively throughout the world by witch doctors, religious healers, and even professional psychotherapists.

Reported results vary from unbounded enthusiasm to a discrediting of the method as an irrational form of influencing.

Because it deals with effects rather than with causes, the method has many limitations; nevertheless, it may in certain cases be the only tactic to which the patient will respond. [www.freepsychotherapybooks.org](http://www.freepsychotherapybooks.org) 193 Suggestion plays a part in every psychotherapeutic relationship even though the therapist seeks to avoid it.

It need not be direct.

A patient came to see me with the complaint that intercourse was getting to be burdensome. Her husband demanded it twice weekly, which in her opinion was too often.

She found it difficult to be aroused, and she had to strain mentally to achieve orgasm.

When she finally climaxed, she was exhausted from the effort, and it took hours to recover.

When I asked her how often she had orgasms, she replied: "Always." "Why must you have orgasms each time?" I asked.

A long pause was followed by, "Maybe I shouldn't." The effects of her restraint were immediately apparent.

She began to stop challenging herself, to relax, and after several weeks to enjoy sexual contacts with and without orgasm.

Generally, patients will tend to select from the content of what the therapist says or implies certain things they want to hear, to which they feel it important to respond.